Documentation Gap Analysis: Independent Audit of TGA COVID-19 Vaccine Safety Monitoring Plan

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Abstract

This independent audit evaluates the TGA COVID-19 Vaccine Safety Monitoring Plan (February 2021). The Plan operationalised National Cabinet's commitment to enhanced pharmacovigilance accompanying provisional approval, covering 68.4 million doses, 91.2% of Australia's rollout. Enhanced monitoring claims cannot be independently verified when the underlying processes are undocumented. This audit tests whether that verification is possible.

The audit finds a systematic failure to demonstrate the enhanced monitoring committed to for Australia's largest provisional medicine rollout. Only 3 of 19 audited outputs (16%) are fully documented; 16 (84%) are partially or not documented. This is despite the TGA's own estimate, in its September 2024 OAIC submission, of 2,218+ pages of potentially relevant material, none identified as Plan-implementation documentation. The TGA reported to the Australian Senate that 148 signals were investigated and 57 regulatory actions were taken (updated to 150 signals in Question on Notice 559). No audit trail links individual signals to their assessment, disposition and resulting regulatory action or decision. The OAIC found that the implementation records "cannot be found or do not exist".

The TGA told the Senate (9 October 2025) that monitoring occurred through "day-to-day processes", indicating no systematic tracking against Plan objectives. The TGA cannot demonstrate documented implementation of its own Safety Monitoring Plan. This is a potential failure of regulatory accountability for the largest provisional medicine rollout in Australia's history.

The audit's open-source methodology applies ISO 19011:2018, ANAO guidance, and OSINT principles to public and FOI records, providing evidence to inform independent investigation and pharmacovigilance reform aligned with international standards and recognised good practice, including ICH E2E and CIOMS. Published under CC BY 4.0: open access and independently replicable.

Keywords

Therapeutic Goods Administration, COVID-19 vaccines, pharmacovigilance, provisional approval, documentary accountability, enhanced safety monitoring, public health governance, information asymmetry

Audit Governance and Reproducibility

Version history and supporting documentation on audit governance, evidence preservation and reproducibility are maintained in the project's Audit Governance documentation: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/tree/main/audit-governance

Complete data set including primary source evidence, documentation and research methodology is available at:
https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit

Disclaimer

This independent audit examines documented evidence of institutional processes and regulatory frameworks in the public interest. It does not attribute misconduct to individuals, question professional competence, or suggest deliberate misrepresentation by any person or organisation.

The findings concern systemic documentation and governance processes. They do not concern individual performance or clinical judgements regarding vaccine safety or efficacy.

This work has not been peer-reviewed. Corrections are welcomed. It is published under fair dealing provisions of the Copyright Act 1968 (Cth) for the purposes of non-commercial research, study, criticism, review and public interest analysis.

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Executive Summary

Background

On 13 November 2020, National Cabinet endorsed the Australian COVID-19 Vaccination Policy, which committed the Commonwealth to ongoing post-market safety monitoring of COVID-19 vaccines through the TGA's COVID-19 Vaccine Pharmacovigilance Plan. The policy identified active surveillance as a core pillar of TGA-led pharmacovigilance and assigned responsibility for monitoring adverse events following immunisation to the TGA.

In February 2021, the TGA published its COVID-19 Vaccine Safety Monitoring Plan, operationalising this policy commitment into a structured national pharmacovigilance framework. Described as building on existing pharmacovigilance systems, the Plan specified 17 strategies across 5 numbered objectives spanning adverse events following immunisation (AEFI) collection, signal detection, regulatory action, communication, and collaboration. This Plan represented TGA's operational implementation of the enhanced-monitoring expectation for provisionally approved COVID-19 vaccines.

This audit assessed the Plan's 17 strategies as outputs, discrete deliverable work products against which implementation evidence can be sought and evaluated, together with two additional governance outputs (GOV.1 Implementation Oversight and GOV.2 Performance Measurement), making 19 outputs audited in total. Defining the criteria and scope of an individual audit in this way is consistent with ISO 19011:2018 Clause 5.5.2 (defining the criteria and scope of an individual audit), and the two governance outputs reflect Commonwealth performance and records obligations (PGPA Act 2013 ss 37–39) and ANAO performance-audit methodology.

Within this framework, AusVaxSafety—coordinated by the National Centre for Immunisation Research and Surveillance (NCIRS) and funded by the Australian Government—described its large-scale active surveillance programme for COVID-19 vaccines as operating "as part of the national COVID-19 Vaccine Pharmacovigilance Plan, led by the TGA and the Australian Government". This positioning established AusVaxSafety as the delivery mechanism for the active surveillance pillar within the TGA-led safety-monitoring framework.

As part of the government's commitment to active and enhanced vaccine safety surveillance for COVID-19 vaccines, AusVaxSafety operated large-scale active surveillance throughout the rollout, completing around 6.8 million SMS-based surveys and reporting approximately 3 million adverse events and 62,000 medical visits by April 2025. However, OAIC-directed searches and FOI processes identified no coordination protocols, integration frameworks or audit trails showing how AusVaxSafety data were systematically incorporated into TGA's signal detection,

cumulative reviews or regulatory decisions, so any linkage between the two systems cannot be independently verified.

Operationally, TGA continued to rely on day-to-day pharmacovigilance processes, including spontaneous reporting to DAEN, sponsor and jurisdictional reports, routine signal-assessment meetings and publication of COVID-19 vaccine safety reports. When an FOI applicant sought an “implementation report” for the Plan (FOI 3643), TGA refused access under section 24A on the basis that no such document existed and directed the applicant to generic vaccine-safety webpages instead. In March 2025, following an Information Commissioner review, Decision [2025] AICmr 54 accepted TGA’s September 2024 MR22/00538 search evidence and upheld the “no documents” finding, confirming that no discrete implementation report could be located.

The pattern of TGA responses to implementation inquiries—from initial email requests through FOI decisions to OAIC review—is documented in Appendix C and is consistent with the audit's finding that Plan implementation was not systematically tracked.

The Australian National Audit Office’s performance audit of the COVID-19 vaccine rollout published in August 2022 examined planning, logistics, governance structures and achievement of vaccination targets, but did not assess vaccine safety monitoring, implementation of the Safety Monitoring Plan or compliance with provisional-approval pharmacovigilance conditions. There has therefore been no independent performance audit of TGA’s COVID-19 vaccine pharmacovigilance system or of documented implementation of the Safety Monitoring Plan.

Purpose

This assessment examines whether TGA’s COVID-19 vaccine pharmacovigilance system can demonstrate, on the available record, that the February 2021 Safety Monitoring Plan was implemented, governed and evaluated as intended. It focuses on:

- the existence (or absence) of a verifiable implementation trail for the Plan’s objectives and strategies for enhanced COVID-19 vaccine safety monitoring
- evidence of integration between AusVaxSafety and TGA signal-management processes
- the adequacy of signal-detection and decision-making governance and auditability
- the effectiveness of FOI and OAIC processes in making Plan-level implementation evidence available.

The review draws on public TGA and partner-agency documents, FOI decisions and OAIC submissions (including MR22/00538 and Decision [2025] AICmr 54), Senate

testimony by senior TGA officials, and targeted online searches. It examines documentation from February 2021, when the Plan was published, onward; searches were conducted through December 2025, with additional relevant sources identified and incorporated during the August 2026 revision. The audit benchmarks documented implementation of TGA's COVID-19 Vaccine Safety Monitoring Plan against the provisional approval framework for early-data COVID-19 vaccines and against international pharmacovigilance standards and recognised good-practice guidance relevant to TGA (ICH E2E, CIOMS, EMA GVP Module IX, and TGA pharmacovigilance guidance).[1]

Key Findings

Overall finding: Enhanced post-market monitoring committed to under these frameworks was not supported by documented implementation, governance or performance-measurement arrangements; records show routine pharmacovigilance activity but no traceable plan-level framework linking safety signals to regulatory actions or demonstrating compliance with Safety Plan commitments requiring enhanced post-market safety monitoring.

The transition of vaccines from provisional to full registration in 2023 leaves unresolved whether enhanced monitoring commitments were discontinued, continued under different arrangements, or—as the characterisation of work as "day-to-day processes" suggests—were never systematically implemented; this transition gap is examined in section 4.2.1.

Finding 1: No enhanced framework documentation

While TGA processed adverse event reports and published safety data, this assessment found no documented frameworks or protocols demonstrating enhanced post-market safety monitoring beyond business-as-usual processes. Systematic review of TGA's public material identified no Plan specific implementation, governance or integration documents, and OAIC-directed MR22/00538 sample searches (TRIM, eight terms) located no records TGA classed as documenting implementation, governance or AusVaxSafety integration for any Plan objective.

FOI 25-0166 indicates that documents within scope of the request exist within TGA systems: the TGA estimated more than 399 documents comprising 3,926 pages fell within scope of the original request, but refused access to the revised request under s 24(1)(b) on the basis that a practical refusal reason existed. The revised request sought up to 20 priority documents and, under item 2, at most thirty documents of

[1] Core framework documents: *TGA Consultation: Provisional Approval pathway for prescription medicines (2017); TGA Provisional registration extension and transition to full registration; Explanatory Memorandum, Therapeutic Goods Amendment (2017 Measures No. 1)* Bill 2017*; TGA COVID-19 Vaccine Safety Monitoring Plan* (Feb 2021); *ICH E2E Pharmacovigilance Planning; CIOMS Signal Detection in Pharmacovigilance; EMA GVP Module IX: Signal management*; *TGA Pharmacovigilance responsibilities of medicine sponsors.*

defined types (SOPs, protocols, workflows, implementation guidelines), six for each of the Plan's five objectives.

TGA's 2022-23 and 2023-24 Performance Report contains no references to the COVID -19 Vaccine Safety Monitoring Plan, no output tracking and no performance metrics for Plan implementation, indicating that enhanced monitoring commitments were not tracked at an institutional level.

The Comirnaty[2] AusPAR (transition decision 13 July 2023) and the Spikevax[3] AusPAR (published May 2023; transition decision 21 April 2023) are the principal public assessment records for these products' transition to full registration. As outlined in Section 4.3, both assess the sponsor's conditions of registration, a separate instrument outside this audit's scope, including clinical data, provisional registration conditions, Risk Management Plan and ongoing pharmacovigilance commitments. Neither identifies the February 2021 COVID-19 Vaccine Safety Monitoring Plan, its 17 strategies, or the Plan's enhanced-monitoring outputs. The AusPARs therefore contain no Plan-level governance or performance assessment, and nothing documenting how the TGA's own enhanced-monitoring commitments informed the transition decision. They are cited here only to show that the Plan-level enhanced-monitoring framework is absent even from the documents where regulatory assessment at transition is most fully recorded.

Finding 2: "Day-to-day processes" vs enhanced monitoring

A TGA official told a Senate committee on 9 October 2025 that "the five key themes outlined in that report essentially describe our day-to-day processes", a "standard sort of process" for post-market surveillance. Describing enhanced monitoring as business as usual does not readily reconcile with the February 2021 Plan's commitment to "enhanced" monitoring. That commitment was a policy commitment, operationalising the enhanced pharmacovigilance endorsed by National Cabinet in the Australian COVID-19 Vaccination Policy, rather than a binding condition imposed under the Therapeutic Goods Act 1989. The Plan explicitly frames its approach as enhanced pharmacovigilance, requiring systematic processes, documented frameworks and coordinated surveillance mechanisms beyond routine operations, responding to the higher uncertainty attaching to provisionally approved products. That the commitment was policy rather than statute does not lessen the

[2] Therapeutic Goods Administration (2023), *Australian Public Assessment Report for Comirnaty COVID-19 Vaccine (tozinameran)*, PM-2023-01183-1-2, AusPAR dated 7 August 2023, submission type: provisional to full registration. The AusPAR expressly records the status of the product-specific provisional conditions and post-approval commitments. https://www.tga.gov.au/resources/auspar/auspar-comirnaty-covid-19-vaccine

[3] Therapeutic Goods Administration (2023), *Australian Public Assessment Report for Spikevax (elasomeran)*, PM-2022-05374-1-2, AusPAR dated 10 May 2023, submission type: transition from provisional registration to full registration. https://www.tga.gov.au/resources/auspar/auspar-spikevax-1

accountability gap; it locates the failure in an unkept public assurance rather than in technical non-compliance with a legislative provision.

This characterisation, combined with TGA's direct communications pattern (generic pharmacovigilance links and FOI 3643's confirmation that no implementation report exists), points to three possibilities: routine pharmacovigilance was treated as sufficient; enhanced processes were implemented but not distinguished or documented; or the enhanced framework remained aspirational while routine processes continued. Each scenario represents a failure to implement the Plan as specified, and prevents TGA from demonstrating that the enhanced monitoring it committed to under the Plan was delivered.

Finding 3: Critical control gaps in signal detection and governance

Objective 2 (signal detection) shows 0 per cent fully documented implementation of the Plan's specified enhancements in the records identified to date. Although TGA investigated 150 signals and reports 57 regulatory actions, no finished COVID-19 specific signal investigation protocols or systematic oversight mechanisms linked to the Plan were identified, and the sequential and routine review processes in FOI 5082 are not accompanied by MaxSPRT or routine review outputs, and OAIC-directed TRIM searches found no AusVaxSafety–TGA coordination protocols, multi-source integration procedures or Plan specific guidelines for signal detection.

International pharmacovigilance guidance referenced by TGA (ICH E2E) and widely recognised best practice (including CIOMS and EMA GVP Module IX) establish principles for pharmacovigilance planning, signal management and documented traceability. EMA GVP Module IX specifically provides for an audit trail of signal-management activities, allowing traceability from assessment through decisions and their rationale. The absence of such documentation prevents independent verification that the Plan's enhanced monitoring framework was systematically applied and prevents determination of whether the enhanced process controls associated with provisional approval safety monitoring were implemented as intended.

Finding 4: Signal–action transparency gap

TGA reports 57 regulatory actions arising from 150 signal investigations, yet no public or FOI accessible documentation links specific signals to specific actions, describes the decision criteria applied or explains the disposition of the 93 signals that did not lead to documented regulatory action. Nor is there documented evidence of how AusVaxSafety data fed into these investigations or actions, leaving the triage and closure of non-actioned signals opaque.

Across approximately 75 million doses administered, TGA investigated around one signal per 507,000 doses, but whether this reflects appropriate sensitivity, under

detection or performance comparable with international benchmarks cannot be assessed without access to investigation protocols, causality assessment frameworks and prioritisation criteria.[4] Without documented decision criteria, investigation notes, or risk assessments, it's impossible to verify whether these 93 closures represented appropriate safety judgments or system failures to detect problems. International standards emphasise such documentation precisely so that systems can be assessed for rigour and sensitivity, not just counted by outputs.

Finding 5: Governance and performance measurement deficiency

FOI 3643, OAIC-directed searches and systematic documentary review identified no governance oversight or performance measurement documentation for Plan implementation. The FOI 3643 decision found no implementation report. In the Senate the TGA said there was “no single specific evaluation report outlining or benchmarking us against the elements of that plan” and that the Plan’s themes “essentially describe our day-to-day processes”, which indicates that implementation was not systematically tracked against the Plan’s objectives. This appears inconsistent with PGPA Act 2013 section 37, which requires the accountable authority of a Commonwealth entity to cause records to be kept that properly record and explain the entity's performance in achieving its purposes, and with ISO based principles that require systematic monitoring, measurement and documented evaluation against defined objectives.

For a plan accompanying provisional approval, the absence of implementation governance or performance tracking represents a fundamental control deficiency that prevents TGA from demonstrating whether enhanced monitoring was delivered as specified; the omission of any Plan related outputs or metrics from either TGA’s 2022-23 and 2023-24 Performance Reports reinforces this institutional accountability gap.

Finding 6: Insufficient evidence of process design and operating effectiveness

TGA cannot provide documented evidence that processes were designed, implemented and operated in accordance with the February 2021 Safety Monitoring Plan; operational activity alone does not constitute evidence of systematic, enhanced monitoring. While adverse event reports were processed, signals investigated and weekly safety reports published, these outputs demonstrate monitoring activity but do not, on their own, demonstrate that the enhanced framework was designed, documented and governed as an integrated system. The available record instead identifies these activities primarily through discrete operational outputs, without a documented Plan-level framework linking them to defined strategies, milestones,

[4] 75M total doses administered as of 30 Sep 2025 (Australian Immunisation Register). This audit uses the latest available official data at time of publication and will not be revised for routine statistical updates.

responsibilities, performance measures, escalation pathways and assurance of implementation.[5]

International pharmacovigilance standards and guidance adopted or referenced by TGA establish principles for pharmacovigilance planning and documented signal management. ICH E2E provides the framework for pharmacovigilance planning,[6] while CIOMS guidance addresses practical approaches to signal detection, prioritisation and evaluation.[7] EMA GVP Module IX provides for an audit trail of signal-management activities through a tracking system, allowing traceability and process control across all steps, including analyses, decisions and their rationale.[8]

For provisionally approved COVID-19 vaccines, where the policy framework presented enhanced monitoring as the compensating safeguard for accepting reduced pre-market evidence, the inability to produce such audit trails four years after Plan publication represents a systemic control deficiency that prevents verification of whether the enhanced monitoring framework was implemented as designed.

AusVaxSafety: active surveillance without documented integration

AusVaxSafety operated SMS based active surveillance throughout the rollout, completing 6.8 million surveys by April 2025, recording around 3 million adverse events (43.5 per cent of respondents) and 62,000 reports of medical attention (0.9 per cent), while TGA reports 150 signals and 57 regulatory actions. Yet OAIC-directed searches and multiple FOI processes identified no coordination protocols, integration frameworks or audit trails showing how AusVaxSafety data were systematically incorporated into TGA signal detection, cumulative reviews or regulatory decisions.

Without documented integration frameworks, independent parties cannot verify how population level active surveillance data informed TGA signal prioritisation, whether AusVaxSafety findings were reconciled with passive surveillance, or what criteria determined when AusVaxSafety results would trigger TGA investigations. This aligns

[5] Senate Community Affairs Legislation Committee*, Supplementary Budget Estimates 2025–26, Question on Notice 559 (SQ25-001584).*
[6] Therapeutic Goods Administration, *ICH Topic E 2 E – Pharmacovigilance Planning (PVP)*, CPMP/ICH/5716/03, overseas effective date June 2005. https://www.tga.gov.au/resources/resources/international-scientific-guidelines-adopted-australia/ich-topic-e-2-e-pharmacovigilance-planning-pvp
[7] Council for International Organizations of Medical Sciences (CIOMS), *Practical Aspects of Signal Detection in Pharmacovigilance: Report of CIOMS Working Group VIII*, Geneva, 2010. https://cioms.ch/publications/product/practical-aspects-of-signal-detection-in-pharmacovigilance-report-of-cioms-working-group-viii/
[8] European Medicines Agency, *Guideline on Good Pharmacovigilance Practices (GVP): Module IX – Signal Management (Rev. 1)*, EMA/827661/2011 Rev. 1, section IX.B.5, “Quality requirements”, pp. 11–12. https://www.ema.europa.eu/en/documents/scientific-guideline/guideline-good-pharmacovigilance-practices-gvp-module-ix-signal-management-rev-1_en.pdf

with the 0 per cent implementation rating for the data-integration category. The available record does not show whether the active and passive systems were coordinated or operated in parallel. The requirement for documented coordination protocols is not bureaucratic formalism but a fundamental accountability mechanism: without them, independent parties cannot verify whether AusVaxSafety's 62,000 medical attention reports and 3 million adverse event reports were systematically considered in TGA's signal prioritisation.

Expert committees distinguished from Plan implementation

Expert advisory committees such as ATAGI, ACV and AEFI CAN clearly existed and provided clinical and technical advice on COVID-19 vaccines, as documented in ANAO findings and public meeting statements. However, the presence of advisory structures does not substitute for documented Plan implementation, governance oversight or performance measurement.

No committee terms of reference, standing agendas or decision records were identified that demonstrate how these committees systematically fed safety advice into TGA’s signal management workflows, regulatory actions or Plan level implementation tracking. Advisory functions address what decisions should be made; the Safety Plan and provisional approval framework require evidence that enhanced monitoring processes were systematically implemented, integrated and measured—requirements that advisory committees alone do not fulfil.

Overall significance

These findings are significant because the policy framework presented enhanced post-market monitoring as a compensating safeguard for accepting reduced pre-market evidence under provisional approval, operationalised through the February 2021 Plan, and 68.4 million of 75 million COVID-19 vaccine doses (91.2 per cent) were administered while vaccines remained provisionally approved.

The inability to demonstrate systematic implementation of enhanced-monitoring commitments represents a fundamental accountability gap in Australia's regulation of provisionally approved medicines, leaving unverifiable the post-market safeguard on which early market access was granted for products with incomplete safety data.

Conclusion

The evidence assembled for this assessment indicates that, despite ongoing day-to-day pharmacovigilance activity and extensive public safety reporting during the largest vaccination rollout in Australia's history, there is no verifiable documentary trail showing that the COVID-19 Vaccine Safety Monitoring Plan's enhanced monitoring, integration and governance commitments were implemented in practice. The fact that the FOI "reasonable search" process required two senior TGA officers (a Senior Medical Officer and an Assistant Director in the Vaccines Surveillance

Section) to conduct manual searches for such a critical implementation document itself suggests the absence of routine governance or records-management systems that would ordinarily track and surface Plan-level implementation documentation.

Australia's COVID-19 vaccine rollout represents the nation's largest deployment of provisionally approved medicines: 68.4 million doses, 91.2 per cent of the total rollout, were administered on the basis of enhanced monitoring that cannot be verified through documented evidence. TGA can resolve this either by producing the missing documentation or, if documentation cannot be produced, by accepting independent investigation by the Australian National Audit Office, the Commonwealth Ombudsman or relevant parliamentary oversight committees.

After identifying 399 documents within scope of the request but refusing to process them under a revised request (FOI 25-0166), after OAIC-directed sample searches reporting no Plan implementation records (MR22/00538), and after four years of consistent institutional inability to demonstrate enhanced monitoring beyond routine processes, two possibilities remain: either the enhanced monitoring framework was never systematically implemented, or it was implemented but TGA cannot demonstrate it through documentation. Both scenarios represent a serious accountability failure, and neither is acceptable for 68.4 million doses administered under provisional approval.

Taken together, the absence of Plan-specific implementation and governance records in public sources, the negative results of OAIC-directed internal searches, and the refusal to process the revised request after the TGA estimated 399+ documents in scope of the original indicate that the probability of locating the required documentation is low. This in turn strongly indicates that the enhanced, documented post-market safety monitoring on which provisional approval of COVID-19 vaccines proceeded cannot be demonstrated to have been delivered, and that there has been a significant shortfall in TGA's pharmacovigilance governance and record-keeping obligations under Australian law.

On the available record, there has been a serious failure in the design, documentation and demonstrability of COVID-19 vaccine safety monitoring. Whether this reflects implementation failure or documentation failure, both constitute a material failure of accountability against the Plan's enhanced-monitoring commitments, Commonwealth governance and record-keeping obligations, and the international pharmacovigilance standards TGA has adopted; the scale of deployment warrants independent investigation.

1. Background and Context

1.1 Regulatory and policy setting

On 13 November 2020, National Cabinet[9] endorsed the *Australian COVID-19 Vaccination Policy*, which committed the Commonwealth to active and comprehensive post-market safety monitoring[10] of COVID-19 vaccines by the TGA, including appropriate adverse-event monitoring arrangements as vaccination scaled up.[11] [12] Development of the monitoring framework had commenced before the Policy was endorsed. In September 2020, the TGA's Advisory Committee on Vaccines (ACV) considered the developing *COVID Vaccine Pharmacovigilance Plan* and provided advice on enhanced adverse-event reporting, signal detection and investigation, regulatory and programmatic responses, communications and collaboration.[13] The ACV subsequently provided further advice and endorsed the amended Plan on 3 February 2021.[14]

In February 2021, the TGA published its *COVID-19 Vaccine Safety Monitoring Plan* (Version 1.0) as the operational implementation of this commitment, stating that it builds on existing vaccine pharmacovigilance systems to strengthen safety surveillance for COVID-19 vaccines and setting out five strategic areas, supported by 17 numbered strategies, covering enhanced adverse-event reporting, proactive signal detection, regulatory response capability, communications and stakeholder

[9] National Cabinet comprises the Prime Minister (Chair), State Premiers and Territory Chief Ministers and is an intergovernmental forum through which Australian governments work collaboratively on issues of national significance. National Cabinet endorsed the Australian Government's COVID-19 Vaccination Policy on 13 November 2020. National Cabinet does not displace the authority of individual jurisdictions; the Commonwealth, states and territories, as appropriate, remain responsible for implementing outcomes agreed by National Cabinet. See National Cabinet, *National Cabinet Terms of Reference* (September 2024), cls 1–3; Australian Government Department of Health, 'COVID-19 Vaccination Policy endorsement' (13 November 2020).

[10] In this report, the term 'enhanced post-market safety monitoring' (or 'enhanced monitoring') is used to summarise the strengthened post-approval monitoring requirements described in TGA's provisional registration guidance, and 'enhanced COVID-19 vaccine safety monitoring' refers to the enhanced monitoring commitments in TGA's February 2021 *COVID-19 Vaccine Safety Monitoring Plan*.

[11] Australian Government Department of Health. *Australian COVID-19 Vaccination Policy*. Canberra: Australian Government; 2020, pp. 6 and 13. Available at: https://www.health.gov.au/sites/default/files/documents/2020/12/covid-19-vaccination-australian-covid-19-vaccination-policy.pdf

[12] The Policy followed the Australian Government's August 2020 *Australia's COVID-19 Vaccine and Treatment Strategy*, which placed regulation and safety within the broader vaccine-and-treatment strategy and provided for comprehensive monitoring and surveillance of vaccine safety. Australian Government Department of Health. *Australia's COVID-19 Vaccine and Treatment Strategy*. Canberra: Australian Government; 2020. Archived by the National Library of Australia, PANDORA. Available at: http://nla.gov.au/nla.arc-184219

[13] Therapeutic Goods Administration. Advisory Committee on Vaccines (ACV). *Meeting statement: Meeting 16, 30 September 2020*. Canberra: Australian Government; 2020. Available at: https://www.tga.gov.au/resources/publication/meeting-statements/acv-meeting-statement-meeting-16-30-september-2020

[14] Therapeutic Goods Administration. Advisory Committee on Vaccines (ACV). *Meeting statement: Meeting 19, 3 February 2021*. Canberra: Australian Government; 2021. Available at: https://www.tga.gov.au/news/committee-meetings/acv-meeting-19-3-february-2021

collaboration.[15] External partners such as AusVaxSafety subsequently described their active surveillance programme as operating as part of a national COVID-19 Vaccine Pharmacovigilance Plan led by the TGA and the Australian Government, positioning active surveillance as a core component of the Commonwealth's enhanced-monitoring framework. [16] [17]

Provisional Approval and Enhanced Monitoring

Under the provisional-approval pathway, medicines may be registered for a time-limited period on the basis of preliminary clinical data where there is an unmet medical need. Because pre-market evidence is incomplete, sponsors' registrations may carry conditions relating to post-market monitoring, including safety surveillance, submission of confirmatory clinical data and active risk-management measures, with sponsors required to demonstrate compliance before any extension of provisional registration or transition to full registration is granted.[18] The enhanced monitoring framework examined in this audit is distinct: it is the TGA's own operational commitment set out in the February 2021 Plan, not a condition imposed on sponsors.

The Therapeutic Goods Act 1989 establishes the statutory pharmacovigilance architecture, including safety-information obligations principally applying to sponsors, while the enhanced signal-management framework examined in this audit arose from the TGA's February 2021 policy commitment rather than from a specific statutory prescription. This report therefore treats the Therapeutic Goods Act 1989, TGA's provisional-registration guidance and the National-Cabinet-endorsed vaccination policy as establishing a provisional-approval pathway that accompanied enhanced post-market safety monitoring, with the February 2021 COVID-19 Vaccine Safety Monitoring Plan providing TGA's operational implementation of those expectations for COVID-19 vaccines; the audit question is whether TGA can demonstrate, through documentation, that this enhanced-monitoring framework was systematically implemented and can be verified across the provisional period, including at the points where provisional registrations were extended and transitioned to full registration.

[15] Therapeutic Goods Administration. *COVID-19 Vaccine Safety Monitoring Plan*. Version 1.0. Canberra: Australian Government Department of Health; February 2021. Available at: https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf
[16] *Active and enhanced vaccine safety surveillance for COVID-19 vaccines in Australia*. Sydney: National Centre for Immunisation Research and Surveillance; 2024. Available at: https://www.ausvaxsafety.org.au/active-and-enhanced-vaccine-safety-surveillance-covid-19-vaccines-australia enhanced-monitoring framework.
[17] Therapeutic Goods Administration. *Consultation: Provisional approval pathway for prescription medicines*. Available at: https://www.tga.gov.au/sites/default/files/consultation-provisional-approval-pathway-for-prescription-medicines.pdf
[18] https://www.tga.gov.au/sites/default/files/consultation-provisional-approval-pathway-for-prescription-medicines.pdf

Independent legal commentary published in May 2021 similarly described provisional registration as permitting time-limited registration on the basis of preliminary clinical data, subject to conditions including the provision of further clinical data and ongoing safety monitoring.[19] It also identified the broader post-market monitoring arrangements operating during the COVID-19 vaccine rollout, including the Black Triangle Scheme, TGA weekly safety reporting and AusVaxSafety surveillance. This contemporaneous commentary provides independent corroboration of the regulatory context in which the TGA's enhanced COVID-19 vaccine safety monitoring arrangements operated.

Appendix A, *Regulatory and accountability framework for COVID-19 vaccine safety monitoring in Australia*, explains the statutory, policy and governance context in which the TGA COVID-19 Vaccine Safety Monitoring Plan operated. It identifies the relevant duty-holders and distinguishes statutory requirements from the TGA's policy and operational commitments and from recognised international guidance. This provides the regulatory context for the implementation assessment undertaken in this audit.

In practice, TGA's COVID-19 vaccine pharmacovigilance relied on familiar day-to-day processes: spontaneous adverse event reports to the DAEN database, sponsor and jurisdictional reporting obligations, periodic safety reports, and regular internal signal-assessment workflows, as described in Senate Community Affairs Legislation Committee testimony on 9 October 2025.[20] The Plan sat above these routine activities and was intended to provide an enhanced, documented framework linking multiple data sources, signal-management processes and governance arrangements for provisionally approved vaccines.

Freedom of Information (FOI) requests and an Office of the Australian Information Commissioner (OAIC) review outcomes confirm not only that no discrete "implementation report" or labelled Plan-implementation file can be located; they do not show whether the Plan's enhanced post market monitoring activities were embedded in day-to-day processes or documented elsewhere.

In February 2022, TGA refused FOI request 3643 on the basis that no "implementation report" for the COVID-19 Vaccine Safety Monitoring Plan existed, while pointing to generic vaccine-safety webpages instead of Plan-specific documentation. The applicant sought an Information Commissioner review, and in March 2025 the Office of the Australian Information Commissioner (Decision [2025] AICmr 54) accepted TGA's September 2024 MR22/00538 search evidence and

[19] Greg Williams, Sheena McKie and Mitchell Donohue, 'COVID-19 Vaccination Information Distillation: TGA's registration process for the COVID-19 vaccine and what to consider now', *Clayton Utz*, 13 May 2021. https://www.claytonutz.com/insights/2021/may/covid-19-vaccination-information-distillation-tgas-registration-process-for-the-covid-19-vaccine-and-what-to-consider-now?

[20] Senate Community Affairs Legislation Committee 2025, Official Committee Hansard – Budget Estimates, 9 October 2025, Parliament of Australia, Canberra.

upheld the “no documents” decision, confirming at FOI/IC-review level that TGA could not identify any discrete Plan-implementation records.

Australia’s COVID-19 vaccines were initially supplied under the TGA’s provisional approval pathway, alongside substantial advance-purchase agreements for multiple vaccine platforms.[21]

Box 1: Scale and Significance of Australia’s COVID-19 vaccine rollout

Between February 2021 and July 2023, 68.4 million COVID-19 vaccine doses were administered in Australia, representing 91.2% of the total rollout. The vast majority were given while vaccines remained under provisional approval.

Enhanced post-market monitoring is treated in this audit as the compensating safeguard on which provisional approval proceeded: reduced pre-market evidence was accepted in exchange for strengthened monitoring after approval. This was a national policy commitment, endorsed by National Cabinet in the Australian COVID-19 Vaccination Policy and operationalised in the TGA's February 2021 Safety Monitoring Plan, on which governments, health professionals and the public relied. This assessment finds that the TGA cannot demonstrate, through documented evidence, that this enhanced monitoring was systematically implemented as the Plan specified.

This is Australia's largest deployment of provisionally approved medicines. Where the safeguard that justified accelerated access cannot be verified, the assurance underpinning the provisional pathway cannot be independently confirmed, a documentary and accountability gap at the point where verification mattered most, and one that bears directly on the integrity of the provisional approval framework for any future emergency deployment.

This report therefore assesses the implementation of the Safety Monitoring Plan in a context where both TGA and the OAIC have formally acknowledged that no dedicated implementation report or Plan-specific governance and integration documents can be located.

TGA's day-to-day pharmacovigilance activities were conducted against the backdrop of the enhanced post-market surveillance expected for provisionally approved vaccines under the National Cabinet-endorsed policy and operationalised in the Plan. Enhanced post-market surveillance was a central commitment of the provisional-registration framework and associated TGA guidance under the Therapeutic Goods Act 1989, intended to address uncertainties inherent in

[21] For background on vaccine types, provisional approval dates and early purchase agreements, see Australian Parliamentary Library, *COVID-19 vaccines: a quick guide*, 2021–22.

accelerated vaccine development and rollout, and was operationalised through the COVID-19 Vaccine Safety Monitoring Plan.[22] Between February 2021 and October 2025, approximately 75 million COVID-19 vaccine doses were administered in Australia under the COVID-19 vaccine approval and rollout framework, the majority during the period when key vaccines were provisionally approved.[23]

The Australian National Audit Office's performance audit Australia's COVID-19 Vaccine Rollout (Auditor-General Report No. 3 2022–23) assessed the planning, governance and implementation of the national rollout by the Department of Health and Aged Care.[24] The audit examined issues such as rollout planning, logistics, governance arrangements, data quality, stakeholder engagement and achievement of vaccination targets, but did not examine vaccine safety monitoring, detailed coordination between vaccine stakeholders or implementation of the COVID-19 Vaccine Safety Monitoring Plan, or compliance with provisional-approval safety-monitoring conditions. The report also noted that the Commonwealth established expert committees, including ATAGI, ACV and SITAG, to provide clinical and technical advice on COVID-19 vaccines, risk–benefit assessment and rollout strategy. The audit therefore provides assurance about aspects of the rollout and governance structures, but it does not assess whether TGA's operational pharmacovigilance processes, systems and records collectively demonstrated implementation of the Safety Monitoring Plan; without such an assessment there is no independent assurance that safety signals were systematically traced through to regulatory decisions and actions during the rollout.

Accordingly, there has been no independent performance audit of TGA's COVID-19 vaccine pharmacovigilance system or of documented implementation of the COVID-19 Vaccine Safety Monitoring Plan; these issues fall outside the scope of Auditor-General Report No. 3 2022–23 and are the focus of this assessment.

This assessment examines the documented implementation of the February 2021 Plan through analysis of: (1) OAIC review submissions and internal audit findings; (2) FOI request responses and online document searches; (3) Senate Community Affairs Legislation Committee testimony under parliamentary privilege; and (4) official statements regarding implementation tracking. The review period covers February 2021 through February 2026 and focuses on whether TGA's routine pharmacovigilance activities and documented records can be shown to implement

[22] See Therapeutic Goods Administration, provisional registration and transition guidance and COVID-19 vaccine approval process (enhanced post-market monitoring for provisionally registered medicines), and COVID-19 Vaccine Safety Monitoring Plan (February 2021).
[23] For example, Department of Health and Aged Care, COVID-19 vaccination – vaccination data – 10 October 2025; TGA, COVID-19 vaccine safety reports (cumulative doses since rollout).
[24] *Auditor-General Report No. 3 2022–23, Australia's COVID-19 Vaccine Rollout, Australian National Audit Office,* 17 August 2022. https://www.anao.gov.au/sites/default/files/2022-10/Auditor-General_Report_2022-23_3_0.pdf

the enhanced monitoring, integration and governance framework set out in the February 2021 Plan.[25]

1.2 Scope and Methodology Overview

Scope of Assessment

This assessment evaluates whether the Therapeutic Goods Administration can demonstrate, through publicly available documentation, implementation of the 19 audited outputs, comprising the 17 numbered strategy items and associated outputs committed to in the February 2021 COVID-19 Vaccine Safety Monitoring Plan and two governance outputs. In its submission to the OAIC, the TGA identified over 2,200 pages of potentially relevant routine pharmacovigilance material. What this assessment did not find was any documentation of Plan-specific governance, integration protocols, or performance measurement: the enhancements that distinguished this Plan from business-as-usual.

In scope:

- Evidence of enhanced post-market pharmacovigilance processes beyond routine operations
- Traceable audit trails from signal detection through investigation to regulatory action
- Governance arrangements, performance tracking, and oversight mechanisms
- Records management effectiveness as demonstrated through FOI responses, OAIC-directed searches, and Senate testimony

Out of scope:

- Clinical safety, efficacy, or risk-benefit evaluations of COVID-19 vaccines
- Appropriateness or quality of individual regulatory decisions
- Operational effectiveness of partner organisations (AusVaxSafety, NCIRS, state health agencies), which are separate entities outside TGA's direct accountability
- Internal communications between TGA and partners unless publicly released

Regulatory framework and analytical scope

A key challenge in assessing regulatory accountability is the complexity and layered nature of the therapeutic goods regulatory framework. The TGA operates within an interconnected framework comprising the *Therapeutic Goods Act 1989* (Cth), associated regulations made under it, legislative instruments, regulatory guidance, policy commitments, and internationally recognised pharmacovigilance standards

[25] https://www.health.gov.au/sites/default/files/2024-10/covid-19-vaccination-vaccination-data-10-october-2024.xlsx

and guidance.[26] These sources differ in legal status and function: the Act and regulations establish statutory or regulatory requirements, while guidance, policy materials, and international standards provide regulatory expectations, government commitments, or recognised good practice without themselves necessarily creating statutory obligations.[27]

The February 2021 COVID-19 Vaccine Safety Monitoring Plan should be understood in this context. It operationalised the National Cabinet-endorsed Australian COVID-19 Vaccination Policy and set out enhanced vaccine safety monitoring commitments during the period of provisional registration.[28] The Plan did not itself create sponsors' statutory obligations or the conditions of provisional registration. Rather, it operated alongside the provisional-registration framework, within which sponsors had ongoing pharmacovigilance, risk-management, and evidence-generation responsibilities, including safety monitoring and adverse-event reporting, risk-management activities, and continuing clinical trials and longer-term safety and efficacy evidence.[29] TGA guidance provides for risk-management plans in provisional registration and recognises that implementation of an RMP may be imposed as a condition of registration.[30]

The Plan also contemplated coordinated review by sponsors and the TGA of emerging safety and effectiveness information, including post-market data and safety signals.[31] [32] It therefore intersected with existing sponsor pharmacovigilance responsibilities without itself becoming a statutory condition of provisional

[26] *Therapeutic Goods Act 1989* (Cth), ss 23AA, 25 and 29. https://www.legislation.gov.au/C2004A03952/2025-09-05

[27] Therapeutic Goods Act 1989 (Cth); Therapeutic Goods Administration (TGA), relevant regulatory guidance; *International Council for Harmonisation (ICH), ICH E2E Pharmacovigilance Planning; Council for International Organizations of Medical Sciences (CIOMS),* relevant pharmacovigilance guidance; *European Medicines Agency (EMA), Good Pharmacovigilance Practices (GVP).*

[28] Therapeutic Goods Administration (2021), *COVID-19 Vaccine Safety Monitoring Plan*, Version 1.0, February 2021. https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf

[29] Therapeutic Goods Administration (2024), *Pharmacovigilance responsibilities of medicine sponsors*. https://www.tga.gov.au/resources/guidance/pharmacovigilance-responsibilities-medicine-sponsors

[30] Implementation of an RMP may be imposed as a condition of registration; provisionally registered medicines are subject to ongoing pharmacovigilance and risk-minimisation requirements, including PSURs. Therapeutic Goods Administration (2024), *Submitting risk management plans for medicines and biologicals*. https://www.tga.gov.au/resources/guidance/submitting-risk-management-plans-medicines-and-biologicals

[31] Therapeutic Goods Administration, *COVID-19 vaccine safety monitoring plan, V1.0, February 2021*, p 4: the Plan states that information from ongoing clinical trials and safety studies would continue to be collected and analysed after provisional approval, and that the TGA, international regulators and vaccine sponsors would continuously review safety and effectiveness information from mass vaccination programs

[32] Therapeutic Goods Administration (2021), *Australian Public Assessment Report for ChAdOx1-S – COVID-19 Vaccine AstraZeneca*, AusPAR dated 15 February 2021, recording implementation of the COVID-19 Vaccine Safety Monitoring Plan, sponsor expectations concerning batch-specific pharmacovigilance, and proposed risk management plan conditions of registration. https://www.tga.gov.au/sites/default/files/auspar-chadox1-s-covid-19-vaccine-astrazeneca-210215.pdf

registration. The *Therapeutic Goods Act 1989* establishes the statutory provisional-registration framework, including the application, evaluation, and duration mechanisms in ss 23AA, 25 and 29.

This audit does not undertake a comprehensive legal analysis of every obligation arising from each regulatory instrument. It focuses on whether the enhanced COVID-19 vaccine safety monitoring commitments identified in relevant policy and regulatory materials were implemented and can be verified through available documentary evidence. Accordingly, the analysis distinguishes between legal requirements, regulatory expectations, policy commitments, and evidence of implementation. Where this report refers to a monitoring commitment as a legal or regulatory requirement, that characterisation is supported by a specific statutory or regulatory provision. Where a commitment is described in policy or governance terms, that reflects its actual source and should not be read as asserting a statutory obligation.

In particular, the enhanced monitoring set out in the February 2021 Plan is treated as a policy commitment operationalising the National Cabinet-endorsed policy framework, not as a statutory precondition of provisional registration. Conditions imposed on individual sponsors' registrations under the *Therapeutic Goods Act 1989* were considered where relevant to understanding the broader regulatory context; the audit does not assess compliance with individual sponsor conditions. The legal and regulatory implications of any non-implementation of the Plan therefore remain indeterminate and would require professional legal and regulatory assessment of the applicable approval conditions, statutory duties, and evidence.

Methodological Framework

This assessment applies ISO 19011:2018[33] audit principles to evaluate the Plan's 17 explicitly numbered strategies (1.1-1.5, 2.1-2.8, 3.1, 4.1, 5.1-5.2) plus two governance and performance-measurement processes required under Commonwealth accountability frameworks (PGPA Act 2013 ss 37–39; Archives Act 1983 s 24).

Each output was assessed against three implementation categories:

- Fully Implemented: Documented evidence of complete implementation
- Partially implemented: Process documentation exists with limited or no records of results
- Not Documented: No relevant evidence located after FOI, OAIC and public-record searches

[33] ISO 19011:2018 was the edition in force throughout the assessment period. It was superseded by Edition 4 2026 (the 2018 edition is applied here as the standard current at the time of the activity assessed).

Box 2: How to Read the Ratings

Classification note: Ratings describe the documentary record, not operational outcomes. They rest on the audit evidence available under ISO 19011:2018 (clause 3.9):

- ✓ means verifiable records of delivery
- ◐ means process documentation exists with limited or no records of results
- ✗ means no relevant evidence was located

No rating establishes that an output was or was not performed.

Evidence Base and Information Asymmetry

This assessment relies exclusively on publicly verifiable evidence:

- TGA's published Plan, official statements, and direct responses to information requests
- FOI-released materials 2022-2025 (FOI 3643, FOI 5082, FOI 25-0166)
- OAIC review submissions (MR22/00538, MR25/01153)
- Senate testimony under parliamentary privilege from senior TGA officials
- Publicly accessible partner organisation reports (AusVaxSafety, NCIRS)

The assessment recognises an inherent information asymmetry: TGA holds internal operational knowledge that may not be publicly documented. However, for a regulatory framework tied to provisional approval, the ability to demonstrate implementation through documentation is fundamental to public accountability and independent verification. The operational success of partner organisations cannot substitute for TGA's obligation to document fulfilment of its own Plan commitments.

Absence of documents after systemic searches provides evidence of significant implementation and governance gaps. If additional documentation exists but was not disclosed through FOI processes, OAIC reviews, or public channels, TGA may provide such documentation to address the findings identified.

Evidence Gathering Approach

This assessment's evidence base derives from two complementary sources, detailed in Appendices A and B:

Systematic Documentary Search (Appendix B): Structured online review of publicly available TGA documents, parliamentary testimony, and partner organisation reports conducted December 2024 – February 2025. Search methodology aligned with ANAO performance-audit methodology and Audit Insights,

using Plan-aligned search terms across TGA repositories, Senate Community Affairs Legislation Committee records, and regulatory partner publications.

Direct TGA Communications (Appendix C): Analysis of artefacts including key escalating requests to TGA (January-February 2022) seeking Plan implementation documentation, including two formal FOI requests: 3643 and 25-0166. Appendix C.4 included detailed statistical analysis of OAIC-directed search results (September 2024) involving TRIM searches using eight search terms, the broadest of which returned 531+ containers of documents that TGA described as relating to manufacturing audits and the vaccine programme, alongside TGA's sample estimate of more than 2,218 pages of potentially relevant material across the Plan's objectives, none identified as Plan-implementation documentation. TGA responses establish baseline institutional position: identical generic pharmacovigilance links provided across all three responses, with FOI decision explicitly confirming "no implementation report exists" while claiming "ample documentation" exists elsewhere.

These parallel evidence streams—researcher-initiated requests establishing institutional position, followed by comprehensive public document review—provide triangulated verification of documentation gaps identified.

Replicability and Falsifiability

The audit's findings are designed to be testable and, where applicable, falsifiable through the production of contradictory documentary evidence. If TGA can provide:

- Documented protocols for coordinating AusVaxSafety data with signal investigations
- Documented signal-investigation procedures specific to COVID-19 vaccines
- Documented decision-making criteria linking investigated signals to regulatory actions
- Documented Plan-implementation governance and performance-measurement systems

then the corresponding findings would be revised or withdrawn as appropriate. The assessment methodology is designed to be correctable through the provision of documentary evidence, consistent with scientific principles of falsifiability and audit standards requiring evidence-based conclusions.

Detailed methodological framework including evidence hierarchy, audit criteria, and standards applied is provided in Section 9.

1.3 Context: Monitoring Activity vs Enhanced Implementation

TGA conducted substantial safety monitoring activities during 2021-2023, documented in the November 2023 Safety Signals Report and confirmed in Senate testimony (October 2025). These activities included investigation of 150 safety signals, 57 regulatory actions, and publication of 150+ weekly safety reports demonstrating operational capability.[34]

Similarly, AusVaxSafety—Australia's active surveillance platform—successfully operated throughout the vaccine rollout, completing over 6.8 million COVID-19 vaccine safety check surveys as at April 2025. Of survey respondents, 43.5% reported at least one adverse event and 0.9% reported seeking medical attention (AusVaxSafety website, 1 December 2025).[35] This demonstrates that active surveillance infrastructure was established, operational, and generating population-level safety data at scale.

This assessment does not question whether monitoring activities occurred.

It examines whether those activities constituted the enhanced monitoring beyond routine pharmacovigilance committed to under the national policy framework and February 2021 Safety Plan, and whether implementation of that Plan was systematically documented and tracked—particularly the coordination between active and passive surveillance systems that Strategy 2.4 explicitly promised.

Available material shows that large volumes of COVID-19 vaccine safety data were collected, analysed and reported in aggregate, but there is no documented, traceable audit trail showing what happened to individual safety signals—how they were evaluated, what criteria were applied, or how they translated into specific regulatory decisions and actions.[36]

In effect, the arrangements are analogous to operating an extensive network of security cameras without a recording and logging system: activity clearly occurred, but there is no verifiable record of how specific safety concerns were handled.

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[34] These oral testimony figures were subsequently updated in the TGA's written answer to *Senate Question on Notice 559,* Department of Health, Disability and Ageing (DHDA reference SQ25-001584), 2025–2026, which reports 150 signals assessed and 57 regulatory actions taken. The 93 signals without a reported action (150 less 57) is the audit's calculation, not a TGA figure, and is indicative only: under EMA, Guideline on good pharmacovigilance practices (GVP) Module IX: Signal management (Rev 1, 2017), a signal may end with no action beyond routine pharmacovigilance or with several actions (IX.C.7). The same module expects an audit trail of signal management activities (IX.B.5). Searchable at https://www.aph.gov.au/SenateQON using QON number 559 or reference SQ25-001584. A detailed strategy-by-strategy analysis of the QON 559 answer is available at https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/analysis/tga-senate-qon-559-gap-analysis-2026.md

[35] According to AusVaxSaftey, adverse events are self-reported, have not been clinically verified, and do not necessarily have a causal relationship with the vaccine.

[36] https://www.tga.gov.au/resources/guidance/pharmacovigilance-responsibilities-medicine-sponsors

Under international pharmacovigilance guidance adopted by TGA (ICH E2E) and internationally recognised best practice (including CIOMS and EMA GVP Module IX), pharmacovigilance planning and signal management are supported by documented processes and traceability, including audit trails for signal-management activities. These framework elements enable verification that monitoring activities followed the enhanced processes specified in the Plan, rather than standard operational procedures. It is the documented enhanced framework—including the coordination of surveillance systems—not merely the occurrence of monitoring activities, that this assessment evaluates.

This research raises a simple question. The February 2021 COVID-19 Vaccine Safety Monitoring Plan was not merely a description of routine pharmacovigilance activity; it operationalised a national policy commitment to enhanced monitoring of provisionally approved COVID-19 vaccines. Its stated emphasis on “strengthening” and “enhanced” monitoring therefore creates an assurance question: was the additional monitoring framework committed to at national level actually implemented, systematically tracked and capable of independent verification, or was monitoring conducted principally through existing day-to-day processes?

1.4 Limitations of This Assessment

This assessment is limited to documentary evidence available through:

- Publicly accessible TGA publications and reports
- FOI-released materials (2022-2025)
- OAIC review submissions (MR22/00538, MR25/01153)
- Senate Community Affairs Legislation Committee testimony under parliamentary privilege
- Partner organisation public reports (AusVaxSafety, NCIRS)

Operational activities that occurred but were not documented cannot be evaluated through this methodology. The assessment does not question the professional judgement or good faith of TGA staff conducting safety monitoring activities. Rather, it evaluates whether the enhanced monitoring framework specified in the February 2021 Plan was implemented with the systematic documentation, audit trails, and governance structures that:

- International pharmacovigilance standards require (ICH E2E, CIOMS VIII)
- ISO audit principles require (ISO 19011:2018)
- Commonwealth accountability frameworks require (PGPA Act 2013, Archives Act 1983)
- Provisional approval arrangements under the Therapeutic Goods Act 1989 provide for registration on the basis of limited data, with continuing evidence-generation and post-market monitoring arrangements, and allow conditions to be imposed on registration.

TGA staff and partner-agency staff were not interviewed as part of this audit; the findings are based solely on documentary evidence and do not incorporate direct stakeholder testimony.

If TGA possesses additional documentation not disclosed through FOI, OAIC processes, or public channels that addresses the identified gaps, this assessment's findings may be revised accordingly upon provision of such evidence.

2. Implementation Status and Key Findings

2.1 Overall Assessment

This assessment identified a significant variance between commitments outlined in the February 2021 Plan documentation and verifiable evidence of implementation obtained through four years of investigation and FOI requests.

Of 19 enhanced pharmacovigilance Plan outputs including two governance outputs:

- 16% fully documented as implemented (3 outputs)
- 68% partially documented (13 outputs)
- 16% no evidence located after four years (3 outputs)

The assessment confirms that substantial operational activity occurred: TGA published over 150 weekly safety reports, processed adverse event notifications through DAEN, investigated 150 safety signals, and documented 57 regulatory actions. Partner organisations, particularly AusVaxSafety, established and operated active surveillance infrastructure at significant scale (6.8+ million surveys completed).[37]

However, the enhanced monitoring framework specified in the Plan—systematic processes, documented protocols, coordinated surveillance integration, governance oversight, and performance measurement—cannot be verified through available documentation. This creates a fundamental accountability gap: operational pharmacovigilance activities demonstrably occurred, but the systematic frameworks that would enable independent verification of “enhanced” monitoring cannot be evidenced.

TGA has not produced any documentation showing how AusVaxSafety data fed into its 150 signal investigations or 57 regulatory actions, or any audit trail explaining triage decisions for the remaining 93 signals where no further regulatory action was taken. Across OAIC-directed TRIM searches using eight search terms, no

[37] https://www.ausvaxsafety.org.au/vaccine-safety-data/covid-19-vaccines

AusVaxSafety–TGA coordination protocols, multi-source integration procedures, or Plan-specific implementation guidelines were identified.[38]

Three findings are particularly significant:

Documentation Pattern: Routine pharmacovigilance activities (weekly reports, DAEN database) are well documented, while enhanced elements specific to provisional approval (dedicated teams, coordination frameworks, systematic integration) are not evidenced.

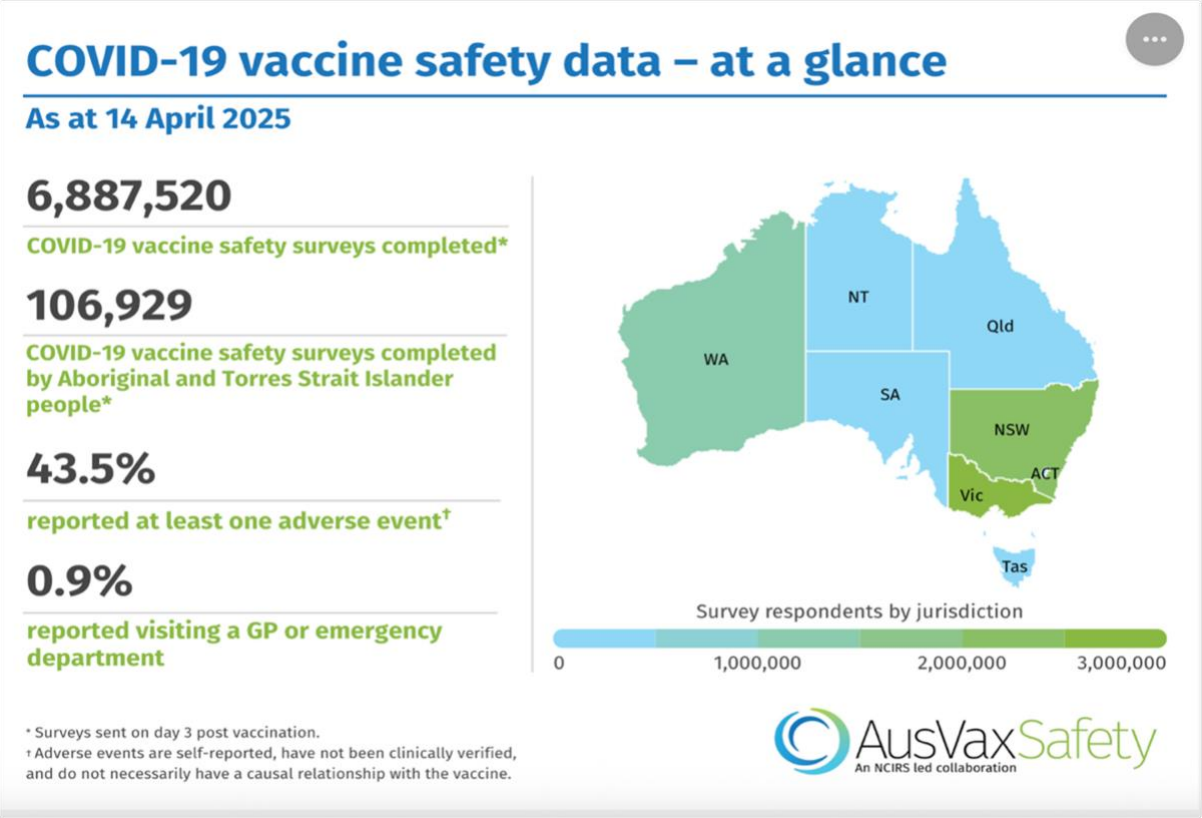
Characterisation Contradiction: A senior official described COVID-19 vaccine safety work as "day-to-day processes," raising a question about how the Plan's commitment to enhanced post-market monitoring was distinguished from routine pharmacovigilance during provisional registration.

Verification Impossibility: The absence of documented protocols, decision criteria, and audit trails prevents independent assessment of whether the pharmacovigilance system functioned with adequate sensitivity, whether decisions were consistently applied, or whether the enhanced monitoring required under provisional approval was implemented.

For provisionally approved products where enhanced monitoring is the safeguard justifying reduced pre-market evidence, the ability to produce such audit trails through systematic documentation is fundamental to demonstrating that the regulatory accountability framework operated as designed.

The scale of provisional approval deployment (68.4 million doses), combined with the systematic absence of implementation documentation across four years of investigation, indicates this is not an isolated records management issue but a systematic documentation deficiency in pharmaceutical governance with implications for future provisional approval frameworks requiring enhanced monitoring commitments.

[38] Therapeutic Goods Administration (2024). *OAIC Submission MR22/00538* – Response to written notice and direction under ss 55V(2) and 55(2)(e)(ii) of the Freedom of Information Act 1982 (20 September 2024). Attachments A–B document TRIM/Promapp search methods and results, including eight search terms yielding no documents TGA classed as Plan implementation records.



2.2 Key Findings

Finding 1: Absence of Enhanced Framework Documentation

While the TGA processed adverse event reports and published data, this assessment found no documented frameworks or protocols that would demonstrate enhanced monitoring capabilities beyond business-as-usual processes. (Appendix B, Section B.5.1; Appendix C FOI 3643 para 7). This represents a material gap between stated commitments and verifiable implementation.

In the FOI 25-0166 consultation notice (24 April 2025), the TGA advised that preliminary estimates identified more than 399 documents comprising 3,926 pages within scope of the original request. The estimate indicates that documents responsive to such a request can be located within TGA systems. The revised request, framed by reference to standard document types (SOPs, protocols, workflows, implementation guidelines), was subsequently refused under s 24(1)(b) on the basis that a practical refusal reason within the meaning of s 24AA existed. Whether any of these documents evidences Plan implementation has not been established, because none was processed.

Verification documentation absent at point of full registration: The Comirnaty[39] and Spikevax[40] AusPARs provide contemporaneous public records of the respective transitions from provisional to full registration and document sponsor-level regulatory assessment. Neither identifies implementation of the February 2021 COVID-19 Vaccine Safety Monitoring Plan or its 17 enhanced-monitoring strategies. This does not establish that such implementation was required to be recorded in an AusPAR; rather, it provides corroborating evidence that the Plan-level monitoring framework is not visible in the public records of the relevant regulatory transitions.[41]

Finding 2: "Day-to-Day Processes" Characterisation Contradicts Enhanced Monitoring Commitment

In Senate Community Affairs Legislation Committee testimony (9 October 2025), the TGA's Acting Medical Officer, Medicines Regulation Division, said that "the five key themes outlined in that report essentially describe our day-to-day processes. This is our standard sort of process in terms of post market surveillance and pharmacovigilance."[42] National Cabinet had committed to active and comprehensive post-market safety monitoring of COVID-19 vaccines, and the Plan was built to deliver it. Describing enhanced monitoring as business as usual does not readily reconcile with that commitment, or with the Plan's commitment to "enhanced" monitoring as the stated basis on which provisional approval proceeded. The February 2021 Plan explicitly frames its approach as enhanced pharmacovigilance, specifying systematic processes, documented frameworks, and coordinated surveillance mechanisms beyond routine operations. Under the Therapeutic Goods Act 1989, provisionally registered medicines are subject to conditions and enhanced post-market surveillance because they are approved on the basis of more limited, early clinical data than standard registrations, which creates greater uncertainty about their benefit–risk profile.[43]

This characterisation is reinforced by TGA's direct communications pattern documented in Appendix C: when explicitly requested to provide Plan implementation evidence in early 2022, TGA provided identical generic

[39] Therapeutic Goods Administration (2023), *Australian Public Assessment Report for Comirnaty COVID-19 Vaccine (tozinameran), PM-2023-01183-1-2, AusPAR* dated 7 August 2023, submission type: provisional to full registration.
https://www.tga.gov.au/resources/auspar/auspar-comirnaty-covid-19-vaccine
[40] Therapeutic Goods Administration (2023), *Australian Public Assessment Report for Spikevax (elasomeran)*, PM-2022-05374-1-2, AusPAR dated 10 May 2023, submission type: transition from provisional registration to full registration.
https://www.tga.gov.au/resources/auspar/auspar-spikevax-1
[41] Therapeutic Goods Administration (2024), *Submission to the Office of the Australian Information Commissioner, MR22/00538,* 20 September 2024.
[42] Senate Community Affairs Legislation Committee, Estimates, Proof Committee Hansard, 9 October 2025, Dr Daniel Dascombe, Acting Medical Officer, Medicines Regulation Division, TGA.
[43] *TGA 2017 Provisional Approval consultation paper, pp. 6, 11–14; TGA, Provisional registration extension and transition to full registration; Explanatory Memorandum*, Therapeutic Goods Amendment (2017 Measures No. 1) Bill 2017, Sch 1.
https://www.tga.gov.au/sites/default/files/consultation-provisional-approval-pathway-for-prescription-medicines.pdf

pharmacovigilance links across three responses, with FOI 3643 decision confirming no implementation report exists (Appendix C, Table 1).

Characterising this work as "day-to-day processes" suggests one of three scenarios:

- TGA's routine pharmacovigilance processes were considered sufficient (contradicting the enhanced framework commitment)
- Enhanced processes were implemented but not distinguished from routine operations (documentation and governance failure)
- The enhanced framework remained aspirational while routine processes continued (implementation failure)

All three scenarios represent a failure to demonstrate implementation of the Plan as specified. Moreover, if enhanced monitoring activities were indistinguishable from routine "day-to-day" operations, TGA cannot demonstrate that the enhanced monitoring it committed to under the plan was delivered.

This characterisation is reinforced by the documentation pattern: routine activities (weekly reports, DAEN database) are well documented, while enhanced elements (dedicated teams, coordination frameworks, systematic integration) cannot be evidenced.

Finding 3: Critical Control Deficiencies in Signal Detection and Governance

Objective 2 (signal detection) shows 0 per cent fully documented implementation of the Plan's specified enhancements. While signal investigations clearly occurred (148/150 signals investigated and 57 actions reported in Senate Community Affairs Legislation Committee testimony and TGA publications, October 2025), no finished COVID-19-specific signal-investigation protocols or systematic oversight mechanisms linked to the Plan were identified, and the sequential and routine review processes in FOI 5082 are not accompanied by MaxSPRT or routine review outputs.

Across OAIC-directed TRIM searches using eight search terms, TGA again identified no AusVaxSafety–TGA coordination protocols, no multi-source integration procedures, and no Plan-specific implementation guidelines for signal detection.

Under international pharmacovigilance standards referenced by TGA (including ICH E2E) and widely recognised best practice frameworks (such as CIOMS guidance), signal management is expected to generate traceable audit trails from detection through assessment to regulatory decision. The absence of such documentation prevents independent verification that the Plan's enhanced monitoring framework was systematically applied and prevents determination of whether the enhanced post-market monitoring and compliance arrangements associated with the provisional approval pathway were implemented as intended.

Finding 4: Signal Investigation and Regulatory Action Transparency Gap

TGA reports 57 regulatory actions resulting from 150 signal investigations. However, no public or FOI-accessible documentation links specific signals to specific actions, describes decision-making criteria applied, or explains the disposition of 93 signals that did not result in documented regulatory action.

Across approximately 75 million doses administered, TGA investigated one signal per 507,000 doses.[44] Whether this detection rate reflects appropriate sensitivity, under-detection, or performance comparable to international benchmarks cannot be assessed without access to investigation protocols, causality assessment frameworks, and prioritisation criteria that international pharmacovigilance standards require.

International standards emphasise comprehensive documentation and audit trails not merely to show that signals were investigated, but to enable independent assessment of whether the pharmacovigilance system functioned with adequate sensitivity and rigour. The figures of 150 signals and 57 actions describe outputs only; the missing documentation concerns the processes that produced those outputs and whether they met enhanced monitoring commitments.

Systematic documentary searches (Appendix B, Section B.5.1) found TGA's 2022-23 and 2023-24 Performance Report contains no references to the COVID-19 Vaccine Safety Monitoring Plan, no output tracking, and no performance metrics—supporting the finding that implementation was not tracked at institutional level. This omission means that implementation and verification of these commitments are invisible in the agency’s primary accountability documents.

[44] https://www.health.gov.au/resources/publications/covid-19-vaccination-vaccination-data-1-october-2025?language=en

Box 3: AusVaxSafety: Active Surveillance Without Documented Signal Investigation

AusVaxSafety was established in 2014 and is led by the National Centre for Immunisation Research and Surveillance (NCIRS). The AusVaxSafety programme exemplifies the documentation gaps identified across Plan implementation: it operated active SMS-based surveillance throughout the COVID-19 vaccine rollout and published regular national safety summaries, yet no TGA documentation could be located showing how AusVaxSafety data were systematically integrated into TGA's signal-detection or decision-making processes. The February 2021 Plan committed to "collaborate with AusVaxSafety to receive active surveillance information and coordinate safety signal detection and investigation activities".

Despite OAIC-directed searches and multiple FOI processes specifically requesting implementation documents, TGA reported no coordination protocols, integration frameworks, workflows or audit trails could be located. TGA's September 2024 search acknowledged AusVaxSafety within scope but omitted it as a search term and could not locate publicly accessible partner organisation reports (OAIC Submission MR22/00538, Tables 3-4, pp.18-21).

The documentation gap is evidenced by unintegrated surveillance at scale:

- AusVaxSafety: approximately 6.8 million surveys completed (April 2025), documenting 3 million self-reported adverse events (43.5% of respondents) and 62,000 reported medical visits (0.9%)
- TGA: 150 signals investigated, 57 regulatory actions documented
- No documented linkage between systems

Without coordination protocols, independent parties cannot verify:

- How population-level adverse event data informed TGA signal prioritisation
- Whether 3 million adverse events from active surveillance were reconciled with passive surveillance (DAEN)
- What criteria determined when AusVaxSafety findings would trigger TGA investigations
- How discrepancies between surveillance systems were resolved

This assessment found no documentation demonstrating systematic integration of AusVaxSafety data into TGA's signal detection, cumulative reviews, or regulatory decisions (Appendix B, Section B.5.3; Appendix C, Objective 5 collaboration claims lacking audit trails). This is consistent with 0% implementation for the cross-cutting data integration category.

International pharmacovigilance standards (ICH E2E, CIOMS VIII) require documented audit trails from signal detection through assessment to regulatory

action to enable verification that integration occurred systematically rather than ad hoc.

Documented coordination protocols are the mechanism by which integration can be verified. Without them, independent parties cannot verify whether AusVaxSafety's 62,000 medical-attention reports and 3 million self-reported adverse events were systematically considered in TGA's signal prioritisation, or whether the two surveillance systems operated in parallel.

Finding 5: Governance and Performance Measurement Control Deficiency

FOI processes, OAIC-directed searches, and systematic documentary review identified no governance oversight or performance measurement documentation for Plan implementation (Appendix C, FOI 3643 decision; Appendix B, Section B.5.1).

The FOI 3643 decision found no implementation report, and in the Senate the TGA said the Plan's themes "essentially describe our day-to-day processes", which indicates that implementation was not systematically tracked against the plan's objectives. This assessment identified no documented governance oversight or performance measurement for Plan implementation (Appendix C, FOI 3643 decision; Appendix C, Section C.5.1) which appears inconsistent with the obligation in PGPA Act 2013 s 37 for the accountable authority of a Commonwealth entity to cause records to be kept that properly record and explain the entity's performance in achieving its purposes, and with ISO 9001:2015 and ISO 31000:2018 principles concerning systematic monitoring, measurement, risk management and documented evaluation of performance against defined objectives.

For a plan tied to provisional approval, the absence of implementation governance or performance tracking represents a fundamental control deficiency preventing TGA from demonstrating whether enhanced post-market monitoring was delivered as specified. The absence of documented governance oversight raises questions about how expert advisory structures related to Plan implementation.

This documentary deficiency is compounded by the Plan's design: although its 17 strategy items were numbered, the multiple outputs associated with them were not separately numbered or incorporated into a systematic framework for tracking and measuring implementation. This limited the capacity to systematically assess and subsequently demonstrate implementation against the Plan's commitments.[45]

[45] For the detailed methodological assessment of the Plan's structure, auditability and the conservative construction of the 19-output audit framework, see the ISO 19011 Conformity Assessment Checklist: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/analysis/ISO-19011-conformity-assessment-checklist.md

Expert Committees and Governance Framework

The existence of expert advisory committees (ATAGI, ACV, AEFI-CAN) that provided advice on COVID-19 vaccine safety is well-documented through ANAO audit findings and publicly available meeting statements. However, expert advisory structures do not substitute for documented Plan implementation, governance oversight, or performance measurement.

Advisory committees address *what decisions should be made* based on clinical and scientific evidence. Plan governance addresses *whether the enhanced monitoring framework was systematically implemented, operated, and measured* as specified. These are distinct accountability questions:

- Advisory function: Did expert committees review safety data and provide appropriate clinical recommendations? (Evidence: ACV meeting statements exist)
- Implementation function: Was the February 2021 Plan's enhanced monitoring framework—including multi-source integration, systematic coordination, and performance tracking—implemented as committed? (Evidence: no documentation located)

No committee terms of reference, standing agendas, or decision records were identified that demonstrate how ATAGI, ACV, or AEFI-CAN systematically fed COVID-19 vaccine safety advice into TGA's signal-management workflows, regulatory actions, or Plan-level implementation tracking. Available documentation shows committees existed and met, but not how their deliberations linked to Plan governance, AusVaxSafety coordination (Strategy 2.4), or the 17 explicitly numbered implementation strategies.

For provisionally approved medicines where enhanced monitoring is treated as a compensating safeguard in this audit, the question is not whether expert advice was available, but whether the enhanced monitoring framework specified in the Plan was systematically implemented with documented processes, integration protocols, and performance measurement—requirements that expert committees do not fulfil.

Finding 6: Insufficient Evidence of Process Design and Operating Effectiveness

The entity is unable to provide documented evidence that processes were designed, implemented, and operated in accordance with the February 2021 Safety Plan. Operational activity alone does not constitute evidence of systematic, enhanced monitoring as specified in Plan documentation.

While TGA clearly conducted operational activities—processing adverse event reports, investigating signals, publishing weekly safety reports—these activities do not demonstrate that the enhanced monitoring framework specified in the Plan was

systematically designed, documented, and governed. International standards (ISO 19011:2018, ICH E2E, CIOMS) distinguish between operational outputs and systematic process implementation:

- Operational output: 150 signals investigated (activity occurred)
- Systematic process: Documented investigation protocols, prioritisation criteria, causality frameworks (design and governance not evidenced)

The absence of process documentation means independent parties cannot verify whether enhanced monitoring occurred through systematic frameworks or through ad hoc application of routine pharmacovigilance processes. For provisionally approved products where enhanced monitoring is the compensating safeguard in this audit, this distinction is fundamental.

The Audit Trail Gap

International pharmacovigilance guidance adopted or referenced by TGA (including ICH E2E) and internationally recognised best practice (including CIOMS and EMA GVP Module IX) establish principles for pharmacovigilance planning, signal management and documented traceability. EMA GVP Module IX specifically provides for an audit trail of signal-management activities, allowing traceability and process control across signal-management steps, including analyses, decisions and their rationale. This documentation supports independent verification, regulatory oversight and continuous improvement of safety monitoring systems.[46]

For provisionally approved products where enhanced monitoring is treated as a safeguard compensating for reduced pre-market evidence, such documentation is not optional; it is fundamental to demonstrating that the framework operated as designed. The inability to produce these audit trails four years after Plan publication represents a systemic control deficiency that prevents verification of whether the enhanced monitoring framework was implemented as designed.[47]

Moreover, the audit trail gap extends beyond signal detection to regulatory action: while 57 regulatory actions are reported, no documentation links specific signals to specific regulatory responses, no decision criteria are documented, and the disposition of investigated signals that did not result in regulatory action remains unexplained. This prevents verification that the enhanced monitoring framework produced systematic, evidence-based regulatory decisions.

[46] This traceability requirement is also discussed in the independent academic literature: Malikova MA (2020), "Practical applications of regulatory requirements for signal detection and communications in pharmacovigilance," *Therapeutic Advances in Drug Safety*, 11:2042098620909614, examines how regulatory signal-detection and communication requirements are applied in practice, and identifies documented traceability from signal to regulatory decision as a core expectation of a functioning pharmacovigilance system

[47] https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf

As shown by the OAIC-directed MR22/00538 TRIM searches, no AusVaxSafety–TGA coordination protocols, multi-source integration procedures or Plan-specific implementation guidelines were identified, and no documentation links AusVaxSafety data to TGA's 150 signals, 57 regulatory actions or the 93 signals closed with no further action.

Independent validation of identified gaps

A peer-reviewed study published in *Vaccine* (September 2021) provides independent corroboration of this audit's findings. Phillips et al. interviewed 17 Australian vaccine safety experts in July–October 2020—before the COVID-19 vaccine rollout—documenting significant system limitations that align precisely with gaps identified in this audit.[48]

Key expert findings included:

- Data linkage was described as a "big hole" and a "huge omission", despite being "technically quite feasible"
- AusVaxSafety-Active and TGA's passive system (AEMS) were characterised as "parallel systems" where notifications and signals could "fall through gaps"
- COVID-19 vaccine safety monitoring would require "significant enhancement", including "systematic approaches to population-level active surveillance" through data linkage

These pre-rollout warnings directly align with TGA Senate testimony (9 October 2025) that the Plan's themes "essentially describe our day-to-day processes", indicating no systematic tracking against the Safety Plan. OAIC-directed searches found no documentation showing expert-identified gaps in linkage, integration, or population-level active surveillance were addressed.

Phillips et al. concluded the COVID-19 immunisation programme represented an opportunity to strengthen Australia's vaccine pharmacovigilance system by improving integration between active and passive surveillance, developing population-level active surveillance through linked data, and better capturing later-onset AEFI.

What happened: The Safety Plan was published (February 2021) but cannot be verified as systematically implemented. On the available record, the opportunity to strengthen the system and capture later-onset AEFI was not realised.

[48] Phillips A, et al. (2021). *Vaccine*, 39(40), 5968-5981. doi:10.1016/j.vaccine.2021.07.059

Critical Unanswered Questions - AusVaxSafety Coordination:

- How were AusVaxSafety's 6.8+ million safety surveys analysed and integrated with TGA's signal detection processes?
- What protocols governed coordination between active surveillance (AusVaxSafety) and passive surveillance (DAEN) systems?
- How did approximately 3 million adverse events reported through AusVaxSafety relate to the 150 signals TGA investigated?
- What threshold determined when AusVaxSafety findings (including 62,000 individuals seeking medical attention) would trigger TGA signal investigations?
- How were discrepancies between surveillance systems reconciled (e.g., if AusVaxSafety identified patterns not visible in DAEN, or vice versa)?
- Why could TGA's September 2024 search not locate AusVaxSafety's publicly accessible website and data despite acknowledging the collaboration within scope?
- What distinguished COVID-19 vaccine safety monitoring from TGA's routine pharmacovigilance operations?
- If enhanced monitoring was conducted through "day-to-day processes," how can TGA demonstrate that the enhanced surveillance associated with provisional approval was delivered?
- Were additional resources, dedicated teams, or systematic processes allocated beyond routine operations?
- How did TGA verify that routine "day-to-day" processes provided the enhanced oversight associated with provisional approval?

These questions cannot be answered without the documented protocols, coordination frameworks, and decision-making criteria that international pharmacovigilance standards require but which remain undocumented.

3. Detailed Plan Implementation Assessment

Table 1: Implementation Status Assessment by Output[49]

Plan Objective & Specific Output[50]	What Was Promised (From February 2021 Plan)	Implementation Status & Evidence (From Audit & FOI Findings)
STRATEGY 1: Enhanced reporting of Adverse Events Following Immunisation		**Status: 40% Implemented**
1.1 Enhanced reporting of AEFI	**Information on how to report published; communication activities to raise awareness.**	✓ Fully Implemented. Evidenced by public website content, advertising, and weekly safety reports.
1.2 Enhanced AEFI reporting forms	Updated TGA AEFI forms to capture COVID-19 specific info and need for follow-up on AESI.	✓ Fully Implemented. Forms were updated and are publicly available.
1.3 Enhanced AEFI report sharing	Criteria and processes for timely sharing of reports with state/territory jurisdictions.	◐ Partially Implemented. Automated data flows likely existed, but no documented criteria/processes for sharing were identified in TGA searches.
1.4 Enhanced AEFI data capabilities	TGA database and team prepared for increased data entry and management.	◐ Partially Implemented. Activity occurred (processing 150+ weekly reports), but no documentation of enhanced preparation (e.g., new system protocols) was found.
1.5 Enhanced AEFI report escalation	Criteria for COVID-19 vaccine report escalation and clinical review incorporated into procedures.	◐ Partially Implemented Clinical reviews happened, but the audit found no documented, COVID-specific escalation criteria or updated SOPs.
STRATEGY 2: Enhanced safety signal detection and investigation		**Status: 0% Fully implemented**
2.1 Understanding safety profiles	Establish expected AEFI rates, background rates, and a list of AESI.	◐ Partially Implemented. A list of AESI was used, but no documentation of established expected rates or calculated background rates was found.

[49] Ratings describe the documentary record, not operational outcomes. They rest on the audit evidence available under ISO 19011:2018 (clause 3.9). ✓ means verifiable records of delivery; ◐ means process documentation exists with limited or no records of results; ✗ means no relevant evidence was located. No rating establishes that an output was or was not performed. For the full output-level assessment, evidence sources and classification basis underlying Table 1, see the *Output Assessment Evidence Matrix*. https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/analysis/output-assessment-evidence-matrix.xlsx

[50] The assessment also considers cross-cutting commitments relating to data integration and evidence generation identified within the Plan's strategies and supporting narrative. These are not additional objectives of the Plan, but analytical categories used to capture relevant commitments that extend across its five stated objectives, principally under Objectives 1, 2 and 5.

Plan Objective & Specific Output[50]	What Was Promised (From February 2021 Plan)	Implementation Status & Evidence (From Audit & FOI Findings)
2.2 Enhanced capacity and capability for investigating individual COVID-19 AEFI reports	Ensure TGA has capacity to undertake appropriate investigation of COVID-19 AEFI reports	◐ Partially Implemented. FOI 5082 contains general vaccine work instructions for signal investigation (VSIG, January 2019; Vaccine STRS, with a COVID-19 referral step) and a COVID-19 signal detection and investigation framework (working draft). No COVID-specific investigation methodology, record of enhanced capacity, or account of how the 150 signals were investigated was located.
2.3 Enhanced cumulative data reviews	Refined processes/statistical methods for signal detection; subpopulation analysis.	◐ Partially Implemented. FOI 5082 contains a 2021 COVID-19 sequential monitoring plan (MaxSPRT, with sex and age subgroup analyses), a weekly cumulative review process, advice citing Plan Strategy 2.3, and the outcomes of an August 2021 DPAR and chi-squared comparison. No MaxSPRT or routine cumulative review outputs were located.
2.4 Active surveillance[51]	Collaborate with AusVaxSafety and coordinate safety signal detection.[52]	◐ Partially Implemented. AusVaxSafety infrastructure operated effectively throughout 2021-2025, completing 6,887,520 COVID-19 vaccine safety surveys as at April 2025. Of respondents, 43.5% reported at least one adverse event (approximately 3 million survey responses) and 0.9% reported medical attention (approximately 62,000 responses).

[51] This assessment evaluates documented implementation of the February 2021 Safety Plan's specific outputs. While TGA reported 148 safety signals in Senate Community Affairs Legislation Committee Supplementary Budget Estimates testimony (October 2025), then 150 in writing (QON 559), and 57 regulatory actions during this period, no public documentation links specific signals to specific regulatory actions, explains the decision-making criteria applied, or describes the disposition of investigated signals. The question is whether enhanced monitoring frameworks beyond routine pharmacovigilance were systematically documented as specified in the Plan.

[52] Public TGA documents, FOI releases and OAIC-directed TRIM searches together show extensive routine pharmacovigilance documentation (AEFI workflows, signal-detection methods, regulatory actions) but no documented implementation of the COVID-19 Vaccine Safety Monitoring Plan's enhanced, multi-source integration requirements—particularly Strategy 2.4 on AusVaxSafety coordination.

Plan Objective & Specific Output[50]	What Was Promised (From February 2021 Plan)	Implementation Status & Evidence (From Audit & FOI Findings)
		Infrastructure capability is documented and not disputed. However, no documentation was identified showing systematic coordination of this active surveillance data with TGA's signal detection processes. TGA's September 2024 search acknowledged AusVaxSafety within scope but omitted it as a search term and could not locate partner organisation reports (OAIC Submission MR22/00538). The coordination component of Strategy 2.4's commitment cannot be verified despite demonstrated operational capability.
2.5 Clinical studies and reports	New processes to analyse post-market safety studies and monthly summary reports.	✗ Not Documented. The audit identified no documentation of new processes for analysing these sponsor-submitted reports. FOI 5275 separately found that TGA does not hold the cancer incidence data requested by reference to the Comirnaty AusPAR.[53]
2.6 Environmental scanning	Processes for monitoring COVID-19 vaccine safety alerts from overseas regulators, medical literature, and other sources.	◐ Partially Implemented. FOI 5082 lists scanning sources in a working draft framework and refers to a PSAB Activities Register. No record of scanning results, or of how they were used, was located.
2.7 International safety signals	Mechanisms for sharing COVID-19 safety information through ICMRA.	◐ Partially Implemented. TGA co-chaired ICMRA working group, but no documented processes for systematic information sharing were found.
2.8 Expert advice	Standing COVID-19 expert advisory committee established.	◐ Partially Implemented. Expert committees existed, but no documentation of a standing COVID-19-specific advisory committee was found.

[53] In a separate FOI (5275), a third-party request for monthly incidence data for conditions listed in the Comirnaty AusPAR, TGA stated that the requested documents 'do not exist' because it does not hold 'incidence data' related to cancer. The Safety Monitoring Plan lists 'timely submission of post-market safety studies and monthly safety summaries' as a condition that may be imposed on registration.

Plan Objective & Specific Output[50]	What Was Promised (From February 2021 Plan)	Implementation Status & Evidence (From Audit & FOI Findings)
		Despite expert committees advising on vaccine policy and safety, OAIC-directed searches and FOI processes have not identified documentation that maps these advisory processes to the Plan's enhanced monitoring commitments, or that demonstrates systematic implementation and performance measurement for each objective[54]
STRATEGY 3: Actions in response to safety concerns		
3.1 Regulatory and programmatic action	Use legislative provisions to achieve timely regulatory action in response to safety concerns	◐ Partially Implemented. 57 regulatory actions taken (warnings, label changes); weekly reports document safety responses. No documented framework or protocol for systematic integration of data sources; no documented methodology for reconciling DAEN vs AusVaxSafety.
STRATEGY 4: Clear communication		**Status: 100% Implemented**
4.1 Communications and media	Key messages; regular safety reports on website; enhanced media scanning.	✓ Fully Implemented. Evidenced by 150+ weekly safety reports, published statements, and media engagement.
STRATEGY 5: Collaborations		**Status: 0% Fully implemented**
5.1 National vaccine safety stakeholder collaborations	Coordinate with state/territory health departments, ATAGI, ACV, NCIRS, SAEFVIC, AEFI-CAN on COVID-19 vaccine safety.	◐ Partially Implemented. Meetings occurred, but no documented frameworks for systematic coordination were identified.
5.2 International collaborations	Information sharing with international regulators and WHO on COVID-19 vaccine safety.	◐ Partially Implemented. International engagement occurred, but no documented processes for systematic information sharing were found.

[54] For example, the Advisory Committee on Vaccines provides independent medical and scientific advice to the Minister for Health and the Therapeutic Goods Administration on the safety, quality and efficacy of vaccines supplied in Australia, including pre-market assessment, post-market monitoring and safe use in national immunisation program. https://www.tga.gov.au/safety/safety-monitoring-and-information

PROGRAMME ASSESSMENT: Governance & Performance Measurement		**Status: 0% Implemented**
GOV.1 Implementation Oversight	(Implicit requirement) Oversight, tracking, and review of the Plan's implementation.	✗ Not Documented. No Plan-specific governance documentation was located: no oversight committee records, progress reports or performance metrics against the Plan.
GOV.2 Performance Measurement	(Implicit requirement) Measuring performance against the stated objectives and outputs.	✗ Not Documented. The TGA's statements indicate that implementation was not systematically tracked. FOI 3643; TGA's MR22/00538 submission identified no performance measurement documents.

4. Detailed Observations

4.1 Implementation Documentation and Governance Gap

This audit identified a fundamental gap between the Safety Monitoring Plan's documented commitments and the TGA's documented implementation and governance commitments.

Observation:

- No systematic tracking of Plan implementation identified
- No governance frameworks documented
- No enhanced signal detection protocols located
- No performance measurement mechanisms evidenced
- AusPAR records show no evaluation against the February 2021 COVID-19 Vaccine Safety Monitoring Plan[55] and no documented assessment of whether its enhanced-monitoring commitments were implemented
- Enhanced monitoring was presented as a safeguard compensating for reduced pre-market evidence accompanying provisional approval; the documentary record does not demonstrate that it was implemented as a distinct, "enhanced" arrangement rather than routine operational pharmacovigilance.[56] [57]

[55] Documentation for the provisional-to-full registration decision summarising clinical and pharmacovigilance data
[56] https://www.tga.gov.au/products/medicines/prescription-medicines/application-and-market-authorisation/covid-19-vaccine-information-sponsors-industry/covid-19-vaccines-regulatory-status
[57] https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf

Key Risk Identified: Inability to demonstrate compliance with Plan commitments, regulatory requirements, and provisional approval, creating regulatory and legal exposure.[58]

OAIC-directed sample TRIM searches in September 2024, using eight distinct search terms and document-type classifications (SOPs, protocols, workflows, implementation guidelines), failed to identify any implementation reports, governance frameworks, performance measurement documents, or Plan-specific coordination protocols for the COVID-19 Vaccine Safety Monitoring Plan. This indicates, under statutory direction, that the absence of identified Plan-implementation documentation persisted beyond the original 2022 search.[59]

FOI 25-0166 is significant for the audit's central question. The TGA's estimate that more than 399 documents fell within scope of the original request indicates that documents responsive to such a request can be located within its systems. The revised request, framed by reference to standard document types, was refused. What those documents contain has not been established, because none was processed. The evidentiary position is therefore that access was refused, not that the records were searched and found wanting. In the same decision the TGA recorded its earlier advice that documents relevant to item 1 "would likely provide you with information helping you to understand how the TGA implemented its COVID-19 Vaccine Safety Monitoring Plan" (decision, paragraphs 46 to 48). It refused the request because item 2 would require "subjective interpretation of the relationship to that objective" (paragraphs 5 to 7).

The Advisory Committee on Vaccines publishes detailed meeting statements summarising its advice on specific safety issues, such as transverse myelitis following mRNA COVID-19 vaccines (Meeting 53, June 2025) and herpes zoster ophthalmicus following Shingrix (Meeting 54, October 2025). These statements confirm that expert review of safety data occurred, but they do not provide a traceable implementation framework linking AusVaxSafety data, internal signal processes and regulatory actions back to the COVID-19 Vaccine Safety Monitoring Plan.[60]

4.1.1 Expert Advisory Committees Distinguished from Plan Governance

ANAO's COVID-19 vaccine rollout audit confirms that expert committees including ATAGI, ACV, and SITAG provided clinical and technical advice on vaccine policy, safety assessment, and rollout strategy. These committees are documented through

[58] https://www.tga.gov.au/products/medicines/prescription-medicines/overview/covid-19-vaccine-approval-process
[59] *TGA-OAIC-Submission-MR22-00538-Sept-2024*
[60] https://www.tga.gov.au/resources/publication/meeting-statements/acv-meeting-statement-meeting-53-4-june-2025

published meeting statements, public recommendations, and parliamentary references.

However, the existence of expert advisory committees does not demonstrate implementation of the February 2021 Safety Monitoring Plan's governance requirements. Advisory committees serve a distinct function:

Advisory committees provide: Expert clinical judgment on safety signals and risk-benefit assessment

Plan governance requires: Systematic implementation tracking, performance measurement, documented coordination protocols, and audit trails linking surveillance data to regulatory decisions

The assessment identified no documentation demonstrating:

- How committee advice was operationalised into Plan-specific processes
- Whether committees were briefed on Plan implementation progress
- What coordination mechanisms existed between committee deliberations and Plan outputs
- How committee recommendations linked to the 150 signals, 57 actions, and 93 "no further action" closures

Advisory Committee on Vaccines meeting statements (such as Meeting 53, June 2025, on mRNA vaccines and transverse myelitis) demonstrate expert review occurred, but do not provide audit trails showing how AusVaxSafety's 6.8 million surveys, DAEN reports, and international signals were systematically integrated into committee briefings or how committee advice translated to documented regulatory decisions under the Plan framework.

4.2 Alignment with Provisional Approval Framework

Table 2: Key Framework Requirements vs Documented Implementation

Framework requirement	Source	What is required or promised	Evidence in TGA record set	Audit assessment
Enhanced post-market monitoring as the policy and regulatory safeguard accompanying provisional registration of early-	TGA provisional approval framework[61]	Provisional approvals are time limited and may proceed on preliminary clinical data, with sponsor-specific	The February 2021 COVID-19 Vaccine Safety Monitoring Plan sets out “enhanced” monitoring objectives, but OAIC-directed searches and FOI processes found no	Gap: 'enhanced monitoring' commitment not supported by documented implementation.

[61] *TGA provisional approval framework (2017 consultation paper; provisional registration extension guidance; EM 2017 Measures No. 1).*

Framework requirement	Source	What is required or promised	Evidence in TGA record set	Audit assessment
data COVID-19 vaccines		conditions and further evidence requirements managing residual uncertainty. For COVID-19 vaccines, this framework was accompanied by a policy commitment to enhanced post-market safety monitoring.[62]	governance frameworks, implementation tracking or performance measurement demonstrating these enhanced arrangements.[63]	
Verification of sponsor obligations before extension or transition, and documentary assurance of TGA enhanced monitoring	TGA provisional approval framework; Provisional registration extension and transition to full registration guidance	Sponsors must provide data demonstrating fulfilment of their registration conditions, and TGA assesses that evidence before granting extensions or full registration. TGA guidance describes the evidence expected at transition; the Act does not establish a separate requirement to certify that the Plan's enhanced-monitoring outputs were implemented.	For Comirnaty and other COVID-19 vaccines, AusPARs recording provisional-to-full or extension decisions summarise clinical data, RMP and the sponsor's conditions, but contain no evaluation against the February 2021 Safety Monitoring Plan or the TGA's own enhanced monitoring outputs.	No documentation identified verifying the TGA's own enhanced monitoring outputs under the Plan before full registration. The sponsor's registration conditions, assessed in the AusPAR, are a separate layer.[64]
Pharmacovigilance Plan specifying safety issues, planned studies,	ICH E2E Pharmacovigil-	A written Safety Specification and Pharmacovigilan	The Safety Monitoring Plan exists, but no implementation reports, study tracking	Gap: plan exists; systematic implementation and tracking

[62] *TGA COVID-19 Vaccine Safety Monitoring Plan, Feb 2021.*
[63] *TGA–OAIC submission MR22/00538* (Sept 2024 TRIM search description).
[64] https://www.tga.gov.au/sites/default/files/provisional-registration-extension-and-transition-full-registration.pdf

Framework requirement	Source	What is required or promised	Evidence in TGA record set	Audit assessment
milestones and reporting	ance Planning[65]	ce Plan should identify important risks, specify post authorisation studies, and define milestones and reporting.	documents or milestone records linked to its 17 numbered strategy items and associated outputs were identified.	documentation absent.
Documented signal management from detection to decision and action	CIOMS/GVP signal management guidance[66] and EMA GVP Module IX signal-management audit-trail principles.	Signal management steps (detection, validation, analysis, prioritisation, decision, action, communication) should be documented and traceable.[67]	TGA officials reported investigation of 150 safety signals and 57 resultant actions at Senate Budget Estimates, but no public documentation links specific COVID-19 signals to specific regulatory decisions or describes the criteria applied.[68]	Gap: traceability requirement not met; no end-to-end audit trails.
Pharmacovigilance (PV) record keeping ensuring retrievability and traceability of decisions	TGA PV responsibilities and related guidance	PV systems must ensure records are maintained so that safety concerns, evaluations and resulting actions are fully retrievable and traceable over time.[69]	OAIC-directed TRIM searches (eight search terms; 2,218+ pages estimated) did not locate implementation reports, Plan specific SOPs, or performance measurement documents for the Safety Monitoring Plan.	Gap: PV record keeping obligations not evidenced for the Plan's enhanced activities.
Systematic use of active surveillance and multi-source data integration	Safety Monitoring Plan and PV standards	Active surveillance systems (e.g. AusVaxSafety) and multiple	AusVaxSafety conducted more than 6.8 million COVID-19 vaccine safety surveys and produced detailed	Gap: operational capability present, but integration and coordination

[65] International Conference on Harmonisation of Technical Requirements for Registration of Pharmaceuticals for Human Use (ICH). *ICH Topic E2E: Pharmacovigilance Planning*. CPMP/ICH/5716/03. November 2004; effective June 2005. Available at: https://www.tga.gov.au/sites/default/files/2024-12/ich571603enplanning_pharmacovigilance_activities.pdf

[66] CIOMS, *Signal Detection in Pharmacovigilance; EMA, GVP Module IX: Signal management.*

[67] *European Medicines Agency (2017). Guideline on good pharmacovigilance practices (GVP) – Module IX: Signal management (Rev. 1). EMA/827661/2011 Rev. 1.*

[68] Senate Community Affairs Legislation Committee, Supplementary Budget Estimates Hansard, 9 October 2025, [page 32]. TGA subsequently updated the figure to 150 signals in its answer to Question on Notice 559 (SQ25-001584), of which 93 have no publicly documented disposition; the testimony figure is cited here.

[69] TGA, *Pharmacovigilance responsibilities of medicine sponsors* (and related PV guidance).

Framework requirement	Source	What is required or promised	Evidence in TGA record set	Audit assessment
		data sources should feed into an integrated signal detection and assessment framework, with roles, processes and data flows defined.	outputs, but no documented framework showing systematic integration of these data into TGA signal management processes was identified.[70]	requirements not documented.

The February 2021 COVID-19 Vaccine Safety Monitoring Plan was intended to operationalise the enhanced post-market monitoring expected for provisionally approved early-data COVID-19 vaccines under the Cabinet-endorsed policy, and to align with the international pharmacovigilance standards adopted by TGA. Table 2 compares framework requirements with documentation identified through OAIC-directed TRIM searches and FOI processes.[71] Across the provisional-approval framework, ICH E2E pharmacovigilance planning, CIOMS/GVP signal-management guidance and TGA's own pharmacovigilance obligations, the common expectation is a traceable, documented chain from plan to signal to regulatory decision. The audit found routine pharmacovigilance activity but no corresponding documentation demonstrating systematic implementation of the Plan's enhanced requirements.

4.2.1 Post-Transition Monitoring Framework

Pfizer Comirnaty and Moderna Spikevax transitioned from provisional to full registration in mid-2023 (July 2023 and April 2023 respectively), after 68.4 million doses had been administered under provisional approval. This transition raises a critical question: what happened to the enhanced monitoring commitments specified in the February 2021 Plan?

Three scenarios are possible:

- **Enhanced monitoring was discontinued** following transition to full registration, on the basis that the Plan's commitments were time-limited to the provisional approval period.
- **Enhanced monitoring continued under a different framework**, with new governance arrangements, documentation systems, or integration protocols established for the post-provisional period.

[70] NCIRS/AusVaxSafety COVID-19 vaccine safety surveillance outputs
[71] Core framework documents*: TGA Consultation: Provisional Approval pathway for prescription medicines (2017); TGA Provisional registration extension and transition to full registration; Explanatory Memorandum, Therapeutic Goods Amendment (2017 Measures No. 1) Bill 2017; TGA COVID-19 Vaccine Safety Monitoring Plan (Feb 2021); ICH E2E Pharmacovigilance Planning*; CIOMS *Signal Detection in Pharmacovigilance; EMA GVP Module IX: Signal management; TGA Pharmacovigilance responsibilities of medicine sponsors.*

- **Enhanced monitoring was never systematically implemented**, meaning the transition from provisional to full registration changed nothing in operational practice—TGA's characterisation of COVID-19 vaccine safety work as "day-to-day processes" remained accurate throughout.

The available evidence cannot distinguish between these scenarios. No public documentation describes:

- Whether the Plan's enhanced monitoring framework was formally discontinued
- What governance arrangements replaced it (if any)
- How the transition affected coordination with AusVaxSafety and other surveillance systems
- Whether performance measurement or implementation tracking was conducted for the provisional approval period before transition

If Scenario 1 is correct, TGA should be able to produce an end-of-Plan evaluation or transition report documenting what was implemented during the provisional period and what monitoring arrangements succeeded it. If Scenario 2 is correct, post-transition monitoring frameworks should be documented and publicly available. If Scenario 3 is correct—that enhanced monitoring commitments were never systematically implemented beyond business-as-usual processes—this has significant implications for the integrity of Australia's provisional approval framework and public assurances given during the rollout.

The inability to determine which scenario occurred represents a fundamental accountability failure. For 68.4 million doses administered under provisional approval, Australia's largest deployment of medicines under this regulatory pathway, the available documentary record does not establish what enhanced monitoring arrangements were in place, when they operated, how the enhanced monitoring arrangements related to the provisional-approval framework, or whether enhanced monitoring continued post-transition. This makes retrospective documentary verification of the enhanced monitoring framework impossible.

4.3 Provisional Approval Lifecycle: Verification Gap

Legislative Framework for Provisional Registration

The Therapeutic Goods Act 1989 (Cth) establishes a structured pathway for the provisional registration of medicines, comprising provisional determination, provisional registration, conditions of registration, defined registration periods, extension mechanisms and transition to full registration.

Provisional determination (ss 22C–22D). An applicant applies for a provisional determination under s 22C. Under s 22D, the Secretary decides whether to make or refuse the provisional determination and may make the determination where the

statutory criteria are satisfied. Sections 22E and 22F separately address the period during which a provisional determination is in force and its revocation. Accordingly, ss 22C–22D describe the application and decision stages, while ss 22C–22F comprise the broader provisional-determination framework.

Provisional registration (s 23AA). Where a provisional determination is in force, s 23AA provides for an application under s 23 to be treated as an application for provisional registration in the circumstances specified by the Act. Provisional determination is therefore a prerequisite to provisional registration through the ordinary pathway, but does not itself guarantee registration. The TGA similarly describes provisional registration as a two-part process involving determination and registration.

Evaluation and registration decision (s 25). For an application for provisional registration, s 25(1)(d) requires the Secretary to evaluate the medicine having regard to the statutory provisional-registration criteria, including the preliminary clinical data concerning safety and efficacy, quality and the applicant's plan for submission of comprehensive clinical data. Section 25(3) provides for the Secretary's decision to register or not register the goods.

Conditions of provisional registration (s 28(2A)(aa)). Section 28(2A)(aa) permits conditions to be specified for provisional registration that differ from those applying to other registration pathways. The particular conditions imposed on a provisionally registered medicine may include requirements concerning confirmatory studies, pharmacovigilance and other post-market activities. TGA guidance confirms that confirmatory studies forming part of a Risk Management Plan/Additional Safety Activities (RMP/ASA) may be imposed as conditions of provisional registration, with milestones and completion dates specified in those conditions. The statutory power to impose conditions should therefore be distinguished from the particular requirements contained in individual conditions of registration.

Time-limited provisional registration and extension (s 29). Provisional registration ordinarily commences for a period of two years under s 29(3). An application may be made for an extension under s 29(4), and the Secretary decides whether to grant the extension under s 29(6). Each extension cannot exceed two years under s 29(7)(b), and no more than two extensions are available under the ordinary extension mechanism. The TGA describes this as an initial two-year period followed by up to two extensions of up to two years each, producing a possible maximum of six years under the ordinary provisional-registration pathway.
In deciding an extension application, the Secretary must consider whether satisfied with the applicant's plan to submit comprehensive clinical data on the safety and efficacy of the medicine before the end of the six-year period, together with any other matters the Secretary considers relevant. The TGA's extension process also assesses progress against the agreed clinical study plan, including milestones and confirmatory studies.

Continuation while full registration is considered (s 29(9)–(10)). Section 29 provides a separate mechanism for dealing with provisional registration where an application for full registration has been made. Under s 29(9), the Secretary may make decisions concerning the ending or continuation of provisional registration in connection with the full-registration decision. Section 29(10) provides for the provisional registration period to continue while the full-registration application is being considered. TGA guidance confirms that provisional registration may therefore continue beyond the ordinary six-year period while a full-registration application is being evaluated.

Transition to full registration (ss 23 and 25). Transition to full registration requires a formal application under s 23. TGA guidance states that sponsors should not apply for full registration until they have completed the obligations outlined for the provisional-registration period and complete confirmatory safety and efficacy data are available. The application follows the standard registration process, with the Secretary making the registration decision under s 25. TGA guidance also requires evidence of having met relevant RMP obligations, including a chronological listing of RMP obligations during the provisional period and their completion status.
No express statutory Plan-verification requirement. The Therapeutic Goods Act 1989 establishes statutory mechanisms for provisional registration, conditions, extension and transition to full registration. It does not expressly establish, as part of the s 29 extension or s 29(9)–(10) continuation mechanisms, a separate requirement that the Secretary verify that all activities in the February 2021 *COVID-19 Vaccine Safety Monitoring Plan* have been implemented. Nor does the Act expressly make implementation of that Plan a statutory condition of provisional registration merely because the Plan existed. The audit therefore does not treat the Plan itself as a statutory instrument or attribute its commitments to the Act without a separate statutory basis.

Sponsor obligations and TGA monitoring are distinct. Statutory and regulatory obligations imposed on sponsors, including conditions of registration and applicable pharmacovigilance requirements, are distinct from the TGA's own monitoring functions and from the February 2021 Safety Monitoring Plan. TGA guidance states that sponsors of provisionally registered medicines must comply with relevant post-market requirements and that confirmatory studies and other obligations may form conditions of provisional registration. These sponsor obligations should not be conflated with the Plan's 17 operational outputs.

Verification and documentation gap

The audit therefore tests a narrower question: whether implementation of the February 2021 Safety Monitoring Plan, including its 17 operational outputs, can be independently demonstrated from the available documentary record. The absence of an express statutory requirement for a separate Plan-verification process does not establish that monitoring did not occur. It means that any conclusion about

implementation must be based on documentary evidence of the relevant activities, decisions, oversight and performance assessment rather than inferred from the existence of the statutory provisional-registration pathway.

Sample searches conducted in September 2024 under Office of the Australian Information Commissioner direction (OAIC reference MR22/00538) used eight Plan-aligned search terms in TRIM. According to the audit record, these searches located no documentation identified as verifying the Plan's enhanced-monitoring outputs before Comirnaty or Spikevax transitioned to full registration, no documented assessment against the February 2021 Safety Monitoring Plan, and no documented decision rationale linking the transition decisions to implementation or assurance of the Plan's enhanced-monitoring commitments.

In addition, the audit record documents a later FOI process concerning FOI 25-0166, in which the TGA estimated more than 399 documents comprising 3,926 pages in scope of the original request and declined to process the revised request on practical-refusal grounds under s 24(1)(b) of the *Freedom of Information Act 1982* (Cth). This material is treated as evidence concerning the availability and retrievability of potentially relevant records, not as evidence that the underlying monitoring activities did or did not occur.

Australian Public Assessment Reports

Australian Public Assessment Reports (AusPARs) are examined as documentary evidence of the regulatory assessment and transition process, not as a basis for reassessing the scientific or clinical merits of the underlying regulatory decisions. The Comirnaty AusPAR records a transition-from-provisional-to-full-registration decision on 13 July 2023 and ARTG entry on 14 July 2023. The Spikevax AusPAR records a transition-from-provisional-to-full-registration decision on 21 April 2023 and ARTG entry on 24 April 2023.

Both reports contain material concerning the sponsor's regulatory obligations and conditions. Neither identifies an assessment of implementation of the February 2021 *COVID-19 Vaccine Safety Monitoring Plan* or its 17 operational outputs. The sponsor's registration conditions are a separate regulatory instrument and remain outside this audit's assessment of the Plan. The AusPARs are therefore used only to establish that the Plan-level enhanced-monitoring framework is not identified in the principal public assessment records documenting the transition decisions.

Implications for Provisional Approval Framework Integrity

The absence of documented verification creates three critical accountability gaps:

Documentary Assurance Gap: The provisional registration pathway permits earlier access while operating within a framework for additional evidence generation and risk management during the provisional period. Available documentation does not

demonstrate a complete audit trail verifying implementation of the Plan's enhanced-monitoring activities and their consideration in decisions supporting transition of 68.4 million provisionally registered doses to full registration.

Framework Integrity Compromised: If enhanced monitoring activities cannot be distinguished from routine pharmacovigilance processes (as indicated by Senate testimony in which a senior TGA official characterised monitoring as “day-to-day processes”)[72], the transparency and auditability of the provisional approval framework are compromised. Without documented evidence demonstrating how enhanced monitoring commitments were implemented, assessed, and linked to regulatory decisions, the framework's reliance on reduced pre-market evidence requirements cannot be independently verified.

Future Emergency Preparedness: These documentation and verification gaps may recur in future emergencies unless provisional approval frameworks are strengthened through mandatory verification records, clearer accountability mechanisms, and transparent reporting of monitoring activities, regulatory assessments, and decision pathways.

Australia's COVID-19 vaccine rollout represents the nation's largest deployment of provisionally approved medicines. Between February 2021 and July 2023, 68.4 million doses (91.2% of the total rollout) were administered while key vaccines remained under provisional approval.[73] This scale makes the documented implementation of the Plan's enhanced monitoring commitments across the provisional period a significant accountability question.

4.4 Stakeholder Communication and Assurance

Observation: By late July 2023, 68.4 million COVID-19 vaccine doses had been administered in Australia since roll-out commencement, the vast majority while primary vaccines were provisionally approved.[74] [75]

- Public communications stated rigorous and enhanced safety monitoring for provisionally approved vaccines, consistent with the COVID-19 vaccine approval framework.
- Assurances could not be verified through documentation review.

[72] Dr Daniel Dascombe, Acting Medical Officer, Medicines Regulation Division, TGA: “There would be some difficulty in doing that. As I said at the outset of my response, the five key themes outlined in that report essentially describe our day-to-day processes. This is our standard sort of process in terms of post market surveillance and pharmacovigilance.”
[73] Appendix D, Dose Count Summary; Senate Community Affairs Legislation Committee testimony
[74] https://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-27-07-2023
[75] https://www.health.gov.au/resources/publications/covid-19-vaccination-vaccination-data-10-october-2024?language=en

Key Risk Identified: Reputational damage and erosion of public trust, particularly for future emergency and provisional approvals, with potential liability exposure.[76]

4.5 Active Surveillance Infrastructure Without Documented Integration

Context

AusVaxSafety is Australia's national active surveillance system operated by NCIRS. The February 2021 TGA COVID-19 Vaccine Safety Monitoring Plan explicitly committed to "collaborate with AusVaxSafety to receive active surveillance information and coordinate safety signal detection and investigation activities" (Strategy 2.4, p.9).[77]

Operational performance:

- 6,887,520 COVID-19 vaccine safety surveys completed as at 14 April 2025.
- 43.5% reported at least one adverse event (approximately 3 million survey responses).
- 0.9% reported seeking medical attention (approximately 62,000 responses).
- Regular public reporting maintained throughout the roll-out period.

This demonstrates high-volume, systematic collection of population-level safety data from a large, nationally representative sample.

Documentation gap:

Despite 6.8 million active surveillance surveys documenting 62,000 medical attention reports, TGA produced no coordination protocols, integration frameworks, or audit trails showing how this data informed its 150 signal investigations or 57 regulatory actions. This prevents verification of systematic implementation of Strategy 2.4's explicit coordination commitment.

Specific documented gaps include:[78]

- No documented protocols for integrating AusVaxSafety data with TGA signal detection processes.
- No documented coordination frameworks between active (AusVaxSafety) and passive (DAEN) surveillance systems.
- No documented decision criteria for when AusVaxSafety findings would trigger TGA signal investigations.

[76] https://pmc.ncbi.nlm.nih.gov/articles/PMC9672672/
[77] https://ncirs.org.au/sites/default/files/2025-10/2023%20AEMS%20Annual%20Report.pdf
[78] Therapeutic Goods Administration. (2024, September). *Submission to Office of the Australian Information Commissioner - Review MR22/00538*. Tables 3-4 (pp.18-21).

- TGA's September 2024 OAIC-directed search acknowledged AusVaxSafety within scope but omitted it as a search term and could not locate publicly available AusVaxSafety reports (OAIC MR22/00538, Tables 3-4, pp.18-21).
- No traceable linkages between AusVaxSafety's approximately 3 million adverse-event reports and TGA's investigation of 150 safety signals over the corresponding period.
- No documentation explaining how AusVaxSafety's medical-attention rate informed TGA regulatory actions.

Implications

The absence of documented coordination between AusVaxSafety (6.8+ million surveys) and TGA signal investigation processes means the assessment cannot verify whether enhanced monitoring commitments were implemented. A senior TGA official characterised COVID-19 vaccine safety monitoring as "day-to-day processes" of pharmacovigilance (October 2025), yet no documentation exists to show that these processes met the enhanced monitoring framework set out in the Plan.

International pharmacovigilance guidance, including ICH E2E and CIOMS guidance on signal detection, emphasises structured pharmacovigilance planning and the use of multiple data sources, including active and passive surveillance streams. In this context, the lack of documented multi-source integration prevents confirmation that the TGA's approach was consistent with accepted principles of enhanced pharmacovigilance.

For provisionally registered COVID-19 vaccines, enhanced post-market monitoring formed part of the broader regulatory and policy framework established around provisional registration under the Therapeutic Goods Act 1989 and associated regulatory guidance. Approximately 91.2% of doses (68.4 million of 75 million) were administered while key vaccines remained under provisional registration. Against this context, the inability to demonstrate systematic coordination between surveillance systems represents a significant assurance and compliance-verification gap.

International pharmacovigilance standards recognised by the TGA, including ICH E2E, emphasise systematic pharmacovigilance planning and the integration of relevant safety information. The February 2021 COVID-19 Vaccine Safety Monitoring Plan operationalised a national policy commitment to enhanced monitoring and described coordination across Australia's broader national vaccine safety surveillance system. Without documented protocols, workflows or other records demonstrating how passive and active surveillance were coordinated under the Plan, the available record does not permit independent verification that the enhanced-monitoring framework was systematically implemented.

4.6 Institutional Response Pattern: Direct TGA Communications

Between 31 January and 27 February 2022, three escalating requests were made to TGA seeking formal implementation reporting on the Plan's objectives and outputs. TGA's responses establish the baseline institutional position regarding Plan documentation.

TGA's consistent response:

- Pre-FOI emails (31 January and February 2022): Provided the same four generic pharmacovigilance links (YouTube explainer, weekly safety reports, DAEN causality guidance, monitoring overview) with no Plan-specific references.
- FOI 3643 decision (27 February 2022): Explicitly stated that "no implementation report exists" (para 7) while claiming that "ample documentation" existed (para 8), repeating the same four links for Objectives 1-4 and offering five unsubstantiated collaboration examples for Objective 5.

Key implications:

- Statutory finding: the FOI decision found no implementation report could be located, which is consistent with a governance gap (Output GOV.2: 0% performance measurement documented).
- Substitution pattern: Routine pharmacovigilance outputs consistently substituted for Plan-specific enhanced monitoring evidence.
- Internal contradiction: TGA asserted capability while refusing access under FOI Act s 24A on the basis that requested implementation documents do not exist.

This primary evidence indicates TGA's institutional baseline in early 2022, later corroborated by OAIC-directed searches (September 2024), annual reporting omissions (Appendix B, Section B.5.1), and Senate testimony in October 2025. Full correspondence analysis is provided in Appendix C.

In addition, the FOI 25-0166 consultation notice of 24 April 2025 estimated that more than 399 documents fell within scope of the request, and stated that reviewing them to determine exemptions and apply redactions "would be a substantial and unreasonable diversion of the TGA's resources", citing FOI Guidelines paragraph 3.117 on the impact of processing on other agency work.

5. Root Cause Analysis

The absence of documented implementation frameworks for signal-detection coordination, data-integration protocols, research tracking and governance oversight indicates systematic control failures rather than isolated incidents. TGA's direct communications documented in Appendix C—including three escalating requests and statutory FOI responses—establish that this documentation gap was evident

from February 2022 and has persisted through OAIC-directed sample searches in September 2024. Five potential root causes have been identified.

Scenario 1: Records never created

Implication: Potential weakness in records management controls and potential non-compliance by the accountable authority of the Department of Health, Disability and Ageing with its duty under PGPA Act 2013 s 16 to establish and maintain systems relating to risk and control, and associated PGPA Rule requirements.[79]

Scenario 2: Records created but not retrievable

Implication: Information-management system deficiency. Operational ineffectiveness of document-management controls, evidenced by failure to retrieve implementation records through OAIC-directed sample searches using eight distinct search terms, despite AusVaxSafety being identified as within scope of the Safety Monitoring Plan but not used as a search term.

Scenario 3: Records created and subsequently destroyed

Implication: Potential non-compliance with Archives Act 1983 requirements governing disposal and destruction of Commonwealth records (including s 24) if relevant records were destroyed without proper authority.

Scenario 4: Enhanced monitoring never implemented

Implication: The enhanced pharmacovigilance framework associated with provisional approval may not have been delivered; as a policy commitment and not a statutory requirement, the Plan created no statutory duty to track it, but a gap remains between published Safety Plan commitments and "day-to-day" operational processes.

Scenario 5: Records exist and are retrievable, but access is administratively denied.

Implication: the FOI 25-0166 estimate indicates that documents within scope of the original request can be located within TGA systems. The revised request, framed by reference to standard document types, was refused, so the contents of such documents remain unexamined. The evidentiary position is one of access refused rather than records searched and found wanting; whether the underlying

[79] *Public Governance, Performance and Accountability Act 2013 (Cth),* including duties of accountable authorities and performance-reporting requirements (for example, s 37), and *Archives Act 1983 (Cth)* s 24 recordkeeping obligations.
https://classic.austlii.edu.au/au/legis/cth/consol_act/pgpaaa2013432/s37.html

documentation of Plan implementation exists is not resolved either way by this process.

Note: Each scenario represents a material control deficiency. Identifying the most likely root cause does not alter the finding that enhanced monitoring implementation cannot be verified through available documentation.

Assessment

Of the five scenarios, the available evidence is most consistent with Scenario 1 (records never created) or Scenario 4 (enhanced monitoring never implemented), although it cannot exclude the others, including Scenario 5. The documented pattern is: formal advice that no implementation report exists (FOI 3643, February 2022), OAIC-directed searches across multiple systems and search terms that identified no Plan-specific implementation, governance or AusVaxSafety-integration documents (MR22/00538, September 2024), subsequent refusal, on practical-refusal grounds, of a refined request for up to 30 documents, after the TGA estimated 399+ documents in scope of the original request (FOI 25-0166, April-June 2025), complete absence of Plan implementation in institutional performance reporting, and characterisation of COVID-19 vaccine safety monitoring as “day-to-day processes” in Senate testimony.

The OAIC-directed MR22/00538 searches, conducted by senior officers using eight distinct search terms and searching TRIM for SOPs, protocols, workflows and implementation guidelines yet locating no Safety-Plan implementation documents, are consistent with Scenario 1 or 4, although as sample searches they do not exclude a search-limitation explanation.

While routine AEFI and signal-processing SOPs and workflows exist within AEMS and related systems, OAIC-directed searches and public documentation reveal no Plan-specific procedures that explain how AusVaxSafety’s active-surveillance data were used to trigger, prioritise or validate the 150 safety signals and 57 regulatory actions reported during the COVID-19 programme.[80] [81] [82]

For provisionally approved vaccines where 68.4 million of 75 million recorded doses (about 91.2 per cent) were administered during the provisional approval period, the inability to demonstrate systematic implementation through documented evidence creates an accountability gap that prevents Parliament and oversight bodies from verifying compliance with enhanced monitoring commitments, a transparency failure

[80] https://ncirs.org.au/sites/default/files/2025-09/2022%20AEMS%20COVID-19%20vaccines%20annual%20report.pdf
[81] https://www.tga.gov.au/resources/guidance/pharmacovigilance-responsibilities-medicine-sponsors
[82] https://www.tga.gov.au/news/covid-19-vaccine-safety-reports

between public assurances and verifiable records, and a governance deficiency with implications for future provisional-approval frameworks.

Whether this represents documentation failure or implementation failure, the accountability gap is the same: the available record cannot independently verify compliance with relevant provisional-approval requirements.

6. Priority Reform Areas

This assessment has documented systematic gaps in implementation documentation, governance frameworks, and transparency mechanisms for TGA's COVID-19 Vaccine Safety Monitoring Plan. The following recommendations address the specific deficiencies identified through FOI requests, OAIC oversight, parliamentary testimony, and comprehensive documentary review.

These recommendations are directed to TGA, the Department of Health, Disability and Ageing, Parliament, and relevant oversight bodies including the Australian National Audit Office, Office of the Australian Information Commissioner, and National Archives of Australia.

6.1 Plan-Level Implementation and Governance Documentation

Documented Gap: No implementation frameworks found for Signal detection coordination, regulatory action, collaboration, or Governance oversight despite OAIC-directed sample searches using eight search terms (2,218+ pages estimated).

Recommendation

TGA should produce and publish a consolidated implementation and governance statement for the COVID-19 Vaccine Safety Monitoring Plan (or its successor), explicitly mapping Plan objectives and strategies to operational processes, data sources, decision pathways, and responsible roles.

Future safety monitoring plans for provisionally approved medicines should include minimum documentation standards: implementation matrix, governance map, integration protocols, and performance indicators.

6.2 Documented Integration of AusVaxSafety and Other Data Sources

Documented Gap: No documented protocols for integrating AusVaxSafety active surveillance data (6.8+ million surveys with approximately 3 million respondents reporting at least one adverse event) with TGA's signal detection processes. No traceable linkages between surveillance systems and the 150 safety signal investigations reported.

Recommendation

TGA should develop and publish protocols describing how AusVaxSafety, DAEN, sponsor, and international data streams are integrated for signal detection, prioritisation, and regulatory action, consistent with ICH E2E and CIOMS guidance.

Establish routine, auditable reconciliation processes between AusVaxSafety outputs and TGA's signal lists and COVID-19 safety report narratives, with documented audit trails for all 150 signal investigations including the 93 signals where no regulatory action was taken.

6.3 Strengthened FOI and Records Management Practice

Documented Gap: Documented Gap: the TGA estimated 399+ documents in scope of the original FOI 25-0166 request, then refused a refined request for up to 30 documents under s24(1)(b) practical refusal provisions. OAIC-directed searches failed to locate Plan-specific implementation, audit, or governance documents despite sample searches in TRIM.

Recommendation

TGA should review its FOI practices for high public interest safety matters to reduce reliance on s 24A decisions that documents cannot be found or do not exist where finite corpora of potentially relevant records exist.

Align records management for pharmacovigilance with Archives Act 1983 and ISO 15489 principles to ensure implementation and governance records for major safety monitoring plans are identifiable, retrievable, and properly retained.

Process the refined FOI 25-0166 request or provide detailed justification for practical refusal that addresses OAIC concerns about TGA's pattern of non-disclosure.

6.4 Governance and Performance Measurement

Documented Gap: No governance frameworks or performance measurement mechanisms identified. TGA 2023-24 Annual Report contains zero Plan references, zero output tracking, zero performance metrics for COVID-19 vaccine safety monitoring located despite 68.4 million doses administered under provisional approval.

Recommendation

The Department of Health, Disability and Ageing should ensure that the TGA maintains documented governance structures and performance measurement frameworks for provisional approval monitoring commitments, consistent with the Department’s obligations under the PGPA Act 2013.

TGA Performance Reports should include specific performance reporting against provisional approval monitoring plan commitments, with measurable indicators demonstrating whether enhanced monitoring occurred as specified.

6.5 Independent Performance Audit of COVID-19 Vaccine Pharmacovigilance

Documented Gap: Australia's largest provisional approval experience (68.4 million doses, 91.2% of rollout) has produced no publicly available implementation assessment, documented lessons learned, or systematic evaluation against Plan objectives.

Recommendation

The Australian National Audit Office should commission a performance audit focused specifically on TGA's COVID-19 vaccine pharmacovigilance system, including:

- Implementation of the February 2021 Safety Monitoring Plan
- Integration with AusVaxSafety and other partner surveillance systems
- Compliance with PGPA Act and Therapeutic Goods Act obligations
- Records management effectiveness and FOI compliance

This audit should benchmark Australian practice against EMA, FDA, and WHO guidance on enhanced post-marketing surveillance for provisionally approved medicines.

6.6 Transparency and Public Accountability

Documented Gap: Of 150 signals assessed, 93 did not result in a reported regulatory action, and no publicly available signal-by-signal audit trail documents their disposition. Public communications stated "rigorous and enhanced safety monitoring" but systematic enhancement beyond routine pharmacovigilance cannot be verified.

Recommendation

TGA should publish comprehensive signal investigation summaries for all COVID-19 vaccine safety signals, including those where no regulatory action was taken, with documented rationale for closure decisions.

Future provisional approval monitoring plans should specify explicit transparency commitments with defined public reporting schedules, investigation disclosure thresholds, and performance metrics enabling independent verification.

7. Evidence Assessment and Anticipated Objections

Three tiers of evidence support these findings, with predictable institutional objections addressed below:

1. **Documented absence of implementation records**
   The record shows that public material contains no Plan-specific implementation, governance or integration documents, and that TGA's September 2024 sample searches using Plan-aligned terms likewise identified none. FOI 3643 and OAIC Decision [2025] AICmr 54 establish that the documents sought cannot be found or do not exist. In FOI 25-0166 the TGA estimated that more than 399 documents fell within scope of the original request, but refused the revised request, framed by document type, before any contents were examined, leaving that avenue unresolved. Together these strands provide a powerful, triangulated basis for concluding that there is no verifiable implementation trail for the Plan.
2. **Signal-detection and AusVaxSafety integration gap.**
   The analysis combines AusVaxSafety outputs (more than 6.8 million surveys and about 62,000 medical-attention reports), TGA’s 150 safety signals and 57 regulatory actions, the absence of any protocols linking the two in MR22/00538 searches or FOI releases, ICH E2E/CIOMS expectations for pharmacovigilance planning and signal management, and EMA GVP Module IX requirements for traceable signal-management audit trails. This makes the finding of a critical control deficiency in signal detection and governance difficult to rebut without producing actual Plan-specific integration and audit-trail documentation.
3. **Pattern over time, not a one-off glitch.**
   The FOI/OAIC timeline shows a consistent trajectory from early 2022 to late 2025: generic assurances in pre-FOI emails, a s 24A “no documents” decision for FOI 3643 and its confirmation in Decision [2025] AICmr 54, OAIC-directed MR22/00538 sample searches that still identified no Plan implementation records, and a 2025 practical-refusal decision on FOI 25-0166 despite 399 in-scope documents containing 3,926 pages being identified. This pattern supports the conclusion that the gap is systemic, not an isolated administrative oversight.

7.1 Potential counter-arguments and responses

“Generic pharmacovigilance SOPs implicitly cover the Plan.

Response: MR22/00538 indicates that while TGA holds extensive AEFI and signal-management SOPs, no documents map those procedures to the Plan’s objectives, outputs or enhanced multi-source integration requirements, particularly Strategy 2.4 on AusVaxSafety coordination. Without such mapping or governance records, there is no evidence that generic SOPs were systematically adapted to deliver the Plan’s additional requirements beyond “business as usual”.

“AusVaxSafety existed and functioned, so enhanced monitoring commitments were met.”

Response: AusVaxSafety’s active-surveillance system clearly operated and published aggregate reports (6.8M+ surveys and ~62,000 medical-attention reports), but MR22/00538 and FOI releases identify no protocols, decision rules or audit trails showing how AusVaxSafety data were integrated with TGA’s AEFI workflows, 150 signals, 57 actions or 93 “no further action” signals, as envisaged by Strategy 2.4. The existence of a system is not, by itself, evidence that Plan-mandated integration and governance were implemented, however without documented decision criteria and risk assessments, it's impossible to verify whether these 93 closures represented appropriate safety judgments or system failures to detect problems.

“Enhanced monitoring occurred even if documentation is incomplete.”

Response: For provisionally approved vaccines, enhanced monitoring can be understood as a safeguard that compensates for reduced pre-market evidence, while international pharmacovigilance guidance such as ICH E2E and CIOMS establishes principles for structured pharmacovigilance planning and signal management. The absence of protocols, decision criteria, cross-system reconciliation rules and case-level audit trails four years after Plan publication means enhanced monitoring cannot be independently verified against the expected control standard.

“Relevant records may exist but were not captured by searches.”

Response: MR22/00538 used eight Plan-aligned search terms in TRIM, with separate sample searches by objective estimating over 2,218 pages. OAIC’s Decision [2025] AICmr 54 accepted those searches as “all reasonable steps” and upheld the s 24A “no documents” finding for an implementation report, which significantly weakens claims that substantial Plan-specific implementation records exist but were missed.

“Workload justified refusing FOI 25-0166.”

Response: the revised request was already tightly scoped: up to 20 priority documents plus, under item 2, at most thirty documents (six per objective), limited to defined document types and a four-year window. The TGA estimated more than 399 documents fell within scope of the original request, and it refused access without processing a prioritised sample, having earlier referred the applicant, in its consultation notices, to published safety reports and Advisory Committee web pages. The matter is under Information Commissioner review (MR25/01153), where s 55D places the onus on the agency to establish that its decision is justified.

“Expert committees substitute for documented implementation.”

Response: Committees such as ATAGI, ACV and SITAG provided clinical and technical advice on vaccines, risk–benefit assessment and rollout strategy, but advisory structures do not replace the need for records showing how that advice was operationalised into Plan-level processes, integrated surveillance and performance measurement. No Plan-specific terms of reference, standing agendas or decision records were identified that demonstrate how ATAGI, ACV and AEFI-CAN systematically fed COVID-19 vaccine safety advice into TGA’s signal-management workflows and regulatory actions; available documentation shows that committees existed and met, but not how their deliberations were linked to Plan-level implementation. The absence of such records remains an accountability gap even if expert advice was sound.

“FOI 3643 and Decision [2025] AICmr 54 have already resolved the issue.”

Response: MR22/00538 searches used eight distinct terms including operational vocabulary (signal detection, pharmacovigilance, implementation, audit) across Plan-aligned objectives, not merely the Plan title. TGA's September 2024 searches successfully located extensive routine pharmacovigilance documentation (>2,218 sampled pages of AEFI workflows, signal procedures, regulatory actions), demonstrating that operational documents *are* retrievable through TGA's systems when they exist.

The absence extends beyond operational documents to governance frameworks, performance measurement systems, and coordination protocols — categories that would have been created specifically for the Plan rather than predating it. The counter-argument would require the TGA to explain why routine procedures are well documented while Plan-specific enhancements are not, and why access to the documents estimated to fall within scope of FOI 25-0166 was refused before their contents were examined. OAIC's Decision [2025] AICmr 54 accepted the TGA's search methodology as meeting the "all reasonable steps" requirement, further weakening claims that substantial Plan-specific implementation records exist but were missed.

“FOI 25-0166 proves implementation records exist, so ‘Not documented’ ratings are invalid”

Response: FOI 25-0166 does not establish that implementation records exist. The TGA's estimate concerned documents within scope of a request framed by reference to standard document types; access was refused before any document was examined, so nothing in that process establishes what the documents contain or whether any evidences Plan implementation. The classification "Not documented" in this report refers to the absence of publicly verifiable, Plan-specific implementation documentation through published sources, FOI 3643, OAIC MR22/00538 and related

processes. That classification is unaffected. The question of access remains open and is before the Office of the Australian Information Commissioner (MR25/01153).

7.2 Overall assessment

On the central test—whether the regulator can demonstrate, on the record, that the Plan's enhanced monitoring objectives and multi-source integration requirements were implemented and governed—the case presented here is very strong. To dislodge it, critics would need to do more than question the interpretation; they would have to produce Plan-specific implementation, governance or integration documents that multiple FOI processes, OAIC-directed searches and this independent review have not been able to identify.

8. Conclusion

The February 2021 COVID-19 Vaccine Safety Monitoring Plan expressly built on existing pharmacovigilance activities while establishing a strengthened and coordinated approach to COVID-19 vaccine safety monitoring. The documentary question is therefore not whether routine pharmacovigilance occurred, but whether the additional commitments, coordination and oversight described in the Plan were implemented and systematically documented in a manner that permits independent verification. This assessment identifies a systematic gap between the Plan and verifiable evidence of its implementation. Key process controls including signal detection coordination, data integration, governance frameworks and performance measurement were not evidenced in the available record. TGA cannot demonstrate through the available documentation that enhanced monitoring under the Plan was systematically implemented and tracked beyond routine operational pharmacovigilance. Accordingly, the available record does not permit independent verification that the enhanced-monitoring framework committed to under the Plan was delivered as intended.

The AusVaxSafety case study exemplifies this gap: 6.8+ million surveys documenting 3 million adverse events and 62,000 medical attention visits, yet zero documented coordination protocols linking this population-level data to TGA's 150 signal investigations. Infrastructure existed and functioned. Systematic integration cannot be verified.

Across FOI processes, OAIC-directed TRIM searches and product-level AusPARs, no verifiable documentation was identified showing implementation, governance or performance measurement of the February 2021 COVID-19 Vaccine Safety Monitoring Plan, including for key products later transitioned from provisional to full registration.

For provisionally registered products, enhanced monitoring formed part of the regulatory and policy framework established to manage the additional uncertainty

associated with provisional registration and incomplete safety data. The February 2021 Plan represented a national commitment to enhanced monitoring beyond routine pharmacovigilance. TGA subsequently characterised COVID-19 vaccine safety work as occurring through "day-to-day processes." If the enhanced-monitoring commitment was operationally indistinguishable from routine pharmacovigilance, and could not be separately demonstrated or tracked, the documentary basis for assuring that the additional oversight contemplated by the provisional-approval framework was delivered is materially weakened.

TGA's own performance reports for 2022-23 and 2023-24 do not report against the COVID-19 Vaccine Safety Monitoring Plan or the enhanced monitoring obligations associated with provisional approvals, leaving these core commitments outside the agency's formal performance framework.

After four years of investigation—statutory FOI processes, OAIC-directed sample searches, institutional performance reporting review, Senate testimony under parliamentary privilege—the probability that systematic implementation documentation exists but remains undisclosed is low. The evidence indicates enhanced monitoring frameworks were not systematically implemented with the documented governance, coordination protocols, and performance tracking required to demonstrate that the enhanced monitoring measures were implemented as promised.

Australia's COVID-19 vaccine rollout represents the nation's largest deployment of provisionally approved medicines: 68.4 million doses, 91.2% of the total rollout, administered under provisional approval accompanied by commitments to enhanced monitoring that cannot be verified through documented evidence.

TGA can resolve this by producing the missing documentation. If documentation cannot be produced, this matter requires investigation by the Australian National Audit Office, Commonwealth Ombudsman, or relevant parliamentary oversight committees. After estimating 399 documents in scope of the original request but refusing to process the revised one (FOI 25-0166), after OAIC-directed sample searches reporting no Plan implementation records (MR22/00538), and after four years of consistent institutional inability to demonstrate enhanced monitoring beyond routine processes, two possibilities remain: either the enhanced monitoring framework was never systematically implemented, or it was implemented but TGA failed to document it. Both scenarios represent a significant governance failure and potential non-compliance with Commonwealth accountability frameworks. Neither is acceptable for 68.4 million doses administered under provisional approval.

9. Detailed Methodological Framework and Standards

9.1 Assessment Framework

Section 9 expands on the scope and methodology overview in Section 1.2.

This assessment employs ISO 19011:2018 audit principles, ISO 15489 (records management), Australian National Audit Office performance-audit methodology and international pharmacovigilance standards (including ARGPM, ICH E2E and EMA GVP) to systematically evaluate implementation of TGA's COVID-19 Vaccine Safety Monitoring Plan. The framework evaluates not only operational activities but also governance processes, performance-measurement arrangements and the adequacy of documented information, and is benchmarked against international pharmacovigilance guidance that TGA has adopted for use in Australia, including ICH E2E Pharmacovigilance Planning (in place internationally since 2005), EMA Good Pharmacovigilance Practice modules and CIOMS risk-minimisation guidance.[83]

9.2 Independence and Scope

This is an independent assessment conducted without TGA funding, authorisation, or oversight. The assessor has no financial, professional, or personal conflicts of interest relating to TGA, COVID-19 vaccine manufacturers, or related commercial entities:

- Assessment period: February 2021 (Plan publication) to February 2026 (assessment finalisation), with additional relevant documents identified and incorporated through August 2026.
- Evidence collection period: January 2022 (initial FOI request) to October 2025 (Senate testimony).
- Key source document: TGA COVID-19 Vaccine Safety Monitoring Plan (February 2021 version).

9.3 Assessment Limitations

This assessment is limited to publicly available documentation and information provided through FOI/OAIC statutory processes. It cannot:

- Access TGA's internal systems, databases, or non-public records
- Interview TGA staff or review internal communications, emails, or meeting records
- Verify whether coordination processes occurred informally without documentation

[83] See TGA *International scientific guidelines adopted in Australia (ICH E2E Pharmacovigilance Planning); ICH E2E guideline; EMA Good Pharmacovigilance Practices and CIOMS IX risk-minimisation guidance.*

- Determine specific root causes without internal investigation and staff interviews
- Assess clinical or scientific quality of safety monitoring (scope limited to documentation and governance)

Implications: Findings reflect what can be verified through documented evidence accessible to Parliament, oversight bodies, and the public. The assessment documents absence of verifiable implementation evidence, not definitive conclusions about what did or did not occur within TGA's internal processes. If systematic implementation occurred without adequate documentation, this would indicate a records management control failure and potential non-compliance with Archives Act 1983 requirements concerning the retention and authorised disposal of Commonwealth records, and with the PGPA Act 2013 duties of the accountable authority of the Department of Health, Disability and Ageing to establish effective systems of risk management and internal control.[84] [85] If systematic implementation did not occur as planned, this would represent an implementation failure inconsistent with the enhanced monitoring commitments attached to provisional approval.

9.4 Evidence Hierarchy

Evidence is ranked by verifiability and reliability (ISO 19011:2018, Clause 6.4.7, under which only verifiable information is accepted as audit evidence and the auditor judges the degree of reliance to place on it) and by evidential authoritativeness (ISO 15489-1:2016, Clause 5.2.2, under which records possessing authenticity, reliability, integrity, and usability constitute authoritative evidence). Higher-tier evidence takes precedence in case of conflict; all findings are anchored in Tier 1 or Tier 2 evidence.

Tier 1: Primary Statutory Evidence[86]

- FOI decision FOI 3643 (27 February 2022) - statutory decision that no implementation report could be identified
- OAIC submissions MR22/00538 (September 2024) and MR25/01153 - formal positions in an OAIC review
- Direct TGA communications to the applicant (31 January - 27 February 2022) - establishing institutional baseline

[84] *Archives Act 1983 (Cth)* s 24 (unlawful destruction or disposal of Commonwealth records).
[85] National Archives of Australia, Disposal of Commonwealth records in the absence of an agency-specific records authority.
[86] "Statutory" is used here in the ordinary sense of arising under, or being regulated by, statute. Tier 1 comprises records generated in the course of processes under the Freedom of Information Act 1982 (Cth) and associated OAIC review processes, including TGA decisions, submissions and correspondence within those processes.

Tier 2: Primary Documentary Evidence

- Senate Community Affairs Legislation Committee testimony under parliamentary privilege (9 October 2025)
- TGA Annual Performance Reports 2022-23 and 2023-24 - statutory performance reporting
- TGA published COVID-19 vaccine safety reports, including November 2023 Safety Signals Report
- TGA COVID-19 Vaccine Safety Monitoring Plan (February 2021)

Tier 3: Secondary Documentary Evidence

- Partner organisation reports (NCIRS AEMS COVID-19 vaccines annual report; AusVaxSafety surveillance data)
- Department of Health, Disability and Ageing Australian Immunisation Register data (vaccination statistics)
- Publicly accessible surveillance data and performance metrics

Tier 4: Tertiary Contextual Evidence

- International regulatory guidance (EMA, MHRA, FDA enhanced monitoring frameworks)
- Pharmacovigilance standards (ICH E2E signal detection; CIOMS multi-source integration)
- Comparative regulatory frameworks

Higher-tier evidence takes precedence in case of conflict. All findings are anchored in Tier 1 or Tier 2 evidence. Full search methodology and source verification are documented in Appendix B. Direct TGA communications establishing the institutional baseline are documented in Appendix C.

9.5 Audit Criteria Hierarchy

The assessment applies multi-layered criteria to ensure findings are grounded in TGA's own commitments while applying recognised professional standards:

- **Primary criteria:** TGA Plan commitments (Strategies 1.1-5.2) - the entity's specified outputs and deliverables
- **Secondary criteria:** ISO 19011 governance requirements (management review, performance measurement, documentation adequacy)
- **Tertiary criteria:** Commonwealth legislative obligations (PGPA Act 2013 ss 37–39, applying to the Department of Health, Disability and Ageing as the relevant

Commonwealth entity; Archives Act 1983 s 24, applying to Commonwealth records).[87]

- **Quaternary criteria:** Quaternary criteria: International pharmacovigilance guidance (ICH E2E pharmacovigilance planning; CIOMS guidance on signal detection and multi-source surveillance)

Primary criteria form the assessment baseline; secondary through quaternary criteria provide benchmarks for evaluating whether implementation met recognised standards for enhanced monitoring of provisionally approved medicines.

9.6 Assessment Scope

The Plan specifies 17 numbered strategies (1.1-1.5, 2.1-2.8, 3.1, 4.1, 5.1-5.2). To ensure comprehensive evaluation consistent with ISO 19011 and provisional approval accountability frameworks, the assessment evaluates 19 outputs:

- **Operational strategies (17 outputs):** Plan-specified activities across five objectives
- **Governance framework (2 outputs):** implementation oversight and performance measurement (PGPA Act 2013 ss 37-39), assessed against the Department of Health and Aged Care's obligations under PGPA Act 2013 ss 37-39
- **Documentation systems:** Records-management adequacy (ISO 15489-1; PGPA Act 2013 s 37; Archives Act 1983 s 24)

This approach evaluates whether enhanced monitoring was designed, implemented, operated, and documented as specified, with particular attention to coordination mechanisms between surveillance systems (Strategy 2.4 AusVaxSafety integration).

Each output was classified as:

- **Implemented:** Documented operational delivery with independently verifiable evidence
- **Partially implemented:** Process documentation exists with limited or no records of results
- **Not documented:** No relevant evidence located through FOI, OAIC and public-record searches

Classification decisions required minimum evidentiary thresholds: 'Implemented' status required documented protocols, verifiable outputs, and audit trails enabling independent verification. 'Not Documented' status was assigned only after FOI

[87] See *Public Governance, Performance and Accountability Act 2013 (Cth)* s 37 on keeping records that properly record and explain an entity's performance, and *Archives Act 1983 (Cth)* s 24 on unlawful destruction or disposal of Commonwealth records. https://classic.austlii.edu.au/au/legis/cth/consol_act/pgpaaa2013432/s37.html

processes, Department searches and public records failed to locate relevant evidence.

9.7 Methodological Principles

Evidence-Based Analysis: All findings are grounded in verifiable documentary evidence and official records rather than speculation.

Falsifiability: Every finding can be disproved through production of contradicting documentation. If TGA provides:

- Documented protocols for coordinating AusVaxSafety data with signal investigations
- Documented signal investigation procedures specific to COVID-19 vaccines
- Documented Plan implementation governance and performance measurement systems
- Documented decision-making criteria linking investigated signals to regulatory actions

then corresponding findings would be revised or withdrawn accordingly.

Temporal Analysis: Tracking institutional statements across four years (FOI 3643 February 2022 → OAIC MR22/00538 September 2024 → TGA Annual Report October 2024 → Senate testimony October 2025) is consistent with systematic patterns rather than isolated incidents or retrieval failures.

Replicability: Independent researchers using identical sources, assessment framework, and audit criteria would reach consistent findings. All evidence derives from publicly available sources or FOI-released documents, ensuring transparency and independent verification.

Search Scope: Evidence collection encompassed statutory FOI processes, OAIC-directed sample searches in TGA's TRIM records management system using eight distinct search terms, systematic review of TGA annual performance reports and safety publications, parliamentary testimony, and partner organisation reporting. Detailed search methodology documented in Appendix B.

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11. Glossary of Key Terms

Active surveillance: Proactive systematic collection of safety data through direct contact with vaccine recipients, typically using structured surveys or follow-up protocols (e.g., AusVaxSafety SMS surveys). Contrasts with passive surveillance which relies on voluntary reporting.

ACV (Advisory Committee on Vaccines): A statutory expert advisory committee, established under the Therapeutic Goods Regulations, that provides independent medical and scientific advice to the Minister for Health and the TGA on the safety, quality and efficacy of vaccines supplied in Australia, including advice on pre-market evaluation, post-market safety monitoring and vaccine use in national immunisation programme.

Adverse Event Following Immunisation (AEFI): Any untoward or unexpected medical occurrence that follows immunisation, which may or may not be causally related to the vaccine, and which must be monitored, managed and reported through vaccine safety surveillance systems.

AEMS (Adverse Event Management System): TGA's electronic system and portal for receiving, storing, and managing adverse event reports for medicines and vaccines, including draft and submitted reports and follow-up information.

ANAO (Australian National Audit Office): Independent Commonwealth auditor providing assurance to Parliament on public sector performance, accountability, and use of public resources. Its Audit Insights and Audit Manual set out professional expectations for performance measurement and reporting. (The earlier ANAO Better Practice Guides were withdrawn by 29 June 2018).

Australian Public Assessment Report (AusPAR): A public regulatory document prepared by the TGA that summarises the evaluation of a prescription medicine submission, explains the scientific reasoning for the approval (or non-approval) decision, and records key outcomes at a specific point in time.

ATAGI (Australian Technical Advisory Group on Immunisation): Expert committee that provides technical advice on immunisation to the Minister and Department of Health, Disability and Ageing, including recommendations on vaccine use, schedules and risk–benefit assessments.

Audit: Systematic, independent examination of processes, systems, and documentation to determine whether activities conform to planned arrangements and whether arrangements are implemented effectively. Conducted according to ISO 19011:2018 principles.

Audit trail: Documented sequence from signal detection through assessment to regulatory decision, enabling independent verification of pharmacovigilance system performance. Systematic documentation and traceability are recognised expectations in international pharmacovigilance guidance and standards.

AusVaxSafety: Active surveillance system operated by NCIRS using SMS surveys to collect safety data from vaccine recipients. Documented 6.8+ million surveys for COVID-19 vaccines capturing 3 million adverse events and 62,000 medical attention visits.

Business as Usual (BAU): Routine operational activities carried out under standard policies, processes, and controls, as opposed to special, time-limited, or “enhanced” arrangements put in place in connection with provisional approval or emergency response requirements.

Critical control: Essential process, procedure, or system required to ensure regulatory compliance, protect public safety, or enable verification of system performance. Failure of critical controls represents material governance deficiency.

DAEN (Database of Adverse Event Notifications): TGA's passive surveillance system for voluntary reporting of adverse events by health professionals and consumers.

Documentary assurance: The extent to which available records allow an external reviewer to verify that planned activities, controls or commitments were implemented, monitored and reported. Documentary assurance does not establish that an activity occurred solely because records are absent; it concerns whether implementation can be independently verified from the available evidence.

Documentation gap: Absence of verifiable records demonstrating systematic implementation of planned activities, protocols, or frameworks. Distinguished from implementation failure (activity did not occur) versus records management failure (activity occurred but was not documented).

Enhanced monitoring: Pharmacovigilance activities intended to strengthen monitoring beyond routine post-market surveillance in the context of provisional registration and incomplete safety data. The February 2021 Plan operationalised a national policy commitment to enhanced monitoring for COVID-19 vaccines, building on existing pharmacovigilance activities.

Evidence base: Complete collection of documented information used to support findings, including primary statutory evidence (FOI decisions, OAIC submissions), primary documentary evidence (Senate testimony, TGA reports), secondary documentary evidence (partner reports), and tertiary contextual evidence (international standards).

Expert advice: Independent specialist assessment provided to inform regulatory decisions on complex safety signals. Should be documented including terms of reference, expert qualifications, advice provided, and how advice informed decisions.

Follow-up: Systematic process of obtaining additional information on reported adverse events or safety signals through targeted investigation, data collection, or clinical assessment. Requires documented protocols and decision criteria.

FOI (Freedom of Information): Australian statutory regime under the Freedom of Information Act 1982 (Cth) that provides a legally enforceable right of public access to documents held by Commonwealth agencies, subject to exemptions, with decisions reviewable internally and externally by the Office of the Australian Information Commissioner (OAIC).

Fully implemented: Classification indicating documented operational delivery with independently verifiable evidence of systematic implementation including protocols, outputs, and audit trails.

Governance: Framework of accountability, oversight, decision-making authority, and performance measurement ensuring activities are conducted systematically and comply with regulatory obligations. Includes management review, documentation systems, and quality assurance.

ICMRA (International Coalition of Medicines Regulatory Authorities): High-level coalition of medicines regulators from multiple jurisdictions that coordinates regulatory responses and information-sharing on issues such as COVID-19 vaccine safety.

Information asymmetry: Situation where regulatory authority possesses complete information about its activities and documentation while external parties (Parliament, oversight bodies, public) can only verify what is documented and disclosed. Creates accountability risk if documentation systems are inadequate.

ISO 15489: International standard providing principles and guidance for records management, including the creation, capture and maintenance of records as evidence of business activities. Used as a records-management reference in this audit alongside applicable Commonwealth recordkeeping requirements.

ISO 19011: ISO 19011:2018: international guideline for auditing management systems. Clause 5.5.2 covers defining the criteria and scope of an individual audit.

Multi-source integration: Pharmacovigilance practice of systematically combining data from multiple surveillance systems (active surveillance, passive reporting, clinical studies, international reports) to enable comprehensive signal detection. Requires documented coordination protocols and integration frameworks.

NCIRS (National Centre for Immunisation Research and Surveillance): Australia's leading independent research organisation on vaccine-preventable diseases and immunisation, which operates AusVaxSafety and provides immunisation surveillance and vaccine-safety research, as well as technical advice to bodies such as ATAGI and the Australian Immunisation Handbook; NCIRS is a partner organisation acknowledged in TGA's COVID-19 Vaccine Safety Monitoring Plan.

Not documented: Classification indicating no relevant evidence located through FOI processes, Department searches and public records across multiple evidence tiers.

OAIC (Office of the Australian Information Commissioner): Independent Commonwealth statutory agency that reviews FOI decisions and ensures government compliance with information access obligations under FOI Act 1982.

Partially implemented: Classification indicating process documentation exists with limited or no records of results.

Passive surveillance: Voluntary reporting of adverse events by health professionals and consumers without proactive follow-up (e.g., DAEN database). Contrasts with active surveillance which systematically contacts vaccine recipients.

PGPA Act (Public Governance, Performance and Accountability Act 2013): Commonwealth legislation establishing governance, performance and accountability obligations for Commonwealth entities. Section 37 requires the accountable authority to cause records to be kept that properly record and explain the entity's performance; broader duties of accountable authorities cover governance, risk management and internal control, while detailed recordkeeping obligations are primarily set out in the Archives Act 1983.[88]

Pharmacovigilance: Science and activities relating to detection, assessment, understanding, and prevention of adverse effects or other medicine-related problems. Systematic pharmacovigilance requires documented processes, performance measurement, and audit trails.

Policy commitment: A stated commitment adopted through government policy or administrative arrangements that establishes an expected course of action, but does not itself create a statutory obligation unless supported by legislation, regulation or another legally binding instrument.

Promapp: A cloud-based business process management tool used to document and maintain workflow process maps (e.g. case-handling steps, escalation flows) in a central repository for operational procedures.

[88] https://classic.austlii.edu.au/au/legis/cth/consol_act/pgpaaa2013432/s37.html

Provisional approval: Time-limited registration pathway under the Therapeutic Goods Act 1989 that allows earlier market access for medicines treating serious conditions where preliminary clinical data indicate likely benefit. Under TGA's provisional-registration framework, sponsors may be subject to additional conditions and post-market obligations, including further evidence generation and pharmacovigilance requirements. For COVID-19 vaccines, these sponsor-specific obligations operated alongside the separate policy commitment to enhanced safety monitoring set out in the February 2021 Safety Monitoring Plan.[89]

Regulatory failure: A situation where the responsible regulator cannot demonstrate, through adequate records, governance frameworks, and performance evidence, that it has complied with the legal and policy requirements and commitments associated with provisional approval governing COVID-19 vaccine safety monitoring, despite having the operational capability to do so and an obligation to document its performance of official functions under Commonwealth record-keeping law.

Regulatory framework: The interconnected framework governing therapeutic goods regulation in Australia, comprising the Therapeutic Goods Act 1989 (Cth), associated regulations, legislative instruments and applicable regulatory guidance, together with relevant policy commitments and recognised international standards. These sources differ in legal status and function: the Act and associated regulations establish statutory requirements; legislative instruments may have binding legal effect where authorised by legislation; regulatory guidance, policy commitments and recognised standards may establish regulatory expectations or recognised good practice but do not themselves create statutory obligations unless supported by an applicable legislative or regulatory provision.

Root cause analysis: Systematic investigation methodology identifying underlying causes of observed problems rather than addressing symptoms. Applied in this assessment to determine whether documentation gaps indicate records management failure, implementation failure, or both.

Safety signal: Information suggesting potential causal association between medicine and adverse event requiring evaluation. Distinct from "signal" (preliminary information) and "confirmed safety issue" (causality established).

Signal: Information arising from one or multiple sources suggesting a new potentially causal association, or a new aspect of a known association, between an intervention and an event or set of related events.

Signal detection: Signal detection: Systematic process of identifying, evaluating, and prioritising potential safety signals from multiple surveillance sources.

[89] See TGA, *Guidance: Applying for provisional registration of a prescription medicine*; TGA, *Guidance: Provisional registration extension and transition to full registration*; and TGA*, COVID-19 Vaccine Safety Monitoring Plan (February 2021).*

International pharmacovigilance guidance provides principles for structured signal detection and management, including pharmacovigilance planning (ICH E2E), signal detection and management (CIOMS VIII), and documented, traceable signal-management processes and audit trails (EMA GVP Module IX).

Signal identification and assessment: Two-stage pharmacovigilance process: (1) identification—systematic screening of data sources to detect potential signals; (2) assessment—evaluation of signal validity, clinical significance, and need for regulatory action. Both stages require documented procedures and decision criteria.

Signal investigation: Detailed examination of validated safety signal including literature review, analysis of additional data sources, statistical evaluation, expert consultation, and risk-benefit assessment. Should be documented with clear outcomes informing regulatory decisions.

Sponsor: The individual or company legally responsible for a therapeutic good in Australia, including applying for inclusion on the Australian Register of Therapeutic Goods (ARTG), maintaining regulatory compliance, and meeting post-market reporting and monitoring obligations.

Standard Operating Procedure (SOP): A formally approved, written document that sets out step-by-step instructions for performing a specific routine task or process, so that staff carry it out the same way each time, ensuring consistency, quality, safety, and compliance with applicable laws, policies and standards.

Surveillance: Ongoing systematic collection, analysis, and interpretation of safety data to detect, assess, and understand adverse events associated with medicines. Includes both active and passive surveillance methodologies.

TGA (Therapeutic Goods Administration): Australia's medicines and therapeutic goods regulator operating within the Department of Health, Disability and Ageing. Responsible for evaluating and regulating therapeutic goods and conducting post-market safety monitoring. In this audit, TGA's implementation of the February 2021 COVID-19 Vaccine Safety Monitoring Plan is assessed as a national policy and governance commitment.

The Therapeutic Goods Act 1989: establishes the high-level framework for registration, listing and provisional registration of medicines, but most detailed requirements for enhanced post-market monitoring, risk-management and conditions of provisional registration are set out in associated regulations, legislative instruments and TGA guidance (including the 2017 Provisional Approval reforms, provisional registration guidance, and pharmacovigilance guidelines).

The Plan: TGA’s COVID-19 Vaccine Safety Monitoring Plan (February 2021), which operationalised a national policy commitment to strengthened safety monitoring for COVID-19 vaccines within the broader provisional-registration and national vaccine

safety surveillance framework. The Plan contains five stated objectives and associated strategies; this audit additionally assesses two governance-related outputs.

TRIM (Content Manager): The Department's electronic document and records management system (EDRMS), used to store, classify, and retrieve official digital and physical records in compliance with Commonwealth recordkeeping obligations.

Verification gap: A circumstance in which available records do not permit independent verification that a specified activity, commitment, control or process was implemented, monitored or considered in a relevant decision.

Appendix A: Regulatory and Accountability Framework

Figure A.1 provides an overview of the regulatory and accountability framework relevant to the audit. It distinguishes the national policy commitment to enhanced safety monitoring, the TGA's February 2021 Safety Monitoring Plan, the statutory provisional-registration pathway and sponsor-specific regulatory obligations, and shows how these elements relate without treating them as a single legal requirement.

Figure A.1 Regulatory and accountability framework for COVID-19 vaccine safety monitoring in Australia

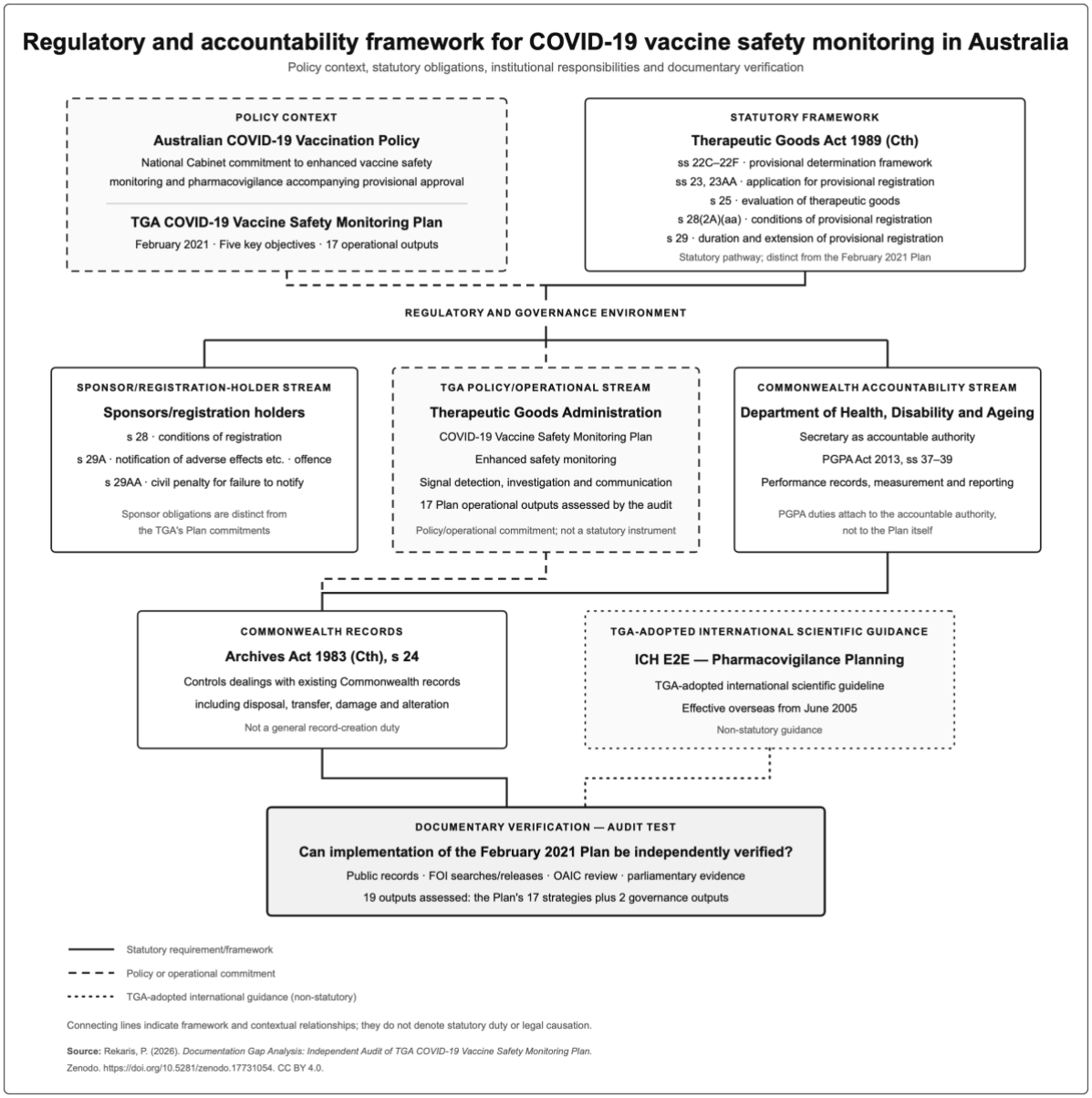


Figure A.1 Regulatory and accountability framework for COVID-19 vaccine safety monitoring in Australia. Solid boxes indicate statutory requirements on the named duty-holder. Dashed boxes indicate policy or operational commitments. Dotted boxes indicate TGA-adopted international scientific guidance outside the statutory chain. Connecting lines show framework and contextual relationships; they do not denote statutory duty. The relevant Commonwealth entity is the Department of Health, Disability and Ageing, whose accountable authority is the Secretary.

A.1 The TGA COVID-19 Vaccine Safety Monitoring Plan

Background

The TGA COVID-19 Vaccine Safety Monitoring Plan (Version 1.0, February 2021) was released as Australia commenced the COVID-19 vaccination program and COVID-19 vaccines were provisionally registered. The Plan described the TGA's approach to enhanced safety monitoring of COVID-19 vaccines introduced through the provisional-registration pathway. It was intended to support the timely identification, investigation and management of emerging vaccine safety issues. The audit assesses documented implementation of the Plan; it does not assess sponsor compliance with conditions of registration.

The sections that follow explain each component and its relevance to the audit.

A.2 Provisional registration and the role of enhanced monitoring

Provisional registration under the Therapeutic Goods Act 1989 (Cth) provides a pathway for medicines where registration is supported by preliminary clinical data. For an application for provisional registration, s 25(1)(d) requires the Secretary to evaluate the medicine having regard to the statutory provisional-registration criteria, including the preliminary clinical data concerning safety and efficacy, quality and the applicant's plan for submission of comprehensive clinical data. Section 25(3) provides for the Secretary's decision to register or not register the goods. Section 29 provides for a defined provisional-registration period and mechanisms for extension. In this audit, enhanced post-market safety monitoring is characterised as a compensating safeguard for the evidentiary uncertainty inherent in provisional registration. This is an analytical characterisation rather than a term used in the legislation.

The February 2021 Plan set out the TGA's own policy and operational framework for this enhanced monitoring. It was separate from the conditions and obligations applying to sponsors. Sponsor pharmacovigilance obligations arose through the therapeutic-goods regulatory framework and individual registration arrangements, including statutory requirements concerning notification of adverse effects under s 29A of the Therapeutic Goods Act 1989. The Plan instead described what the TGA itself intended to undertake in monitoring the safety of COVID-19 vaccines.

The Plan therefore sits within the broader provisional-registration framework but is not itself a statutory condition of registration. Sponsor compliance with reporting and risk-management requirements does not establish that the TGA implemented the commitments described in its own Plan.

A.3 National vaccination policy context

The provisional registration of COVID-19 vaccines occurred within a broader national policy framework for the introduction and monitoring of COVID-19 vaccines. National Cabinet's vaccination policy provided the policy context for the accelerated introduction of vaccines, including the use of the TGA's provisional-registration pathway and the commitment to enhanced safety monitoring.

The statutory pathway and the enhanced-monitoring commitment were related but operated at different levels. The *Therapeutic Goods Act 1989 (Cth)* provided the legal basis for provisional registration, including the provisions governing provisional registration, its conditions, duration and extension. The national policy framework provided the broader policy setting in which provisional registration was used and committed the Commonwealth to enhanced monitoring of vaccine safety.

Within that framework, the February 2021 TGA COVID-19 Vaccine Safety Monitoring Plan provided the TGA's policy and operational implementation of the enhanced-monitoring commitment. For this audit, the Plan is therefore treated as a policy safeguard accompanying provisional registration and intended to address the evidentiary uncertainty associated with reliance on preliminary evidence. This is an analytical characterisation of the Plan's role; the *Therapeutic Goods Act 1989* does not itself describe the Plan as a statutory safeguard or expressly require the Plan's implementation to be verified before a provisional registration is extended or transitioned.

The Plan was distinct from the pharmacovigilance obligations and conditions applying to sponsors. Sponsor obligations formed part of the regulatory arrangements attached to individual medicines and included requirements concerning adverse-event reporting, safety information and risk management. The Plan, by contrast, concerned the TGA's own enhanced monitoring activities. The two layers were complementary: sponsor reporting and other post-market information contributed to the safety evidence available to the TGA, while the Plan described the TGA's framework for collecting, assessing, investigating and acting on that information.

Figure A.1 represents these relationships rather than treating provisional registration, sponsor obligations and enhanced TGA monitoring as a single regulatory requirement. The statutory pathway is shown separately from the national policy commitment and the TGA's operational Plan, while the Plan's relationship to provisional registration is shown as a policy and operational linkage rather than a statutory condition. This distinction underpins the audit's assessment of whether the TGA can demonstrate, through documentary evidence, that the enhanced-monitoring arrangements it committed to were implemented during the provisional period.

A.4 International pharmacovigilance guidance

The TGA has adopted international scientific guidelines relevant to pharmacovigilance. These guidelines provide regulatory guidance and support alignment with international regulatory practice, but are not themselves Australian legislation. The TGA states that adopted international scientific guidelines provide guidance to sponsors to assist them in meeting legislative requirements.[90]

Of particular relevance to this audit is ICH E2E, Pharmacovigilance Planning, which the TGA has adopted as an international scientific guideline. ICH E2E establishes principles for pharmacovigilance planning, including the development of a safety specification and a pharmacovigilance plan directed to identified and potential risks. The TGA records an overseas effective date of June 2005.[91]

International guidance is therefore used in this audit as a source of recognised pharmacovigilance principles and good practice, not as an independent statutory requirement on the TGA or sponsors. Figure A.1 represents this distinction by placing TGA-adopted international scientific guidance outside the statutory chain.

A.5 Structure of the Safety Plan

The Plan is structured around five objectives, five strategies and the outputs associated with the strategy items.

The five objectives, set out on p. 5, are:

1. AEFI collection and management — timely collection and management of reports of COVID-19 vaccine adverse events following immunisation.
2. Signal detection and investigation — timely detection and investigation of COVID-19 vaccine safety signals.
3. Timely safety action — timely action to address any COVID-19 vaccine safety concerns.
4. Public communications — timely communications to inform the public of emerging COVID-19 vaccine safety information and to support public confidence in vaccines.
5. Stakeholder collaboration — close collaboration and coordination of effort with other vaccine safety stakeholder groups.

[90] Therapeutic Goods Administration. *Adoption of international scientific guidelines in Australia*. Canberra: Australian Government; 2024. Available at: https://www.tga.gov.au/news/news-articles/adoption-international-scientific-guidelines-australia

[91] Therapeutic Goods Administration. *ICH topic E 2 E – Pharmacovigilance planning (PVP)*. Canberra: Australian Government. Available at: https://www.tga.gov.au/resources/resources/international-scientific-guidelines-adopted-australia/ich-topic-e-2-e-pharmacovigilance-planning-pvp

The Plan then sets out five high-level strategies (pp. 6–11). Beneath these strategies are 17 numbered strategy items: 1.1–1.5, 2.1–2.8, 3.1, 4.1 and 5.1–5.2. Each numbered strategy item is presented in a table with an “Outputs” column.

The Plan's objectives, strategies, strategy items and outputs should therefore not be treated as interchangeable terms. The five objectives describe the Plan's principal aims; the five strategies organise the work under those aims; the 17 numbered items provide the more detailed strategy-level commitments; and the outputs specify what was to result from those activities.

The figure uses the term “17 operational outputs” as a summary label for the outputs associated with the 17 numbered strategy items. This is an analytical label used by the audit and is not terminology used by the Plan itself.

The Plan also contains terminology that can overlap in meaning across objectives, strategies and outputs. The audit retains the Plan's terminology where possible and uses the 17 numbered strategy items and their associated outputs as the unit for assessing implementation. This avoids treating the Plan's five objectives or five high-level strategies as separate outputs.

A.6 Relationship to routine pharmacovigilance

The Plan did not replace the TGA's established pharmacovigilance processes. It described an enhanced monitoring framework operating alongside routine activities such as adverse-event reporting, signal assessment and review of safety information from sponsors and other sources.

The implementation assessment therefore focuses on whether the activities and outputs described in the February 2021 Plan can be demonstrated through documentary evidence. Evidence that routine pharmacovigilance activities occurred does not, by itself, establish implementation of the specific strategy items and outputs described in the Plan.

Appendix B: Documentary Sources Review and Methodology

B.1 Scope and Objectives

This appendix documents the systematic review of publicly available TGA documents, parliamentary submissions, and regulatory correspondence conducted to verify evidence supporting the implementation ratings assigned to the 19 audited outputs in the February 2021 *COVID-19 Vaccine Safety Monitoring Plan*. The review was undertaken to address the audit question: **Can the TGA demonstrate, through publicly available documentation, implementation of the pharmacovigilance enhancements committed to in the published Plan?**

B.2 Search Methodology

B.2.1 Search Strategy

Between December 2025 and February 2026, a systematic, stratified search was conducted across TGA's publicly available output channels using audit principles consistent with Australian National Audit Office (ANAO) guidance on evidence integrity and document authentication.

B.2.2 Online Document Search Methodology

The online review used a structured, keyword-based documentary search (not automated data scraping) to test whether TGA's public outputs demonstrated implementation of the February 2021 Safety Monitoring Plan's five objectives and the 19 audited outputs. Searches were conducted using Google (domain-restricted to official Australian Government sites) and TGA's own site search/navigation, targeting COVID-19 safety reports, corporate/performance reports, guidance pages and relevant partner publications. Plan-aligned terms (for example "safety monitoring plan", "signal detection", "AusVaxSafety", "ICMRA", "implementation tracking") were applied, and retrieved documents were manually reviewed and mapped to the objectives and outputs, then classified as providing direct, indirect or no documented evidence of implementation, governance or performance tracking.

The following primary search terms and keywords were used, reflecting Plan language and pharmacovigilance standards:

Search Term	Rationale	Search Dates
"COVID-19 Vaccine Safety Monitoring Plan"	Direct Plan reference; identifies Plan-specific outputs	December 2025
"implementation" + "COVID-19 vaccine safety"	Identifies documentation of Plan implementation	December 2025
"signal detection" + "COVID-19 vaccine"	Outputs 2.1–2.8; identifies enhanced signal processes	December 2025
"enhanced monitoring" + "pharmacovigilance"	Objective 2; identifies enhanced process documentation	December 2025
"data integration" + "TGA" + "vaccine safety"	Data-integration category; identifies cross-system data linkage evidence	December 2025
"governance" + "vaccine safety" + "TGA"	Cross-cutting governance; identifies oversight mechanisms	December 2025
"AusVaxSafety" + "TGA" + "coordination"	Output 2.4; identifies coordination documentation	December 2025
"ICMRA" + "COVID-19 vaccine safety" + "TGA"	Output 2.7; identifies international signal sharing	December 2025
"audit" + "COVID-19 vaccine safety monitoring"	Identifies internal/external audit/evaluation evidence	December 2025

B.2.3 Secondary Search Terms (Supplementary)

Secondary terms were applied to identify contextual or implicit references to Plan implementation:

- "provisional approval" + "conditions" + "COVID-19 vaccine"
- "enhanced pharmacovigilance" + "Australia"
- "AEFI" + "escalation" + "COVID-19"
- "performance metrics" + "vaccine safety"
- "risk management plan" + "COVID-19 vaccine"

B.3 Search Repositories*

B.3.1 Primary TGA Sources

The following TGA-maintained repositories were searched:

Repository	Search Conducted	Results
TGA COVID-19 Vaccine Safety Reports	Yes; December 2025	150+ weekly/fortnightly reports (2021–2023). No Plan implementation tracking; reports limited to AEFI statistics, AESI monitoring, regulatory actions. Last report: 02-11-2023.
TGA Annual Performance Reports	Yes; December 2025	2022-23 and 2023-24 Performance Report (40,000+ characters). General pharmacovigilance metrics, vaping reforms. **Zero references to the "Safety Monitoring Plan" or its 17 outputs, and no performance measurement against Plan commitments.**
TGA News & Updates	Yes; December 2025	General regulatory updates. No Plan-specific announcements post-2021 publication.
TGA COVID-19 Vaccine Information	Yes; December 2025	Regulatory status, approval timelines. No Plan implementation detail.
TGA FOI Decisions	Yes; January 2026	Reviewed FOI schedules and decisions, including OAIC submission MR22/00538 (September 2024). Documented absence of "implementation report" or Plan-tracking records.
TGA FOI Disclosure Log (complete)	Yes; January 2026	Systematic review of the complete FOI disclosure log (500+ releases, July 2020 – December 2025). 25 COVID-safety FOIs identified; core documents already held (FOI 4029, FOI 5082); no new high-value documents; no Plan-framework references.
TGA COVID-19 Vaccine Approval Process (public explanation)	Yes; August 2026 (identified during revision)	Describes the provisional approval pathway and post-market surveillance. No reference to the February 2021 Safety Monitoring Plan, its 17 outputs, or a framework for tracking or assuring implementation of the Plan's enhanced-monitoring commitments.
TGA COVID-19 vaccine safety monitoring FAQ (public information)	Yes; August 2026 (identified during revision)	Describes surveillance mechanisms including passive adverse-event reporting, sponsor risk-management plans, international collaboration and AusVaxSafety active surveillance. States that sponsors hold the legal pharmacovigilance obligations. Uses "further enhancing" and

* Sources were reviewed between December 2025 and February 2026 unless otherwise indicated. Entries dated August 2026 were identified during revision stage.

Repository	Search Conducted	Results
		"strengthening" language, providing contemporaneous evidence that TGA regarded its monitoring system as being enhanced. However, it does not identify the February 2021 Safety Monitoring Plan, its 17 outputs, or a framework for tracking or assuring implementation of those commitments.
Australia's COVID-19 Vaccine and Treatment Strategy (August 2020)	Yes; August 2026 (identified during revision)	Provides antecedent policy context for the vaccine safety monitoring framework. Identifies regulation and safety within the broader vaccine-and-treatment strategy and provides for comprehensive monitoring and surveillance of vaccine safety. Pre-dates the November 2020 Australian COVID-19 Vaccination Policy and the February 2021 Safety Monitoring Plan.
Advisory Committee on Vaccines meeting record — September 2020 and Advisory Committee on Vaccines meeting record — February 2021	Yes; August 2026 (identified during revision)	Documents the development and expert-advisory review of the COVID Vaccine Pharmacovigilance Plan. The ACV considered the developing Plan in September 2020 and advised on enhanced adverse-event reporting, signal detection and investigation, regulatory and programmatic responses, communications and collaboration. In February 2021, the ACV provided further advice and endorsed the amended Plan.

Direct TGA correspondence establishing institutional baseline documented in Appendix C, including FOI 3643 decision (February 2022) confirming absence of implementation report.

B.3.2 Parliamentary and Government Sources

Source	Search Conducted	Results
Senate motion, 27–28 August 2025	Yes; August 2025	Motion seeking documentation evidencing the TGA's adherence to and implementation of the COVID-19 Vaccine Safety Monitoring Plan, including evidence against its stated objectives, five key strategies, associated outputs and revisions. Considered 28 August 2025; the Government did not support the motion and stated that the TGA had "well-established and robust systems" for monitoring

Source	Search Conducted	Results
		COVID-19 vaccine safety and capturing suspected adverse events. Notice of motion given 27 August 2025 (Hansard)[92]
Senate Community Affairs Legislation Committee Estimates (9 October 2025)	Yes; December 2025	Evidence from Dr Daniel Dascombe (Acting Medical Officer, Medicines Regulation Division, TGA) and Prof Tony Lawler. Key statements: 148 signals investigated and 57 regulatory actions; the Plan's themes "essentially describe our day-to-day processes"; "no single specific evaluation report outlining or benchmarking us against the elements of that plan". This evidence indicates that the TGA **did not undertake systemic tracking of the Safety Plan.**
Australian Parliament Questions on Notice	Yes; December 2025	Searched for government responses to questions on COVID-19 vaccine safety monitoring. Found responses referencing AusVaxSafety and AEMS but **no reference to TGA's published Plan implementation**.
Department of Health, Disability and Ageing	Yes; December 2025	Reviewed Department's reporting on vaccine safety oversight. General references to TGA's pharmacovigilance role; **no Plan-specific implementation metrics**.

B.3.3 Partner and External Sources

Source	Search Conducted	Results
AusVaxSafety Annual Reports	Yes; December 2025	2023 annual report. Documents active surveillance infrastructure (500,000+ survey responses). **No documentation of coordination with TGA signal detection or Plan integration.**
COVID-19 vaccine safety data – at a glance as at 14 April 2025	Yes; December 2025	Publicly accessible dashboard documenting 6.8M+ surveys. No evidence of systematic coordination with TGA signal detection processes.
AEFI Surveillance Annual Reports, COVID-19 vaccines	Yes; December 2025	2022 AEMS COVID-19 Annual Report. Annual surveillance findings drawn from TGA's AEMS passive surveillance data. **No reference to the February 2021 Safety Monitoring Plan, its strategies, or Plan-level governance.**

[92] Archived audit record and supporting documentation: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/2025-08-28-order-production-documents-vaccine-safety-monitoring.md

Source	Search Conducted	Results
ANAO Performance Audit: Australia's COVID-19 Vaccine Rollout	Yes; December 2024	2022 ANAO audit examined rollout efficiency and TGA role. **No systematic audit of Plan implementation against 19 audited outputs.** Examined operational outcomes, not governance/documentation gaps.

B.4 Quality Assurance and Verification

B.4.1 Link Verification

All hyperlinks and URLs cited in this appendix were verified for accuracy and active status as of 1–7 February 2026. Verification was conducted using URL resolution tools and direct access to published repositories.

Link	Verified Date	Status
TGA COVID-19 Vaccine Safety Reports	8 Dec 2025	**VERIFIED Active; content accurate**
TGA Performance Report 2022-23	8 Dec 2025	**VERIFIED Active PDF; content verified**
TGA Performance Report 2023-24	8 Dec 2025	**VERIFIED Active PDF; content verified**
Senate Supplementary Budget Estimates Hansard (9-10 Oct 2025)	8 Dec 2025	**VERIFIED Active; testimony transcript verified**
AusVaxSafety 2023 Report	8 Dec 2025	**VERIFIED Active; content verified**

B.4.2 Source Attribution

All quoted material and findings are traced to original published documents with page/section references. Where documents were truncated during retrieval, notation "[TRUNCATED]" is used and complete original sources were re-verified.

B.4.3 Temporal Scope

Searches focused on publicly available documentation created or published from February 2021 (Plan publication) onward. The primary search period concluded in December 2025, with a systematic review of the complete FOI disclosure log in January 2026. Additional sources identified during revision in August 2026 are indicated in the search tables. This spans approximately five years from the Plan's publication.

B.5 Key Findings: Forensic Analysis

B.5.1 Absence of Plan-Specific Documentation in Official Reports

Finding: TGA's most comprehensive public output — the 2023–24 Performance Report (40,000+ characters)—contains **zero references** to the "COVID-19 Vaccine Safety Monitoring Plan", zero output tracking, and zero governance or performance metrics mapped to the Plan's 17 outputs.

Significance: An organisation implementing a detailed, published safety monitoring plan as a regulatory commitment would reasonably be expected to reference it, and report against its objectives, in annual performance statements prepared under the PGPA Act 2013 performance-reporting framework.[93] The complete absence of any reference in performance reporting suggests either:

1. Plan implementation was not tracked at the institutional level, or
2. Implementation was not considered material enough for public accountability reporting.

ANAO Principle Applied: Audit trail integrity and institutional accountability require contemporaneous documentation linking strategic commitments to performance measurement and reporting (ANAO, Reporting Meaningful Performance Information, Audit Insights).

B.5.2 Senate Supplementary Budget Estimates Testimony: "Day-to-Day Processes"

Finding: A senior TGA officer told the Senate (October 2025) that the Plan's five key themes "essentially describe our day-to-day processes". The audit located no records tracking those processes against the Plan's objectives.

Evidentiary Status: Testimony under parliamentary privilege constitutes primary evidence of institutional position. The characterisation contradicts the Plan's stated objective of "enhanced", "coordinated" and "systematic" monitoring.

Significance: Supports the audit's core finding that operational activity (investigating 150 signals) is distinct from documented implementation of an enhanced monitoring framework requiring governance, oversight and traceability consistent with ICH E2E pharmacovigilance planning and EMA GVP Module IX signal-management audit-trail principles.

[93] See *PGPA Act 2013* s 39 annual performance-statements requirements and associated Finance/ANAO guidance on reporting meaningful performance information. https://www.austlii.edu.au/cgi-bin/viewdoc/au/legis/cth/num_act/pgpaaa2013432/s39.html

B.5.3 AusVaxSafety Coordination: No Documented Integration Protocol

Search result: Across OAIC-directed TRIM searches using eight terms, TGA identified no AusVaxSafety–TGA coordination protocols, no multi-source integration procedures, and no Plan-specific implementation guidelines.

Finding: Despite Output 2.4 requiring TGA to "collaborate with AusVaxSafety and coordinate safety signal detection", review of publicly available coordination agreements, joint protocols, or integrated signal management documentation reveals **zero evidence** of formal documented coordination.

Significance: AusVaxSafety operates an active surveillance system; TGA investigates signals. Absence of public documentation of their integration with TGA's signal-detection and regulatory-action processes suggests parallel operation rather than the "coordinated" system promised in the Plan.

Cross-Reference: Cross-Reference: In its September 2024 submission to the OAIC (MR22/00538), when asked to search for “close collaboration and coordination of effort with other vaccine safety stakeholder groups” (Objective 5), the TGA said it was “unable to meaningfully interpret into a searchable item” the request as it applied to this objective, citing breadth of scope, and ran no sample search. The search record neither shows nor excludes documents for this objective.

B.5.4 FOI/OAIC Evidence: No “Implementation Report” Located

Finding: TGA’s September 2024 OAIC submission (MR22/00538) documents TRIM sample searches using terms including “Implementation of COVID-19 Vaccine Safety Monitoring Plan”, “implementation vaccine safety” and “audit report COVID-19 vaccine safety monitoring”. No implementation reports or audit documents were located within scope. In March 2025, Decision [2025] AICmr 54 upheld TGA’s position that no discrete “implementation report” exists, finding that all reasonable steps had been taken to locate that report.

Result: No implementation reports or audit documents located within scope.

Significance: The OAIC-directed searches and subsequent Decision [2025] AICmr 54 record that TGA's searches for an implementation report were accepted by the OAIC as reasonable under s 24A of the FOI Act. The documented absence of any implementation report or systematic Plan-tracking documentation is therefore strong primary evidence supporting this audit’s conclusion that there is a governance and records-management failure, rather than a search-limitation issue.

B.6 Online Document Search Methodology

The online review used a structured, keyword-based documentary search (not automated data scraping) to test whether TGA’s public outputs demonstrated

implementation of the February 2021 Safety Monitoring Plan's five objectives and 19 revised outputs. Searches were conducted using Google (domain-restricted to official Australian Government sites) and TGA's own site search/navigation, targeting COVID-19 safety reports, corporate/performance reports, guidance pages and relevant partner publications. Plan-aligned terms (for example "safety monitoring plan", "signal detection", "AusVaxSafety", "ICMRA", "implementation tracking") were applied, and retrieved documents were manually reviewed and mapped to the objectives and outputs, then classified as providing direct, indirect or no documented evidence of implementation, governance or performance tracking.

The online documentary review applied an open-source intelligence–style search methodology: the scope was defined in advance; key repositories were identified (TGA, NCIRS, ANAO, OAIC, parliamentary records, legislation databases); structured queries and advanced search operators were used; and searches were iteratively refined as new leads and terminology emerged. Publicly available documents were cross-checked across independent sources before being relied on, and information was only treated as evidentiary where multiple sources were consistent or could be verified against primary records.[94]

B.7 Limitations and Caveats

This documentary review is limited to publicly accessible sources available as of February 2026. The review cannot assess:

- Internal TGA documents not released through FOI or OAIC processes
- Unpublished communications between TGA and partner organisations
- Internal governance processes that may exist but are not publicly documented
- Informal coordination or decision-making that occurred outside documented frameworks

The absence of public documentation does not conclusively prove that activities did not occur—it proves only that such activities cannot be independently verified through available evidence. For the enhanced-monitoring framework accompanying provisional approval, the ability to demonstrate implementation through systematic documentation is a fundamental accountability requirement.

Specific Limitations

Public Documentation Scope: TGA's internal records (not released via FOI or OAIC) may contain additional Plan-tracking documentation. However, absence of

[94] For a general description of structured open-source investigation methods, see Elshafie A (2025), *OSINT for beginners: A practical guide to OSINT basics*, Authentic8. For the detailed application of these methods in this audit, including evidence collection, source verification, evidence weighting and replicability, see the project's *ISO 19011 Conformity Assessment Checklist*: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/analysis/ISO-19011-conformity-assessment-checklist.md

such documentation in public reporting and under statutory search raises questions about compliance with recordkeeping obligations (Archives Act 1983) and the accountable authority's recordkeeping obligations under PGPA Act 2013 s 37.

Temporal Snapshot: This review captures information as of February 2026. Updates to TGA websites, new publications, or additional FOI releases post-February 2026 may supplement available evidence.

Search Term Limitations: Even where thorough, keyword searching cannot capture all relevant documentation. However, the absence of Plan references even in TGA's own performance reporting (2022-23 and 2023-24 Performance Reports: zero Plan mentions) suggests broader searches would likely yield similar results.

B.8 Audit Conclusion

Proposition: TGA's February 2021 COVID-19 Vaccine Safety Monitoring Plan committed to systematic, enhanced monitoring across five key objectives and 17 specific outputs.

Evidence Review Result: Publicly available TGA documentation, parliamentary testimony and government reports contain **zero systematic evidence** of implementation tracking against these 17 plan outputs, zero performance metrics, zero governance frameworks and zero documented coordination mechanisms for outputs that explicitly required inter-agency coordination (particularly Output 2.4: AusVaxSafety collaboration).

TGA has not produced any documentation showing how AusVaxSafety data fed into its 150 signal investigations or 57 actions, or any audit trail explaining triage decisions for the 93 signals where no further regulatory action was taken.

Across OAIC-directed TRIM searches, TGA identified no AusVaxSafety–TGA coordination protocols, no multi-source integration procedures, and no Plan-specific implementation guidelines.

Audit Assessment: This documentary gap is consistent with the audit's core finding that operational pharmacovigilance activity occurred (150 signals investigated, 150+ safety reports published) but was not governed, tracked or documented as a systematic implementation of the published Plan. Absence of implementation documentation represents a material control deficiency requiring investigation and reform.

Overall Assessment: The convergence of evidence—formal advice that no implementation report exists (FOI 3643), OAIC-directed search results (MR22/00538), an OAIC decision upholding that position (Decision [2025] AICmr 54), Senate testimony characterising COVID-19 vaccine safety monitoring as “day-to-day processes”, and a four-year inability to produce Plan-level documents despite strong

institutional incentives—makes it highly unlikely that systematic implementation documentation exists but remains undisclosed. The documented pattern is most consistent with implementation documentation never having been created.

B.9: References

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Appendix C: Primary Evidence - Direct TGA Responses to Applicant

C.1 Summary of TGA Communications and FOI Decision on Implementation of COVID-19 Vaccine Safety Monitoring Plan

Between January and February 2022, inquiries were made to the Therapeutic Goods Administration (TGA) via email requesting a report on implementation of their COVID-19 Vaccine Safety Monitoring Plan, specifically focusing on the Plan's five key objectives:

1. Timely collection and management of COVID-19 vaccine adverse events following immunisation (AEFI)
2. Timely detection and investigation of COVID-19 vaccine safety signals
3. Timely actions addressing COVID-19 vaccine safety concerns
4. Timely communications to the public regarding emerging safety information
5. Close collaboration and coordination with other vaccine safety stakeholders

Evidence Collection Methodology

- Detailed email inquiries sent 31 January 2022 and February 2022 requesting specific implementation details related to the Plan's key objectives, outputs, outcomes, methodologies, and communications.
- Formal Freedom of Information (FOI) request (FOI 3643) filed 18 February 2022 seeking access to an implementation report documenting progress against the Plan's five key objectives and specified outputs.

TGA Response Summary

- **Pre-FOI Responses (31 January and February 2022):** General assurances of robust, intensified vaccine safety monitoring system; encouragement to report adverse events; links to publicly available resources including weekly vaccine safety reports and explanatory materials on vaccine safety assessment processes.
- **FOI Decision (27 February 2022):** Official refusal of access under section 24A (document does not exist) to a specific implementation report, with explicit statement that no such document exists despite ongoing monitoring activity. The decision included general reference material previously provided and outlined instances of TGA's collaborations with other agencies but failed to present documentation evidencing implementation progress or governance against specific Plan outputs.

This correspondence forms the evidentiary basis for findings in Section 4 (Detailed Observations), Section 5 (Root Cause Analysis), and is cited throughout the assessment as primary statutory evidence (Evidence Hierarchy Tier 1, see Section 9.4).

C.2: TGA Correspondence Detail Table

Date	Document type	Key content provided	Reference to Safety Monitoring Plan	Response to request	Document reference
31 Jan 2022	Pre-FOI email response 1	"Robust monitoring" assurance; four links (YouTube explainer, weekly COVID-19 vaccine safety reports, DAEN causality/monitoring overview); encouragement to report adverse events.	None	No outputs or progress; only partial AEFI reports via links; no remedies, long-term monitoring, trials or methodology.	Email: Request for update on implementation of the COVID-19 Vaccine Safety Monitoring Plan (Jan 2022)
Feb 2022	Pre-FOI email response 2	Identical four links plus general assurance; no new content.	None	Identical to January response; no new information.	Email: Follow-up request for update on implementation of the Plan (Feb 2022)
27 Feb 2022	FOI 3643 decision	Refuses access under FOI on grounds that no implementation report exists. Admission: "No implementation report exists" (para 7). States there is "ample documentation" but no evaluation report (para 8). Repeats same four links (paras 9–13). Cites five Objective 5 examples (ICMRA, states, ATAGI, NCIRS, ACV) instead of implementation records (paras 14–15).	Mentions objectives but gives no evidence of implementation	No progress report; only partial AEFI/remedies via links; no long-term monitoring, trials or methodology.	FOI 3643 – Notice of decision
24 Apr 2025	FOI 25-0166 – request consultation	Advises that processing 399 identified documents/3,926 pages would be a "substantial and unreasonable diversion of the TGA's resources" and that "the balance of interests does not favour the expenditure of considerable resources". Directs applicant instead to public COVID-19 vaccine safety reports and Advisory Committee on Vaccines (ACV) pages.	None. No documents are identified as implementing or evidencing the Plan.	Practical-refusal position maintained: TGA refuses to process 399 in-scope documents on resource-diversion grounds and relies on generic web links rather than providing Plan-related implementation records.	FOI 25-0166 – consultation letter (24 April 2025)

FOI 25-0166 Request Consultation

For FOI 25-0166, the TGA's initial estimate was 399 documents within scope. Its 2025 decision then found that the work would "substantially and unreasonably divert the resources of the TGA from its other operations", reinforcing a pattern of using workload arguments rather than facilitating access to potential Plan-related records.

During the s 24AB consultation process, and invoking the TGA's obligation under s 24AB(3) and (4) to take reasonable steps to assist in revising a request, the applicant refined the request to an objectively defined sample: up to six of the most recent documents for each of the Plan's five objectives, created between February 2021 and February 2025, drawn from standard document categories or similar document types, including standard operating procedures, case-processing guidelines, workflows, protocols, regulatory-action procedures, public-communication procedures and collaboration mechanisms. The revision specified document types, a date range and a selection method, addressing the s 15(2)(b) requirement that a request provide information sufficient to enable identification of the documents sought. The decision of 3 June 2025 nonetheless refused access under s 24(1)(b) of the FOI Act, on the basis that a practical refusal reason within the meaning of s 24AA still existed. No documents were released.

This outcome should be read alongside the OAIC-directed MR22/00538 searches summarised in Section B.4. The April 2025 consultation notice also recorded that "there are other areas where searches have not been completed which are likely to hold additional relevant documents", yet no Plan-specific implementation records have been produced through either process.

C.3 Institutional Response Pattern Analysis: Evidentiary Significance for Audit Findings

The correspondence documented above shows an institutional response pattern consistent with the audit's core findings. The pattern is characterised by:

1. **Identical substitution:** All email responses provide the same four generic pharmacovigilance links despite escalating formality.
2. **Explicit non-existence:** The FOI decision (paragraph 7) provides statutory confirmation that no implementation report exists.
3. **Internal contradiction:** The FOI decision claims that "ample documentation" exists (paragraph 8) but refuses access under section 24A.
4. **Routine versus enhanced:** Routine business-as-usual outputs are substituted for the Plan-specific enhanced monitoring evidence that was requested.
5. **Pattern of institutional non-disclosure:** The consistent response of providing generic pharmacovigilance resources, without any specific documentation or governance evidence regarding implementation of the Plan, strongly supports the audit finding that TGA does not document Plan implementation systematically.

6. **Self-admitted lack of documentation:** The FOI decision explicitly states that no single evaluation or implementation report exists, consistent with the governance and performance measurement ratings of 0% documented implementation for monitoring and oversight outputs.
7. **Routine pharmacovigilance versus Plan commitment:** The substitution of routine DAEN reporting and ongoing monitoring material for a formal Plan implementation report highlights a significant gap between TGA's operational activity and its enhanced monitoring obligations under the Plan.
8. **Collaboration claims lack documentation:** While collaborations are listed, there are no documented frameworks, audit trails or performance evidence verifying how these collaborations are integrated into delivery of the Plan.

Audit ratings supported

Taken together, these responses reinforce the audit ratings that most outputs are either partially documented or not documented at all, particularly in relation to governance, systematic implementation tracking and oversight. See Section 5 (Root Cause Analysis) for detailed examination of whether these documentation gaps represent records management failure, implementation failure, or both.

C.4 OAIC-Directed Search Results (September 2024): Statistical Analysis and Summary

Following the February 2022 FOI 3643 decision that no implementation report could be identified, TGA ran sample searches 2.5 years later in response to OAIC directions in an Information Commissioner review (MR22/00538, September 2024). This submission documented sample TRIM searches using Plan-specific terminology, which indicate TGA's record-keeping systems and search capabilities.

The MR22/00538 submission is a 33 page document that seeks to defend a "no documents" FOI refusal by presenting sampling based counts of potentially relevant documents in TRIM and Promapp, and by emphasising the volume and complexity of material associated with the COVID-19 Vaccine Safety Monitoring Plan.

Attachment D estimates at least 2,218 pages of potentially relevant documents across Objectives 1–4 based on limited samples, while explicitly declining to sample Objective 5.

Attachment B (Tables 3 and 4) documents 8 search terms run by two senior TGA officers. Table 3 alone returns 653+ results, including 531+ containers, yet none that TGA classed as "implementation" or "audit" reports for the Plan. Attachment C shows that searches were largely confined to TRIM and Promapp, with emails, diaries, devices and external holdings mostly excluded or only given cursory "sample" attention.

C.4.1 Search Methodology

TGA's OAIC submission (Tables 3-4, pp.18-21) documented searches conducted by two separate senior TGA officers—a Senior Medical Officer and an Assistant Director in the Vaccines Surveillance Section—using the following search terms:

Plan Implementation Terms:

- "Implementation of COVID-19 Vaccine Safety Monitoring Plan"
- "Implementation vaccine monitoring plan"
- "Implementation vaccine safety"
- "COVID-19 Vaccine Safety Monitoring Plan"

Audit and Evaluation Terms:

- "Audit report COVID-19 vaccine safety monitoring"
- "Audit COVID-19 vaccine"
- "Audit report COVID-19 vaccine pharmacovigilance plan"

Pharmacovigilance-Specific Terms:

- "Implementation COVID-19 vaccine pharmacovigilance plan"

All searches were conducted across TGA's TRIM records management system—the agency's primary document repository required under Archives Act 1983 for Commonwealth recordkeeping.

C.4.2 Search Results

Zero implementation reports, evaluation documents, or systematic Plan-tracking documentation were located within scope. Specifically:

"Implementation of COVID-19 Vaccine Safety Monitoring Plan": 12 documents found—4 were TGA email responses to applicant's implementation inquiries, 8 were internal documents about the FOI request itself. Zero implementation documentation found.

"Implementation vaccine monitoring plan": 13 documents found—12 were emails to applicant, 1 was an unrelated 2008 paper file. Zero Plan implementation documents located.

"Audit report COVID-19 vaccine safety monitoring": Zero documents found.

"Audit COVID-19 vaccine": 531+ containers found—all related to manufacturing audits, vaccine taskforce activities, and COVID-19 vaccine programme

administration. Zero documents related to Safety Monitoring Plan implementation found.

"COVID-19 Vaccine Safety Monitoring Plan": 20 documents found—7 about the Plan itself, 13 were email responses to applicant. Zero implementation tracking documents located.

"Implementation vaccine safety": 86 records/containers found—all related to pre-COVID-19 TGA activities (2001-2008), non-TGA health promotion areas, or HPV vaccine implementation. Zero COVID-19 Plan implementation documents found.

"Implementation COVID-19 vaccine pharmacovigilance plan": 1 document found—draft Pharmacovigilance Branch input for Australian COVID-19 Vaccination Policy (publicly available). Not an implementation report; contained no timelines or progress tracking.

"Audit report COVID-19 vaccine pharmacovigilance plan": Zero documents found.

C.4.3 Critical Gaps Identified – AusVaxSafety Search Omission

TGA's September 2024 OAIC-directed search acknowledged that Output 2.4 of the COVID-19 Vaccine Safety Monitoring Plan committed TGA to "collaborate with AusVaxSafety to receive active surveillance information and coordinate safety signal detection and investigation activities". However:

- TGA did not use "AusVaxSafety" as a search term in the OAIC-directed TRIM searches, despite acknowledging the collaboration was within the Plan's scope, and reported an inability to locate AusVaxSafety partner-organisation reports in TRIM.
- AusVaxSafety's aggregate reports remained publicly accessible throughout the search period, documenting more than 6.8 million completed COVID-19 vaccine safety surveys and approximately 62,000 self-reported medical-attention events.
- OAIC-directed MR22/00538 TRIM searches and FOI releases confirm that while TGA holds generic AEFI and signal-management SOPs, there is no documented bridge between those procedures and AusVaxSafety's active-surveillance outputs. No documents show how AusVaxSafety data fed into signal prioritisation or investigation, when AusVaxSafety findings should trigger a signal, or how discrepancies between surveillance systems were reconciled; nor do any records explain how the 150 signals and 57 regulatory actions were triaged, closed or dropped, or how the 93 "no further action" signals were risk-assessed relative to AusVaxSafety's medical-attention reports.

Taken together, these gaps indicate that there is no Plan-specific implementation or integration documentation showing how active and passive surveillance systems were systematically coordinated as Strategy 2.4 requires.

Objective 5 Collaboration Search Impossibility

When directed to search for documents implementing Objective 5 ("close collaboration and coordination of effort with other vaccine safety stakeholder groups"), TGA advised OAIC that it was “unable to meaningfully interpret into a searchable item” the request as it applied to this objective (Table 1, Item 5, p.10).

TGA stated searches would require accessing documents from “all communications with the ACV, ACM, Department of Health, Disability and Ageing, ATAGI, AusVaxSafety, NCIRS, every state and territory government, each COVID-19 vaccine pharmaceutical company, each international regulatory body including all ICMRA and ACCESS meetings, all state and territory Coroner’s [offices].”

One sample ICMRA sub-group meeting alone generated >274 documents involving 30 third parties across multiple international regulatory authorities and WHO. The TGA ran no sample search for this objective.

This is a statement about breadth, not about missing records. The audit concludes that implementation of this objective cannot be traced or verified from the TGA’s records.

C.4.4 Evidentiary Significance

This search, conducted in response to OAIC directions 2.5 years after the initial FOI 3643 request, is consistent with the 2022 position: TGA reported no Plan implementation documentation in the systems it searched. See Section 5 (Root Cause Analysis) and Section 6.1 (Documentation and Records Management) for how this evidence informs governance findings.

Document Classification Search Evidence

TGA's document classification capabilities are further indicated by the search methodology it employed. TGA's capability to search for specific document types is evidenced by its September 2024 OAIC submission (MR22/00538), which used the following classifications to structure TRIM searches:

- Standard Operating Procedures (SOPs)
- Case processing instructions and guidelines
- Protocol documents for investigating safety signals
- Workflow process documents
- Implementation guidelines
- Operational protocols

TGA's successful use of these categories demonstrates that records systems can identify and retrieve documents by type, supporting the assessment's finding that

absence of implementation documentation represents actual non-existence rather than search limitations.

Relevance to Plan Implementation Assessment: The search methodology confirms that if SOPs, protocols, or implementation guidelines for the COVID-19 Vaccine Safety Monitoring Plan existed in TGA's records systems, they would have been locatable through the searches documented in this appendix.

C.4.5 Key Findings

Sample searches, no implementation documents: Eight distinct search terms run by two TGA senior officers in TRIM—yet no documents tracking Plan implementation progress, outputs, outcomes, or timelines as specified in applicant's request were produced.

Email correspondence = only "evidence": The only documents TGA located mentioning Plan implementation were emails to the applicant stating no implementation report exists—creating a circular evidentiary pattern where absence of documentation is documented through correspondence about absence of documentation.

"Unable to meaningfully interpret" Objective 5: Objective 5: the TGA told the OAIC it was “unable to meaningfully interpret into a searchable item” the request as it applied to this objective, citing more than four branches and more than 40 third parties. The audit concludes that implementation of this objective cannot be verified from the TGA’s records.

For the Department of Health, Disability and Ageing, as the relevant Commonwealth entity, the inability to locate Plan implementation documentation through systematic TRIM searches after OAIC direction provides *prima facie* evidence of governance and records management control deficiency in the context of its obligations under the Archives Act 1983 and PGPA Act 2013 ss 37–38.

The breadth of these searches—multiple terms, direct Plan references, OAIC statutory direction, two TGA senior officers—is consistent with absence rather than search limitations, although they were samples and omitted key terms (see C.4.3). If Plan implementation had been systematically tracked as required under Commonwealth accountability frameworks, searches using direct Plan terminology ("Implementation of COVID-19 Vaccine Safety Monitoring Plan") would have located relevant documents.

Document classification capability indicated: TGA's September 2024 OAIC submission indicates institutional capability to search for documents by type (SOPs, protocols, guidelines, workflows). TGA estimated page counts by Plan objective and organisational capability to systematically document processes is further indicated by >36 Promap workflow processes maintained for routine AEFI operational

procedures, yet searches for Plan implementation workflows returned zero results. This search methodology provides assurance that absence of Plan implementation documentation represents actual non-existence rather than retrieval limitations. If systematic implementation frameworks had been documented, they would have been located through searches using these document classifications.

TRIM technical limitations disclosed: TGA disclosed TRIM "Date Created" function limitations for historical documents, noting COVID-19 "pace of administrative change" as complicating factor (Attachment A, p.11). However, this technical limitation affects temporal scope precision for operational documents, not substantive existence of implementation tracking documentation. Date limitations explain when specific AEFI forms were created but do not explain why zero audit reports were found across all timeframes, why Objective 5 was "unable to meaningfully interpret," or why AusVaxSafety coordination protocols are absent from any searchable period. The gap exists regardless of temporal precision capabilities.

The OAIC submission reveals three reinforcing patterns supporting this conclusion:

First, operational activity without implementation tracking: Sample searches located >2,218 pages documenting routine pharmacovigilance operations (AEFI case processing: >303 pages, general signal detection procedures: >679 pages, regulatory actions: >406 pages), contrasted with zero pages produced documenting systematic Plan implementation including audit reports, evaluation reports, governance meeting minutes, AusVaxSafety coordination protocols specific to Output 2.4, or progress tracking against Plan objectives (Tables 3-4, pp.18-21). This demonstrates operational activity occurred but was not tracked as Plan-specific enhanced monitoring implementation.

Second, TGA internal admission: TGA's search notes acknowledged documents fragmented across >5 teams without unified frameworks, with "no guarantee all relevant documents identified," and available documentation "is not likely to answer the applicant's question" regarding implementation progress, outputs, outcomes, and timelines (Attachment D, pp.24-25).

Third, forced choice framework: Evidence indicates two possibilities: either enhanced monitoring was implemented but not adequately documented, raising potential non-compliance risks with Commonwealth recordkeeping obligations under the Archives Act 1983 s 24, or systematic implementation did not occur as envisaged, meaning the enhanced monitoring commitments associated with provisional approval may not have been demonstrably satisfied under the Therapeutic Goods Act 1989 provisional-registration framework. Either scenario points to a material governance deficiency when assessed against the Commonwealth public-sector accountability and performance framework established by the PGPA Act and related policies.

These findings, documented in TGA's response to OAIC directions and conducted by two senior TGA officers (Senior Medical Officer and Assistant Director, Vaccines Surveillance Section), provide the strongest available documentary evidence that TGA did not locate Plan implementation documentation in the sample searches run.

TGA's Principal Medical Advisor estimated the totality keyword search would take "no longer than 30mintues" [sic] (MR22/00538, p.32) — a half-hour, eight-term exercise offered as the basis for concluding that implementation records for a framework covering 68.4 million doses could not be found.

It is notable that the FOI “reasonable search” process required two senior TGA officers (a Senior Medical Officer and an Assistant Director in the Vaccines Surveillance Section) to undertake manual searches for such a critical implementation document, which itself suggests the absence of routine governance or records-management systems that would ordinarily track and surface Plan-level implementation documentation.

At a minimum, it implies three theoretical weaknesses in the TGA’s document control environment:

1. No central register of critical implementation documents
   If senior specialists must manually trawl systems to look for an implementation report, it suggests there is no routine mechanism (register, dashboard, or governance log) that tracks the existence and status of key Plan-level documents.
2. Reliance on individual knowledge rather than systematised recordkeeping
   The need for senior officers to conduct the search implies that locating core governance records depends on who remembers where things might be, rather than on structured records-management controls and metadata.
3. Weak integration between safety governance and FOI/assurance processes
   For a flagship safety commitment, you would normally expect the implementation artefacts to be easily retrievable for internal assurance, external audit, and FOI. The fact they are not suggests a gap between operational safety work and the entity’s broader accountability and documentation frameworks.

Cross-Reference: Full OAIC submission: MR22/00538 (September 2024), Tables 3-4 (pp.18-21) documenting TRIM search methodology and results.

C.5 Conclusion

Statistically, the submission demonstrates that a large body of potentially relevant COVID-19 vaccine safety documentation exists in TRIM, Promapp and related systems, but it does not quantify coverage, does not search key Plan concepts such as AusVaxSafety, and does not identify a single implementation or audit document for the Plan despite using multiple search strings. The heavy reliance on “non-exhaustive sampling”, the omission of critical search terms, and the absence of any Plan-output-by-document matrix mean the submission cannot, on its own numbers, establish that “all reasonable steps” have been taken or that Plan implementation documentation does not exist. Instead, it quantifies a large, poorly mapped search space and confirms that, as at September 2024, TGA has not produced or cannot locate any formal implementation, governance or performance documentation for the COVID-19 Vaccine Safety Monitoring Plan.

The three TGA responses documented in this appendix—two pre-FOI emails and one formal FOI decision—establish TGA’s baseline institutional position regarding Plan implementation documentation as at February 2022. The consistent pattern of substituting generic pharmacovigilance links for Plan-specific evidence, culminating in statutory confirmation that no implementation report exists, provides primary evidence supporting the audit’s governance and performance-measurement findings.

TGA's sample searches in the OAIC review (September 2024) were consistent with this pattern, with findings detailed in B.4 reporting no implementation documentation in the sample searches undertaken. Combined with annual reporting omissions (Appendix B) and Senate testimony (October 2025), the evidence demonstrates a four-year pattern of institutional inability to produce Plan implementation documentation when explicitly requested through multiple channels.

This pattern is characterised by: identical responses across escalating formality levels (email → FOI decision → OAIC review); explicit statutory decision that no implementation report exists; substitution of routine pharmacovigilance outputs for Plan-specific enhanced-monitoring evidence; and inability to locate or meaningfully search for collaboration-framework documentation. See Section 5 (Root Cause Analysis) for examination of whether this pattern represents records-management failure, implementation failure, or both; the Evidence Hierarchy (Section 9.4) classifies this correspondence as Tier 1 primary statutory evidence.

C.6 References

1. Therapeutic Goods Administration (2022). Email response to request for update on implementation of COVID-19 Vaccine Safety Monitoring Plan (31 January 2022). Reference: CCEMS_00780000319. [Email correspondence held by applicant]
2. Therapeutic Goods Administration (2022). Follow-up email response to request for update on implementation of COVID-19 Vaccine Safety Monitoring Plan (18 February 2022). Reference: CCEMS_09930001535. [Email correspondence held by applicant]
3. Therapeutic Goods Administration (2022). *Freedom of Information Request FOI 3643: Notice of Decision* (27 February 2022). Decision maker: Elspeth Kay, Assistant Secretary, Pharmacovigilance Branch. TRIM Reference: D22-5201218. [FOI decision letter held by applicant]
4. Therapeutic Goods Administration (2024). *OAIC Submission MR22/00538 - FOI Review Decision* (20 September 2024). Tables 3-4 (pp.18-21) document TRIM search methodology using eight distinct search terms, yielding no documents TGA classed as Plan implementation records.
5. Therapeutic Goods Administration (2021). *How the TGA monitors COVID-19 vaccine safety* [Video]. YouTube. Available at: https://www.youtube.com/watch?v=PT4M9fX9sPI
6. Therapeutic Goods Administration. *COVID-19 vaccine safety reports*. Australian Government Department of Health and Aged Care. Available at: https://www.tga.gov.au/news/covid-19-vaccine-safety-reports

Appendix D: COVID-19 Vaccine Doses Administered Under Provisional Approval

D.1 Dose Count Summary

By 23 July 2023, more than 68.4 million COVID-19 vaccine doses had been administered in Australia since the rollout commenced in February 2021. The vast majority of these doses were given while primary vaccines remained under provisional approval, and were therefore subject to the enhanced monitoring commitments in the COVID-19 Vaccine Safety Monitoring Plan.

See Section 8 (Conclusion) for discussion of provisional approval scale and precedent implications.

D.2 Data Sources and Verification

Table D.1: COVID-19 Vaccine Dose Data

Source	Report Date	Doses Administered	Period Covered
TGA COVID-19 Vaccine Safety Report	29 June 2023[95]	> 68 million	To 25 June 2023
TGA COVID-19 Vaccine Safety Report	27 July 2023[96]	> 68.4 million	To 23 July 2023
Department of Health, Disability and Ageing AIR Data	14 October 2025[97]	75 million	Total since Feb 2021

D.3 Provisional Approval Status and Transition to Full Registration

The following vaccines transitioned from provisional to full registration during or after the main rollout period, as recorded in TGA's COVID-19 vaccines regulatory status history:[98]

- **Pfizer Comirnaty:** Provisional approval 25 January 2021 → Full registration 13 July 2023[99]
- **Moderna Spikevax:** Provisional approval 9 August 2021 → Full registration 21 April 2023[100]

[95] https://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-29-06-2023
[96] https://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-27-07-2023
[97] https://www.health.gov.au/resources/publications/covid-19-vaccination-vaccination-data-1-october-2025?language=en
[98] https://www.tga.gov.au/products/medicines/prescription-medicines/application-and-market-authorisation/covid-19-vaccine-information-sponsors-industry/covid-19-vaccines-regulatory-status
[99] https://www.tga.gov.au/sites/default/files/2023-08/auspar-comirnaty-230807.pdf
[100] https://www.tga.gov.au/news/media-releases/modernas-covid-19-vaccine-spikevax-receives-approval-full-registration

- **AstraZeneca Vaxzevria:** Provisional approval 15 February 2021 → Withdrawn April 2024[101]

Significance for Plan Implementation

Approximately 68.4 million of the 75 million total doses (about 91.2 per cent) were administered while primary vaccines remained under provisional approval. Enhanced post-market safety monitoring was therefore a critical safeguard accompanying the acceptance of reduced pre-market evidence under the provisional-approval pathway, operationalised at the TGA level through the February 2021 Plan. The unprecedented scale of deployment under provisional approval conditions—Australia's largest-ever use of provisionally approved medicines—heightened the materiality of documented implementation of the COVID-19 Vaccine Safety Monitoring Plan as accountability evidence.

[101] https://www.tga.gov.au/news/regulatory-decision-notices/covid-19-vaccine-astrazeneca-vaxzevria

D.4 References

1. Therapeutic Goods Administration (2023). *COVID-19 vaccine safety report, 29-06-2023*. Australian Government Department of Health and Aged Care. Available at: https://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-29-06-2023
2. Therapeutic Goods Administration (2023). *COVID-19 vaccine safety report – 27 07 2023*. Australian Government Department of Health and Aged Care. Available at: https://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-27-07-2023
3. Australian Government Department of Health and Disability and Ageing. *COVID-19 vaccination – vaccination data – 1 October 2025*. Canberra: Australian Government; 2025 Oct 13 [cited 2026 Jan 6]. Available from: https://www.health.gov.au/resources/publications/covid-19-vaccination-vaccination-data-1-october-2025?language=en.health
4. Australian Government Department of Health and Aged Care (2024). *COVID-19 vaccination, Vaccination data, 10 October 2024*. Available at: https://www.health.gov.au/resources/publications/covid-19-vaccination-vaccination-data-10-october-2024
5. Therapeutic Goods Administration (2023). *COVID-19 vaccines, Regulatory status and history*. Available at: https://www.tga.gov.au/products/medicines/prescription-medicines/application-and-market-authorisation/covid-19-vaccine-information-sponsors-industry/covid-19-vaccines-regulatory-status

Appendix E: FOI and OAIC Timeline

Table E.1: Comprehensive FOI and OAIC Timeline for the COVID-19 Vaccine Safety Monitoring Plan

Date	Document type/stage	Key content	Reference to Safety Monitoring Plan	Response to request
31 Jan 2022	Pre-FOI email – Response 1	TGA provides "robust monitoring" assurance and four generic links (YouTube explainer, weekly safety reports, DAEN, monitoring overview). Encourages adverse event reporting; no Plan-specific material.	None	No outputs, progress or implementation evidence. Only partial AEFI reporting via links; no long-term monitoring, trial or methodology documentation.
18 Feb 2022	Pre-FOI email – Response 2	Second reply repeats the same four links and generic assurances; no new content.	None	Identical to January response; still no Plan-specific implementation information.
18 Feb 2022	FOI 3643 – Request	Applicant seeks a COVID-19 Vaccine Safety Monitoring Plan "implementation report" under the FOI Act.	Explicitly references the Plan and requests implementation documentation.	Initiates formal FOI process.
27 Feb 2022	FOI 3643 – Notice of decision	TGA refuses access under s 24A (no documents exist), stating "no implementation report exists", while also claiming "ample documentation" exists but not as an evaluation report. Repeats the same four links and cites Objective 5 examples (ICMRA, states, ATAGI, NCIRS, ACV) instead of implementation records.	Mentions Plan objectives but provides no implementation evidence.	No progress report; only partial AEFI/recovery material via links; no long-term monitoring, trial or methodology documentation.
2022–2024	OAIC IC review MR22/00538 – commenced	Applicant seeks Information Commissioner review of FOI 3643 decision. Applicant expressed concern about being required to make a new, broader request under the FOI Act on the basis that it would result in additional delay and deny transparency over public	Review concerns whether TGA took "all reasonable steps" to identify documents about Plan implementation.	OAIC opens MR22/00538 investigation.

Date	Document type/stage	Key content	Reference to Safety Monitoring Plan	Response to request
		health issues. Applicant subsequently indicated in June 2024 that they were willing to accept a broader range of material after receiving the Department's decision.		
9 Aug 2024	OAIC directions (s 55V, s 55(2)(e)(ii))	OAIC directs TGA to conduct further searches, document methodology, and demonstrate that all reasonable steps have been taken to find records relevant to the Plan.	Directions explicitly reference Plan objectives and outputs.	Triggers comprehensive TRIM/Promapp search programme.
20 Sept 2024	OAIC submission MR22/00538 – TGA response	TGA provides a detailed submission and attachments documenting eight search terms and an estimated 2,218+ pages from samples. Searches identify extensive routine pharmacovigilance material but no Plan-specific implementation, governance, integration or AusVaxSafety coordination records.	Submission explicitly acknowledges the Plan and searches by objective and strategy.	Records that the sample searches did not locate any Plan-specific implementation or integration documentation.
26 Mar 2025	OAIC Decision [2025] AICmr 54 (IC review of FOI 3643 / MR22/00538)	OAIC issues its decision on the applicant's review of TGA's s 24A "no documents" decision for FOI 3643 (Decision [2025] AICmr 54), considering whether TGA took all reasonable steps to locate records and whether the s 24A refusal can stand. Relying in part on the September 2024 MR22/00538 TRIM searches, the Commissioner accepts that no discrete implementation report or additional Plan-specific records have been identified and upholds TGA's refusal under s 24A.	Decision addresses the Plan only insofar as it relates to the adequacy of TGA's searches and its s 24A decision; it does not assess the quality of pharmacovigilance or Plan implementation.	Confirms, at OAIC level, that TGA's "no implementation report held" position for FOI 3643 stands, leaving the absence of Plan-specific implementation documentation unresolved.

Date	Document type/stage	Key content	Reference to Safety Monitoring Plan	Response to request
1 Apr 2025	FOI 25-0166 – Request	Applicant lodges FOI request, later revised (5, 9 and 20 May) to up to 20 priority documents plus six of the most recent documents per Plan objective (Feb 2021–Feb 2025) across standard document categories (SOPs, workflows, protocols, regulatory-action procedures, public-communication procedures, collaboration mechanisms), with explicit document-type definitions and sampling method.	Request is explicitly structured around the Plan's objectives and TGA document types.	Tests whether a narrowly scoped, Plan-aligned and objectively identifiable sample can be obtained.
24 Apr 2025	FOI 25-0166 – s 24AB(2) consultation notice	TGA advises that preliminary estimates from the relevant line areas identified "more than 399 documents containing 3,926 pages" within scope of the request, and that "there are other areas where searches have not been completed which are likely to hold additional relevant documents". States that the time to review the 399 documents in detail to determine exemptions and apply redactions "would be a substantial and unreasonable diversion of the TGA's resources. Notes that processing would engage the administrative team supporting pharmacovigilance activities and the FOI team; relies on FOI Guidelines paragraph 3.117 regarding the impact of processing on other agency work. Finds that "the balance of	None identified. The notice does not identify any document as implementing or evidencing the Plan.	Practical refusal position signalled. TGA identifies more than 399 documents in scope of the request but states that reviewing and redacting them would substantially and unreasonably divert its resources, directing the applicant to published safety reports and Advisory Committee web pages rather than Plan-related implementation records.

Date	Document type/stage	Key content	Reference to Safety Monitoring Plan	Response to request
		interests does not favour the expenditure of considerable resources by the TGA". Directs the applicant instead to COVID-19 vaccine safety reports and Advisory Committee on Vaccines (ACV) web pages.		
Apr–Jun 2025	FOI 25-0166 – Consultation submissions	Applicant and TGA exchange consultation correspondence. Applicant narrows the request on 5, 9 and 20 May and states that the revised criteria are objective under s 15(2)(b) and that the public interest in Plan accountability outweighs the claimed burden. TGA maintains its view that processing would unreasonably divert resources.	Explicit references to Plan objectives and to the September 2024 MR22/00538 searches.	Shows applicant's progressive narrowing of scope and TGA's sustained reliance on workload arguments.
3 Jun 2025	FOI 25-0166 – Notice of decision	TGA refuses access under s 24(1)(b) on the basis that, following consultation under s 24AB, a practical refusal reason within the meaning of s 24AA still exists. Relies on FOI Guidelines paragraph 3.117 and refers the applicant to published safety reports and Advisory Committee web pages.	None identified. The decision does not identify any document as implementing or evidencing the Plan.	Access refused after consultation and refinement. More than 399 documents initially estimated to be in scope of the original request; none released. Under IC review (MR25/01153).
June 2025	FOI 25-0166 – IC review application (OAIC MR25/01153))	Applicant applies to the OAIC under s 54L for review of the decision refusing access under s 24(1)(b), submitting that the TGA has not demonstrated an unreasonable diversion of resources and that the request is tightly scoped and in the public interest. The application cites the MR22/00538 search	Application reiterates the need for Plan-specific implementation documentation and cites the MR22/00538 search outcomes.	Escalates FOI 25-0166 to independent review. Applicant subsequently responds to the OAIC's s 54Z notice and makes further submissions during September 2025–January 2026.

Date	Document type/stage	Key content	Reference to Safety Monitoring Plan	Response to request
		outcomes and reiterates the need for Plan-specific implementation documentation.		

E.2: Summary of FOI and OAIC Escalation - Timeline Analysis

Across the timeline from January 2022 to October 2025, TGA has consistently declined to produce or release implementation documentation for the COVID-19 Vaccine Safety Monitoring Plan. The pattern shows:

- **Pre-FOI (Jan–Feb 2022):** TGA confirms that no implementation report exists and twice responds with only generic “robust monitoring” assurances and web links, without engaging with the specific request for Plan implementation evidence.

- **FOI 3643 (Feb 2022–Mar 2025):** TGA refuses access under s 24A on the basis that no implementation report is held; in March 2025, OAIC Decision [2025] AICmr 54 accepts TGA’s September 2024 MR22/00538 search evidence and upholds the finding that the requested documents cannot be found or do not exist.

- **OAIC MR22/00538 (Aug–Sept 2024):** Following OAIC directions, TGA conducts sample searches, using eight TRIM search terms and separate sampling that estimated more than 2,218 pages, identifying extensive routine pharmacovigilance documentation but no Plan implementation, governance or AusVaxSafety coordination records were identified.

- **FOI 25-0166 (Apr–Jun 2025):** The TGA's initial estimate was 399+ documents within scope. It refused to process the request on practical refusal grounds under s 24(1)(b) of the FOI Act, stating that the documents sought "have not been previously identified or related to that objective" and that processing would substantially and unreasonably divert its resources.

- **OAIC MR25/01153/s54Z application (Jun 2025–current):** The applicant applied for Information Commissioner review under s 54L of the FOI Act of the TGA's practical refusal decision in FOI 25-0166 on 16 June 2025. The IC review subsequently proceeded under Part VII of the Act, including the s 54Z general notice requirement. Under s 55D, the agency bears the onus of establishing that its decision is justified, while s 55DA requires the agency to use its best endeavours to assist the Information Commissioner. The review remained outstanding at the time of writing (August 2026).

Throughout this period, TGA has not released or produced any documented Plan implementation, governance, integration, or performance measurement records.

Appendix F: Links and Resources

Project overview

Accessible overview of the audit, key findings, methodology and permanent research record: https://paulrekaris.github.io/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/

Research repository

Latest version including updates to the dataset, methodology, and supporting materials:
https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit

Permanent archives

Zenodo: https://doi.org/10.5281/zenodo.17731054

Harvard Dataverse: https://doi.org/10.7910/DVN/BDKZQJ

Australian Web Archive (NLA): https://webarchive.nla.gov.au/tep/221557

Companion dataset archive

Mendeley Data: https://doi.org/10.17632/y5wmt6f8j9

Research papers and preprints

SSRN — Audit report: https://doi.org/10.2139/ssrn.6333058

SSRN — Dataset and methodology paper: https://doi.org/10.2139/ssrn.6610438

SocArXiv — Preprint: https://osf.io/preprints/socarxiv/sb4gz

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Updates and contact

News: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/NEWS.md

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